MOUNJARO- tirzepatide injection, solution
MOUNJARO KWIKPEN- tirzepatide injection, solution
Eli Lilly and Company

HIGHLIGHTS OF PRESCRIBING INFORMATION
These highlights do not include all the information needed to use MOUNJARO safely and effectively. See full prescribing information for MOUNJARO.
MOUNJARO® (tirzepatide) Injection, for subcutaneous use
Initial U.S. Approval: 2022

WARNING: RISK OF THYROID C-CELL TUMORS

See full prescribing information for complete boxed warning.

- **Tirzepatide causes thyroid C-cell tumors in rats. It is unknown whether MOUNJARO causes thyroid C-cell tumors, including medullary thyroid carcinoma (MTC), in humans as the human relevance of tirzepatide-induced rodent thyroid C-cell tumors has not been determined (5.1, 13.1).**
- **MOUNJARO is contraindicated in patients with a personal or family history of MTC or in patients with Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). Counsel patients regarding the potential risk of MTC and symptoms of thyroid tumors (4, 5.1).**

RECENT MAJOR CHANGES

Indications and Usage	
Pediatric Patients 10 Years of Age and Older (1)	12/2025
Major Adverse Cardiovascular Events (1)	08/2026
Dosage and Administration	
Recommended Dosage (2.1)	08/2026
Important Administration Instructions (2.2)	01/2026
Warnings and Precautions	
Severe Gastrointestinal Adverse Reactions (5.6)	12/2025
Diabetic Retinopathy Complications in Patients with a History of Diabetic Retinopathy (5.7)	08/2026
Never Share a MOUNJARO KwikPen Between Patients (5.10)	01/2026

INDICATIONS AND USAGE

MOUNJARO® is a glucose-dependent insulinotropic polypeptide (GIP) receptor and glucagon-like peptide-1 (GLP-1) receptor agonist indicated:

- as an adjunct to diet and exercise to improve glycemic control in adults and pediatric patients 10 years of age and older with type 2 diabetes mellitus. (1)
- to reduce the risk of major adverse cardiovascular (CV) events (CV death, non-fatal myocardial infarction, or non-fatal stroke) in adults with type 2 diabetes mellitus who are at high risk for these events. (1)

DOSAGE AND ADMINISTRATION

- The recommended starting dosage is 2.5 mg injected subcutaneously once weekly. (2.1)
- If additional glycemic control is needed, increase the dosage in 2.5 mg increments after at least 4 weeks on the current dose. (2.1)
- Maximum dosage (2.1):
  - Adults: 15 mg subcutaneously once weekly.
  - Pediatric patients 10 years of age and older: 10 mg subcutaneously once weekly.
- Administer once weekly at any time of day, with or without meals. (2.2)
- Inject subcutaneously in the abdomen, thigh, or have another person inject in the back of the upper arm. Rotate injection sites with each dose. (2.2)
- Refer to the Full Prescribing Information for additional important administration instructions about

MOUNJARO presentations. (2.2)

DOSAGE FORMS AND STRENGTHS

Injection:
Single-dose pen or single-dose vial: 2.5 mg/0.5 mL, 5 mg/0.5 mL, 7.5 mg/0.5 mL, 10 mg/0.5 mL, 12.5 mg/0.5 mL, or 15 mg/0.5 mL (3)
Multi-dose vial or single-patient-use KwikPen®: 10 mg/2.4 mL (4.17 mg/mL) for four 2.5 mg/0.6 mL doses, 20 mg/2.4 mL (8.33 mg/mL) for four 5 mg/0.6 mL doses, 30 mg/2.4 mL (12.5 mg/mL) for four 7.5 mg/0.6 mL doses, 40 mg/2.4 mL (16.7 mg/mL) for four 10 mg/0.6 mL doses, 50 mg/2.4 mL (20.8 mg/mL) for four 12.5 mg/0.6 mL doses, or 60 mg/2.4 mL (25 mg/mL) for four 15 mg/0.6 mL (3)

CONTRAINDICATIONS

- Personal or family history of medullary thyroid carcinoma or in patients with Multiple Endocrine Neoplasia syndrome type 2. (4)
- Known serious hypersensitivity to tirzepatide or any of the excipients in MOUNJARO. (4)

WARNINGS AND PRECAUTIONS

- *Acute Pancreatitis:* Has been observed in patients treated with GLP-1 receptor agonists, or MOUNJARO. Discontinue if pancreatitis is suspected. (5.2)
- *Hypoglycemia with Concomitant Use of Insulin Secretagogues or Insulin:* Concomitant use with an insulin secretagogue or insulin may increase the risk of hypoglycemia, including severe hypoglycemia. Reducing dose of insulin secretagogue or insulin may be necessary. (5.3)
- *Hypersensitivity Reactions:* Serious hypersensitivity reactions (e.g., anaphylaxis and angioedema) have been reported. Discontinue MOUNJARO if suspected and promptly seek medical advice. (5.4)
- *Acute Kidney Injury Due to Volume Depletion:* Monitor renal function in patients reporting adverse reactions that could lead to volume depletion. (5.5)
- *Severe Gastrointestinal Adverse Reactions:* Use has been associated with gastrointestinal adverse reactions, sometimes severe. MOUNJARO is not recommended in patients with severe gastroparesis. (5.6)
- *Diabetic Retinopathy Complications in Patients with a History of Diabetic Retinopathy:* Monitor patients with a history of diabetic retinopathy for progression. (5.7)
- *Acute Gallbladder Disease:* Has occurred in clinical trials. If cholelithiasis is suspected, gallbladder studies and clinical follow-up are indicated. (5.8)
- *Pulmonary Aspiration During General Anesthesia or Deep Sedation:* Has been reported in patients receiving GLP-1 receptor agonists undergoing elective surgeries or procedures. Instruct patients to inform healthcare providers of any planned surgeries or procedures. (5.9)
- *Never Share a MOUNJARO KwikPen Between Patients:* even if the pen needle is changed. (5.10)

ADVERSE REACTIONS

The most common adverse reactions, reported in ≥5% of patients treated with MOUNJARO are nausea, diarrhea, decreased appetite, vomiting, constipation, dyspepsia, and abdominal pain. (6.1)
To report SUSPECTED ADVERSE REACTIONS, contact Eli Lilly and Company at 1-800-LillyRx (1-800-545-5979) or FDA at 1-800-FDA-1088 or www.fda.gov/medwatch.

DRUG INTERACTIONS

MOUNJARO delays gastric emptying and has the potential to impact the absorption of concomitantly administered oral medications. (7.2)

USE IN SPECIFIC POPULATIONS

- *Pregnancy:* Based on animal studies, may cause fetal harm. (8.1)
- *Females of Reproductive Potential:* Advise females using oral contraceptives to switch to a non-oral contraceptive method or add a barrier method of contraception for 4 weeks after initiation and for 4 weeks after each dose escalation. (8.3)

See 17 for PATIENT COUNSELING INFORMATION and Medication Guide.

Revised: 8/2026

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FULL PRESCRIBING INFORMATION

WARNING: RISK OF THYROID C-CELL TUMORS

- **In both male and female rats, tirzepatide causes dose-dependent and treatment-duration-dependent thyroid C-cell tumors at clinically relevant exposures. It is unknown whether MOUNJARO causes thyroid C-cell tumors, including medullary thyroid carcinoma (MTC), in humans as human relevance of tirzepatide-induced rodent thyroid C-cell tumors has not been determined *[see Warnings and Precautions (5.1) and Nonclinical Toxicology (13.1)].***
- **MOUNJARO is contraindicated in patients with a personal or family history of MTC or in patients with Multiple Endocrine Neoplasia syndrome type 2 (MEN 2) *[see Contraindications (4)]*. Counsel patients regarding the potential risk for MTC with the use of MOUNJARO and inform them of symptoms of thyroid tumors (e.g., a mass in the neck, dysphagia, dyspnea, persistent hoarseness). Routine monitoring of serum calcitonin or using thyroid ultrasound is of uncertain value for early detection of MTC in patients treated with MOUNJARO *[see Contraindications (4) and Warnings and Precautions (5.1)].***

1 INDICATIONS AND USAGE

MOUNJARO® is indicated:

- as an adjunct to diet and exercise to improve glycemic control in adults and pediatric patients 10 years of age and older with type 2 diabetes mellitus.
- to reduce the risk of major adverse cardiovascular (CV) events (CV death, non-fatal myocardial infarction (MI), or non-fatal stroke) in adults with type 2 diabetes mellitus who are at high risk for these events.

2 DOSAGE AND ADMINISTRATION

2.1 Recommended Dosage

Follow the starting dosage recommendation and dosage escalation schedule below to reduce the risk of gastrointestinal adverse reactions *[see Warnings and Precautions*

(5.6) and Adverse Reactions (6.1)].

Recommended Starting Dosage and Escalation

- The recommended starting dosage of MOUNJARO is 2.5 mg injected subcutaneously once weekly *[see Dosage and Administration (2.2)]*.
- If additional glycemic control is needed, increase the dosage in 2.5 mg increments after at least 4 weeks on the current dose.
- The maximum dosage of MOUNJARO is:
  - 15 mg injected subcutaneously once weekly in adults.
  - 10 mg injected subcutaneously once weekly in pediatric patients.

Recommendations Regarding Missed Doses

- If a dose is missed, instruct patients to administer MOUNJARO as soon as possible within 4 days (96 hours) after the missed dose. If more than 4 days have passed, skip the missed dose and administer the next dose on the regularly scheduled day. In each case, patients can then resume their regular once weekly dosing schedule.
- The day of weekly administration can be changed, if necessary, as long as the time between the two doses is at least 3 days (72 hours).

2.2 Important Administration Instructions

- Inform patients and their caregiver(s) which MOUNJARO presentation (e.g., vial, pre-filled single-dose pen, single-patient-use KwikPen) they will receive and ensure they receive training appropriate for that specific presentation. If the prescribed MOUNJARO presentation changes, ensure patients and caregivers receive appropriate training and instruct them to consult the Instructions for Use for the newly prescribed presentation.
- Prior to initiation, train patients and their caregiver(s) on proper injection technique for the prescribed MOUNJARO presentation *[see Instructions for Use]*. After training, a patient may self-inject MOUNJARO if the healthcare provider determines that it can be properly administered, except for the following:
  - MOUNJARO KwikPen is not recommended for self-administration by pediatric patients.
  - MOUNJARO KwikPen is not recommended for self-administration by those who are visually impaired.
- Instruct patients using MOUNJARO vials to use a syringe appropriate for dose administration (e.g., a 1 mL syringe capable of measuring a 0.5 mL or 0.6 mL dose) and always use a new syringe and needle for each injection.
- Administer MOUNJARO once weekly, any time of day, with or without meals.
- Inject MOUNJARO subcutaneously in the abdomen, thigh, or have another person inject MOUNJARO in the back of the upper arm.
- Rotate injection sites with each dose.
- Inspect MOUNJARO visually before use. It should appear clear and colorless to slightly yellow. Do not use MOUNJARO if particulate matter or discoloration is seen.
- When using MOUNJARO with insulin, administer as separate injections and never mix. It is acceptable to inject MOUNJARO and insulin in the same body region, but the injections should not be adjacent to each other.

3 DOSAGE FORMS AND STRENGTHS

Injection: Clear, colorless to slightly yellow solution in pre-filled single-dose pens, single-dose vials, multi-dose vials, or single-patient-use KwikPens, each available in the following strengths. The multi-dose vials and single-patient-use KwikPen each contain 4 doses:

Single-dose Pen or Vial
2.5 mg/0.5 mL
5 mg/0.5 mL
7.5 mg/0.5 mL
10 mg/0.5 mL
12.5 mg/0.5 mL
15 mg/0.5 mL

Multi-dose Vial (4 doses per vial)		
Dose per Injection	**Total Strength per Total Volume**	**Strength per mL**
2.5 mg/0.6 mL	10 mg/2.4 mL	4.17 mg/mL
5 mg/0.6 mL	20 mg/2.4 mL	8.33 mg/mL
7.5 mg/0.6 mL	30 mg/2.4 mL	12.5 mg/mL
10 mg/0.6 mL	40 mg/2.4 mL	16.7 mg/mL
12.5 mg/0.6 mL	50 mg/2.4 mL	20.8 mg/mL
15 mg/0.6 mL	60 mg/2.4 mL	25 mg/mL

Single-Patient-Use KwikPen (4 doses per KwikPen)		
Dose per Injection	**Total Strength per Total Volume**	**Strength per mL**
2.5 mg	10 mg/2.4 mL	4.17 mg/mL
5 mg	20 mg/2.4 mL	8.33 mg/mL
7.5 mg	30 mg/2.4 mL	12.5 mg/mL
10 mg	40 mg/2.4 mL	16.7 mg/mL
12.5 mg	50 mg/2.4 mL	20.8 mg/mL
15 mg	60 mg/2.4 mL	25 mg/mL

4 CONTRAINDICATIONS

MOUNJARO is contraindicated in patients with:

- A personal or family history of medullary thyroid carcinoma (MTC) or in patients with Multiple Endocrine Neoplasia syndrome type 2 (MEN 2) *[see Warnings and Precautions (5.1)]*.
- Known serious hypersensitivity to tirzepatide or any of the excipients in MOUNJARO. Serious hypersensitivity reactions, including anaphylaxis and angioedema, have been reported with MOUNJARO *[see Warnings and Precautions (5.4)]*.

5 WARNINGS AND PRECAUTIONS

5.1 Risk of Thyroid C-Cell Tumors

In both sexes of rats, tirzepatide caused a dose-dependent and treatment-duration-

dependent increase in the incidence of thyroid C-cell tumors (adenomas and carcinomas) in a 2-year study at clinically relevant plasma exposures *[see Nonclinical Toxicology (13.1)]*. It is unknown whether MOUNJARO causes thyroid C-cell tumors, including medullary thyroid carcinoma (MTC), in humans as human relevance of tirzepatide-induced rodent thyroid C-cell tumors has not been determined.

MOUNJARO is contraindicated in patients with a personal or family history of MTC or in patients with MEN 2. Counsel patients regarding the potential risk for MTC with the use of MOUNJARO and inform them of symptoms of thyroid tumors (e.g., a mass in the neck, dysphagia, dyspnea, persistent hoarseness).

Routine monitoring of serum calcitonin or using thyroid ultrasound is of uncertain value for early detection of MTC in patients treated with MOUNJARO. Such monitoring may increase the risk of unnecessary procedures, due to the low test specificity for serum calcitonin and a high background incidence of thyroid disease. Significantly elevated serum calcitonin values may indicate MTC and patients with MTC usually have calcitonin values >50 ng/L. If serum calcitonin is measured and found to be elevated, the patient should be further evaluated. Patients with thyroid nodules noted on physical examination or neck imaging should also be further evaluated.

5.2 Acute Pancreatitis

Acute pancreatitis, including fatal and non-fatal hemorrhagic or necrotizing pancreatitis, has been observed in patients treated with GLP-1 receptor agonists, or MOUNJARO *[see Adverse Reactions (6)]*.

After initiation of MOUNJARO, observe patients carefully for signs and symptoms of acute pancreatitis, which may include persistent or severe abdominal pain (sometimes radiating to the back) and which may or may not be accompanied by nausea or vomiting. If pancreatitis is suspected, discontinue MOUNJARO and initiate appropriate management.

5.3 Hypoglycemia with Concomitant Use of Insulin Secretagogues or Insulin

Patients receiving MOUNJARO in combination with an insulin secretagogue (e.g., sulfonylurea) or insulin may have an increased risk of hypoglycemia, including severe hypoglycemia *[see Adverse Reactions (6.1), Drug Interactions (7.1)]*.

The risk of hypoglycemia may be lowered by a reduction in the dose of sulfonylurea (or other concomitantly administered insulin secretagogue) or insulin. Inform patients using these concomitant medications of the risk of hypoglycemia and educate them on the signs and symptoms of hypoglycemia.

5.4 Hypersensitivity Reactions

Serious hypersensitivity reactions (e.g., anaphylaxis, angioedema) have been reported in patients treated with MOUNJARO. If hypersensitivity reactions occur, discontinue use of MOUNJARO; treat promptly per standard of care, and monitor until signs and symptoms resolve. Do not use in patients with a previous serious hypersensitivity reaction to tirzepatide or any of the excipients in MOUNJARO *[see Contraindications (4), Adverse Reactions (6.2)]*.

Anaphylaxis and angioedema have been reported with GLP-1 receptor agonists. Use caution in patients with a history of angioedema or anaphylaxis with a GLP-1 receptor

agonist because it is unknown whether such patients will be predisposed to these reactions with MOUNJARO.

5.5 Acute Kidney Injury Due to Volume Depletion

There have been postmarketing reports of acute kidney injury, in some cases requiring hemodialysis, in patients treated with GLP-1 receptor agonists, or MOUNJARO. The majority of the reported events occurred in patients who experienced gastrointestinal adverse reactions leading to dehydration such as nausea, vomiting, or diarrhea *[see Adverse Reactions (6)]*.

Monitor renal function in patients reporting adverse reactions to MOUNJARO that could lead to volume depletion, especially during dosage initiation and escalation of MOUNJARO.

5.6 Severe Gastrointestinal Adverse Reactions

Use of MOUNJARO has been associated with gastrointestinal adverse reactions, sometimes severe *[see Adverse Reactions (6)]*. In the pool of placebo-controlled trials in adults, severe gastrointestinal adverse reactions occurred more frequently among patients receiving MOUNJARO (5 mg 1.3%, 10 mg 0.4%, 15 mg 1.2%) than placebo (0.9%). Severe gastrointestinal adverse reactions have also been reported postmarketing with GLP-1 receptor agonists.

MOUNJARO is not recommended in patients with severe gastroparesis.

5.7 Diabetic Retinopathy Complications in Patients with a History of Diabetic Retinopathy

Temporary worsening of diabetic retinopathy has been reported with rapid improvement in glucose control. Patients with a history of diabetic retinopathy should be monitored for progression of diabetic retinopathy.

5.8 Acute Gallbladder Disease

Acute events of gallbladder disease such as cholelithiasis or cholecystitis have been reported in GLP-1 receptor agonist trials and postmarketing.

In MOUNJARO placebo-controlled clinical trials in adults, acute gallbladder disease (cholelithiasis, biliary colic, and cholecystectomy) was reported by 0.6% of MOUNJARO-treated patients and 0% of placebo-treated patients. If cholelithiasis is suspected, gallbladder diagnostic studies and appropriate clinical follow-up are indicated.

5.9 Pulmonary Aspiration During General Anesthesia or Deep Sedation

MOUNJARO delays gastric emptying *[see Clinical Pharmacology (12.2)]*. There have been rare postmarketing reports of pulmonary aspiration in patients receiving GLP-1 receptor agonists undergoing elective surgeries or procedures requiring general anesthesia or deep sedation who had residual gastric contents despite reported adherence to preoperative fasting recommendations.

Available data are insufficient to inform recommendations to mitigate the risk of pulmonary aspiration during general anesthesia or deep sedation in patients taking MOUNJARO, including whether modifying preoperative fasting recommendations or

temporarily discontinuing MOUNJARO could reduce the incidence of retained gastric contents. Instruct patients to inform healthcare providers prior to any planned surgeries or procedures if they are taking MOUNJARO.

5.10 Never Share a MOUNJARO KwikPen Between Patients

Never share MOUNJARO KwikPen between patients, even if the pen needle is changed. Sharing poses a risk for transmission of blood-borne pathogens.

6 ADVERSE REACTIONS

The following serious adverse reactions are described below or elsewhere in the prescribing information:

- Risk of Thyroid C-cell Tumors *[see Warnings and Precautions (5.1)]*
- Acute Pancreatitis *[see Warnings and Precautions (5.2)]*
- Hypoglycemia with Concomitant Use of Insulin Secretagogues or Insulin *[see Warnings and Precautions (5.3)]*
- Hypersensitivity Reactions *[see Warnings and Precautions (5.4)]*
- Acute Kidney Injury Due to Volume Depletion *[see Warnings and Precautions (5.5)]*
- Severe Gastrointestinal Adverse Reactions *[see Warnings and Precautions (5.6)]*
- Diabetic Retinopathy Complications in Patients with a History of Diabetic Retinopathy *[see Warnings and Precautions (5.7)]*
- Acute Gallbladder Disease *[see Warnings and Precautions (5.8)]*
- Pulmonary Aspiration During General Anesthesia or Deep Sedation *[see Warnings and Precautions (5.9)]*

6.1 Clinical Trials Experience

Because clinical trials are conducted under widely varying conditions, adverse reaction rates observed in the clinical trials of a drug cannot be directly compared to rates in the clinical trials of another drug and may not reflect the rates observed in practice.

Adverse Reactions in the Clinical Trials of Adults with Type 2 Diabetes Mellitus

Pool of Two Placebo-Controlled Clinical Trials in Adults

The data in Table 1 are derived from 2 placebo-controlled trials [1 monotherapy trial (SURPASS-1) and 1 trial in combination with basal insulin with or without metformin (SURPASS-5)] in adult patients with type 2 diabetes mellitus *[see Clinical Studies (14.2, 14.4)]*. These data reflect exposure of 718 patients to MOUNJARO and a mean duration of exposure to MOUNJARO of 36.6 weeks. The mean age of patients was 58 years, 4% were 75 years or older and 54% were male. The population was 57% White, 27% Asian, 13% American Indian or Alaska Native, and 3% Black or African American; 25% identified as Hispanic or Latino ethnicity. At baseline, patients had type 2 diabetes mellitus for an average of 9.1 years with a mean HbA1c of 8.1%. As assessed by baseline fundoscopic examination, 13% of the population had retinopathy. At baseline, eGFR was $\geq$90 mL/min/1.73 m^2 in 53%, 60 to 90 mL/min/1.73 m^2 in 39%, 45 to 60 mL/min/1.73 m^2 in 7%, and 30 to 45 mL/min/1.73 m^2 in 1% of patients.

Pool of Seven Controlled Clinical Trials

Adverse reactions were also evaluated in a larger pool of adult patients with type 2 diabetes mellitus participating in seven controlled clinical trials which included two placebo-controlled trials (SURPASS-1 and -5), three trials of MOUNJARO in combination with metformin, sulfonylureas, and/or SGLT2 Inhibitors (SURPASS-2, -3, -4) *[see Clinical Studies (14.3)]* and two additional trials conducted in Japan. In this pool, a total of 5119 adult patients with type 2 diabetes mellitus were treated with MOUNJARO for a mean duration of 48.1 weeks. The mean age of patients was 58 years, 4% were 75 years or older and 58% were male. The population was 65% White, 24% Asian, 7% American Indian or Alaska Native, and 3% Black or African American; 38% identified as Hispanic or Latino ethnicity. At baseline, patients had type 2 diabetes mellitus for an average of 9.1 years with a mean HbA1c of 8.3%. As assessed by baseline fundoscopic examination, 15% of the population had retinopathy. At baseline, eGFR was ≥90 mL/min/1.73 m^2 in 52%, 60 to 90 mL/min/1.73 m^2 in 40%, 45 to 60 mL/min/1.73 m^2 in 6%, and 30 to 45 mL/min/1.73 m^2 in 1% of patients.

Common Adverse Reactions in Adults

Table 1 shows common adverse reactions, not including hypoglycemia, associated with the use of MOUNJARO in the pool of placebo-controlled trials in adults. These adverse reactions occurred more commonly on MOUNJARO than on placebo and occurred in at least 5% of patients treated with MOUNJARO.

Table 1: Adverse Reactions in Pool of Placebo-Controlled Trials Reported in ≥5% of MOUNJARO-treated Adult Patients with Type 2 Diabetes Mellitus

Adverse Reaction	Placebo (N=235) %	MOUNJARO 5 mg (N=237) %	MOUNJARO 10 mg (N=240) %	MOUNJARO 15 mg (N=241) %
Nausea	4	12	15	18
Diarrhea	9	12	13	17
Decreased Appetite	1	5	10	11
Vomiting	2	5	5	9
Constipation	1	6	6	7
Dyspepsia	3	8	8	5
Abdominal Pain	4	6	5	5

Note: Percentages reflect the number of patients who reported at least 1 occurrence of the adverse reaction.

In the pool of seven clinical trials in adults, the types and frequency of common adverse reactions, not including hypoglycemia, were similar to those listed in Table 1.

In a CV outcomes trial of 6,647 adults with type 2 diabetes mellitus and established CV disease exposed to MOUNJARO, adverse reactions reported with MOUNJARO were similar with the exception of dysesthesia (0.7%), decreased appetite (17%), constipation (13%), vomiting (12%), abdominal pain (10%), and fatigue (10%) *[see Clinical Studies (14.6)]*.

Gastrointestinal Adverse Reactions

In the pool of placebo-controlled trials in adults, gastrointestinal adverse reactions

occurred more frequently among patients receiving MOUNJARO than placebo (placebo 20.4%, MOUNJARO 5 mg 37.1%, MOUNJARO 10 mg 39.6%, MOUNJARO 15 mg 43.6%). More patients receiving MOUNJARO 5 mg (3.0%), MOUNJARO 10 mg (5.4%), and MOUNJARO 15 mg (6.6%) discontinued treatment due to gastrointestinal adverse reactions than patients receiving placebo (0.4%). The majority of reports of nausea, vomiting, and/or diarrhea occurred during dose escalation and decreased over time.

The following gastrointestinal adverse reactions were reported more frequently in MOUNJARO-treated adult patients than placebo-treated patients (frequencies listed, respectively, as: placebo; 5 mg; 10 mg; 15 mg): eructation (0.4%, 3.0%, 2.5%, 3.3%), flatulence (0%, 1.3%, 2.5%, 2.9%), gastroesophageal reflux disease (0.4%, 1.7%, 2.5%, 1.7%), abdominal distension (0.4%, 0.4%, 2.9%, 0.8%).

Adverse Reactions in the Clinical Trial of Pediatric Patients 10 Years of Age and Older with Type 2 Diabetes Mellitus

MOUNJARO was administered to 97 pediatric patients 10 years of age and older with type 2 diabetes mellitus for a mean duration of 39.9 weeks *[see Clinical Studies (14.5)]*. The mean age was 15 years and 61% of patients were female. The population was 58% White, 11% Black or African American, 6% Asian, 20% American Indian or Alaska Native, and 5% were other races; 66% identified as Hispanic or Latino ethnicity. At baseline, pediatric patients had type 2 diabetes mellitus for an average of 2.4 years with a mean HbA1c of 8.0%.

The incidences of adverse reactions reported in pediatric patients treated with MOUNJARO 5 mg and 10 mg subcutaneously once-weekly were consistent with those described above for adult patients with type 2 diabetes mellitus with the exception of a higher incidence of vomiting, abdominal pain, and hypoglycemia. During the 30-week placebo-controlled period of the study, vomiting occurred in 3%, 16%, and 12% of patients and abdominal pain occurred in 9%, 22%, and 15% of patients treated with placebo, MOUNJARO 5 mg, and 10 mg, respectively.

Other Adverse Reactions in Adults and Pediatric Patients 10 Years of Age and Older with Type 2 Diabetes Mellitus

Hypoglycemia

Adults with Type 2 Diabetes Mellitus

Table 2 summarizes the incidence of hypoglycemic events in the placebo-controlled trials in adults.

Table 2: Hypoglycemia Adverse Reactions in Placebo-Controlled Trials in Adult Patients with Type 2 Diabetes Mellitus

	Placebo **%**	**MOUNJARO 5 mg %**	**MOUNJARO 10 mg %**	**MOUNJARO 15 mg %**
Monotherapy				
(40 weeks)*	N=115	N=121	N=119	N=120
Blood glucose <54 mg/dL	1	0	0	0
Severe hypoglycemia**	0	0	0	0

Add-on to Basal Insulin with or without Metformin				
(40 weeks)*	N=120	N=116	N=119	N=120
Blood glucose <54 mg/dL	13	16	19	14
Severe hypoglycemia**	0	0	2	1

Note: Percentages reflect the number of patients who reported at least 1 episode of hypoglycemia in respective categories.

* Reflects the study treatment period. Data include events occurring during 4 weeks of treatment-free safety follow up. Events after introduction of a new glucose-lowering treatment are excluded.

** Episodes requiring the assistance of another person to actively administer carbohydrate, glucagon, or other resuscitative actions.

Hypoglycemia was more frequent when MOUNJARO was used in combination with a sulfonylurea *[see Clinical Studies (14)]*. In an adult clinical trial up to 104 weeks of treatment, when administered with a sulfonylurea, hypoglycemia (glucose level <54 mg/dL) occurred in 13.8%, 9.9%, and 12.8%, and severe hypoglycemia occurred in 0.5%, 0%, and 0.6% of patients treated with MOUNJARO 5 mg, 10 mg, and 15 mg, respectively.

Pediatric Patients 10 years of Age and Older with Type 2 Diabetes Mellitus

No severe hypoglycemia episodes were reported during the trial in pediatric patients 10 years of age and older with type 2 diabetes mellitus. Table 3 summarizes the incidence of hypoglycemic events with blood glucose <54 mg/dL in this trial.

Table 3: Hypoglycemia Adverse Reactions in the 30 Week Trial of MOUNJARO Added to Metformin or Basal Insulin, or Both in Pediatric Patients 10 Years of Age and Older with Type 2 Diabetes Mellitus

	Placebo %	**MOUNJARO 5 mg %**	**MOUNJARO 10 mg %**
Add on to basal insulin with or without metformin[a]	N=10	N=10	N=11
Blood glucose <54 mg/dL	10	30	27
Add on to metformin alone[a]	N=24	N=22	N=22
Blood glucose <54 mg/dL	4	9	9

Note: Percentages reflect the number of patients who reported at least 1 episode of blood glucose <54 mg/dL.

[a] Events after the introduction of a new glucose-lowering treatment are excluded.

Acute Pancreatitis

In clinical studies, 14 events of acute pancreatitis were confirmed by adjudication in 13 MOUNJARO-treated adult patients (0.23 patients per 100 years of exposure) versus 3 events in 3 comparator-treated patients (0.11 patients per 100 years of exposure).

Heart Rate Increase

In the pool of placebo-controlled trials in adults, treatment with MOUNJARO resulted in a mean increase in heart rate of 2 to 4 beats per minute compared to a mean increase of 1 beat per minute in placebo-treated patients. Episodes of sinus tachycardia, associated

with a concomitant increase from baseline in heart rate of ≥15 beats per minute, also were reported in 4.3%, 4.6%, 5.9% and 10% of subjects treated with placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. For patients enrolled in Japan, these episodes were reported in 7% (3/43), 7.1% (3/42), 9.3% (4/43), and 23% (10/43) of patients treated with placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. The clinical relevance of heart rate increases is uncertain.

Hypersensitivity Reactions

Hypersensitivity reactions have been reported with MOUNJARO in the pool of placebo-controlled trials in adults, sometimes severe (e.g., urticaria and eczema); hypersensitivity reactions were reported in 3.2% of MOUNJARO-treated patients compared to 1.7% of placebo-treated patients.

In the pool of seven clinical trials in adults, hypersensitivity reactions occurred in 106/2,570 (4.1%) of MOUNJARO-treated adult patients with anti-tirzepatide antibodies and in 73/2,455 (3.0%) of MOUNJARO-treated patients who did not develop anti-tirzepatide antibodies. In the clinical trial in pediatric patients 10 years of age and older, hypersensitivity reactions occurred in 2/50 (4%) of MOUNJARO-treated pediatric patients with anti-tirzepatide antibodies and in 0/43 (0%) of MOUNJARO-treated pediatric patients who did not develop anti-tirzepatide antibodies *[see Clinical Pharmacology (12.6)].*

Injection Site Reactions

In the pool of placebo-controlled trials in adults, injection site reactions were reported in 3.2% of MOUNJARO-treated patients compared to 0.4% of placebo-treated patients.

In the pool of seven clinical trials, injection site reactions occurred in 119/2,570 (4.6%) of MOUNJARO-treated adult patients with anti-tirzepatide antibodies and in 18/2,455 (0.7%) of MOUNJARO-treated adult patients who did not develop anti-tirzepatide antibodies. In the clinical trial in pediatric patients 10 years of age and older, injection site reactions occurred in 3/50 (6%) of MOUNJARO-treated pediatric patients with anti-tirzepatide antibodies and in 0/43 (0%) of MOUNJARO-treated pediatric patients who did not develop anti-tirzepatide antibodies *[see Clinical Pharmacology (12.6)].*

Acute Gallbladder Disease

In the pool of placebo-controlled clinical trials in adults, acute gallbladder disease (cholelithiasis, biliary colic and cholecystectomy) was reported by 0.6% of MOUNJARO-treated patients and 0% of placebo-treated patients.

Dysesthesia

In the pool of placebo-controlled clinical trials in adults, dysesthesia was reported by 0.4%, 0.4%, and 0.4% of patients treated with MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. No events were reported by patients receiving placebo. In a CV outcomes trial, 0.7% of MOUNJARO-treated patients reported dysesthesia.

Dysgeusia

In the pool of placebo-controlled clinical trials in adults, dysgeusia was reported by 0.1% of MOUNJARO-treated patients and 0% of placebo-treated patients.

Laboratory Abnormalities

Amylase and Lipase Increase

In the pool of placebo-controlled clinical trials in adults, treatment with MOUNJARO resulted in mean increases from baseline in serum pancreatic amylase concentrations of 33% to 38% and serum lipase concentrations of 31% to 42%. Placebo-treated patients had a mean increase from baseline in pancreatic amylase of 4% and no changes were observed in lipase. The clinical significance of elevations in lipase or amylase with MOUNJARO is unknown in the absence of other signs and symptoms of pancreatitis.

In a CV outcomes trial, treatment with MOUNJARO resulted in mean increases from baseline in pancreatic amylase of 29% and lipase of 28%.

6.2 Postmarketing Experience

The following adverse reactions have been reported during post-approval use of MOUNJARO. Because these reactions are reported voluntarily from a population of uncertain size, it is not always possible to reliably estimate their frequency or establish a causal relationship to drug exposure.

Hypersensitivity: anaphylaxis, angioedema

Gastrointestinal: acute pancreatitis, hemorrhagic and necrotizing pancreatitis sometimes resulting in death, ileus, intestinal obstruction, severe constipation including fecal impaction

Pulmonary: Pulmonary aspiration has occurred in patients receiving GLP-1 receptor agonists undergoing elective surgeries or procedures requiring general anesthesia or deep sedation

Renal: acute renal failure or worsening of chronic renal failure, sometimes requiring hemodialysis

Skin and Subcutaneous Tissue: alopecia

7 DRUG INTERACTIONS

7.1 Concomitant Use with an Insulin Secretagogue (e.g., Sulfonylurea) or with Insulin

When initiating MOUNJARO, consider reducing the dose of concomitantly administered insulin secretagogues (e.g., sulfonylureas) or insulin to reduce the risk of hypoglycemia *[see Warnings and Precautions (5.3)].*

7.2 Oral Medications

MOUNJARO delays gastric emptying and thereby has the potential to impact the absorption of concomitantly administered oral medications. Caution should be exercised when oral medications are concomitantly administered with MOUNJARO.

Monitor patients on oral medications dependent on threshold concentrations for efficacy and those with a narrow therapeutic index (e.g., warfarin) when concomitantly administered with MOUNJARO.

Advise patients using oral hormonal contraceptives to switch to a non-oral contraceptive method or add a barrier method of contraception for 4 weeks after initiation and for 4 weeks after each dose escalation with MOUNJARO. Hormonal contraceptives that are

not administered orally should not be affected *[see Use in Specific Populations (8.3) and Clinical Pharmacology (12.2, 12.3)].*

8 USE IN SPECIFIC POPULATIONS

8.1 Pregnancy

Risk Summary

Available data with MOUNJARO use in pregnant women are insufficient to evaluate for a drug-related risk of major birth defects, miscarriage, or other adverse maternal or fetal outcomes. There are risks to the mother and fetus associated with poorly controlled diabetes in pregnancy *(see Clinical Considerations)*. Based on animal reproduction studies, there may be risks to the fetus from exposure to tirzepatide during pregnancy. MOUNJARO should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus.

In pregnant rats administered tirzepatide during organogenesis, fetal growth reductions and fetal abnormalities occurred at clinical exposure in maternal rats based on AUC. In rabbits administered tirzepatide during organogenesis, fetal growth reductions were observed at clinically relevant exposures based on AUC. These adverse embryo/fetal effects in animals coincided with pharmacological effects on maternal weight and food consumption *(see Data)*.

The estimated background risk of major birth defects is 6–10% in women with pre-gestational diabetes with an HbA1c >7% and has been reported to be as high as 20–25% in women with an HbA1c >10%. The estimated background risk of miscarriage for the indicated population is unknown. In the U.S. general population, the estimated background risk of major birth defects and miscarriage in clinically recognized pregnancies is 2–4% and 15–20%, respectively.

Clinical Considerations

Disease-Associated Maternal and/or Embryo/Fetal Risk

Poorly controlled diabetes in pregnancy increases the maternal risk for diabetic ketoacidosis, pre-eclampsia, spontaneous abortions, preterm delivery, and delivery complications. Poorly controlled diabetes increases the fetal risk for major birth defects, stillbirth, and macrosomia-related morbidity.

Data

Animal Data

In pregnant rats given twice weekly subcutaneous doses of 0.02, 0.1, and 0.5 mg/kg tirzepatide (0.03-, 0.07-, and 0.5-fold the MRHD of 15 mg once weekly based on AUC) during organogenesis, increased incidences of external, visceral, and skeletal malformations, increased incidences of visceral and skeletal developmental variations, and decreased fetal weights coincided with pharmacologically-mediated reductions in maternal body weights and food consumption at 0.5 mg/kg. In pregnant rabbits given once weekly subcutaneous doses of 0.01, 0.03, or 0.1 mg/kg tirzepatide (0.01-, 0.06-, and 0.2-fold the MRHD) during organogenesis, pharmacologically-mediated effects on the gastrointestinal system resulting in maternal mortality or abortion in a few rabbits occurred at all dose levels. Reduced fetal weights associated with decreased maternal

food consumption and body weights were observed at 0.1 mg/kg. In a pre- and post-natal study in rats administered subcutaneous doses of 0.02, 0.10, or 0.25 mg/kg tirzepatide twice weekly from implantation through lactation, F_1 pups from F_0 maternal rats given 0.25 mg/kg tirzepatide had statistically significant lower mean body weight when compared to controls from post-natal day 7 through post-natal day 126 for males and post-natal day 56 for females.

8.2 Lactation

Risk Summary

In a single-dose clinical lactation study, the concentration of tirzepatide in breast milk was found to be either undetectable or low compared to the maternal administered dose (*see Data*). There are no available data on the effects of tirzepatide on the breastfed infant or on milk production. The developmental and health benefits of breastfeeding should be considered along with the mother's clinical need for MOUNJARO and any potential adverse effects on the breastfed infant from MOUNJARO or from the underlying maternal condition.

Data

Following subcutaneous administration of a single 5 mg dose to 11 healthy lactating adult females, the concentration of tirzepatide in breast milk was found to be undetectable (limit of detection in breast milk 4 ng/mL) in 164/171 samples assayed. The cumulative amount of tirzepatide detected in the remaining 7 breast milk samples over the 28-day sampling window was equivalent to less than 0.02% of the maternal administered dose, with the last measurable concentrations occurring 5 days post-dose. The AUC of tirzepatide in breast milk could not be calculated, due to insufficient quantifiable concentrations.

8.3 Females and Males of Reproductive Potential

Contraception

Use of MOUNJARO may reduce the efficacy of oral hormonal contraceptives due to delayed gastric emptying. This delay is largest after the first dose and diminishes over time. Advise patients using oral hormonal contraceptives to switch to a non-oral contraceptive method or add a barrier method of contraception for 4 weeks after initiation and for 4 weeks after each dose escalation with MOUNJARO *[see Drug Interactions (7.2) and Clinical Pharmacology (12.2, 12.3)].*

8.4 Pediatric Use

The safety and effectiveness of MOUNJARO as an adjunct to diet and exercise to improve glycemic control in pediatric patients 10 years of age and older with type 2 diabetes mellitus have been established. Use of MOUNJARO for this indication is supported by a 30-week, randomized, double-blind, placebo-controlled trial with a 22-week open label extension in 99 pediatric patients *[see Clinical Studies (14.5)]*.

Adverse reactions reported in pediatric patients 10 years of age and older treated with MOUNJARO were similar to those reported in adults with the exception of a higher incidence of vomiting, abdominal pain, and hypoglycemia *[see Adverse Reactions (6.1)]*.

The safety and effectiveness of MOUNJARO have not been established in pediatric

patients less than 10 years of age.

8.5 Geriatric Use

In the pool of seven clinical trials, 1,539 (30.1%) MOUNJARO-treated patients were 65 years of age or older, and 212 (4.1%) MOUNJARO-treated patients were 75 years of age or older at baseline *[see Adverse Reactions (6.1), Clinical Studies (14.2, 14.3, 14.4)]*.

In a CV outcomes trial, 3,327 (50%) of MOUNJARO-treated patients were 65 years of age or older, and 705 (11%) of MOUNJARO-treated patients were 75 years of age or older at baseline *[see Clinical Studies (14.6)]*.

No overall differences in safety or effectiveness of MOUNJARO have been observed between patients 65 years of age and older and younger adult patients.

8.6 Renal Impairment

No dosage adjustment of MOUNJARO is recommended for patients with renal impairment. In subjects with renal impairment including end-stage renal disease (ESRD), no change in tirzepatide pharmacokinetics (PK) was observed *[see Clinical Pharmacology (12.3)]*. Monitor renal function when initiating or escalating doses of MOUNJARO in patients with renal impairment reporting severe adverse gastrointestinal reactions *[see Warnings and Precautions (5.5)].*

8.7 Hepatic Impairment

No dosage adjustment of MOUNJARO is recommended for patients with hepatic impairment. In a clinical pharmacology study in subjects with varying degrees of hepatic impairment, no change in tirzepatide PK was observed *[see Clinical Pharmacology (12.3)]*.

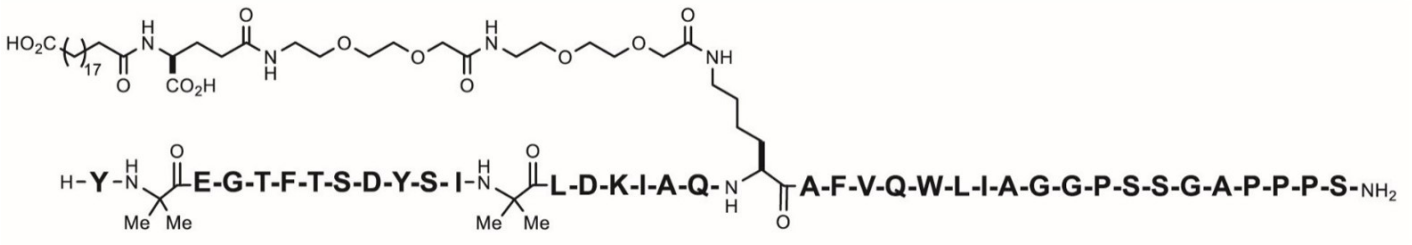
10 OVERDOSAGE

In the event of an overdosage, consider contacting the Poison Help line (1-800-222-1222) or a medical toxicologist for additional overdosage management recommendations. Initiate appropriate supportive treatment according to the patient's clinical signs and symptoms. A period of observation and treatment for these symptoms may be necessary, taking into account the half-life of tirzepatide of approximately 5 days.

11 DESCRIPTION

MOUNJARO (tirzepatide) injection, for subcutaneous use, contains tirzepatide, a once weekly GIP receptor and GLP-1 receptor agonist. Tirzepatide is based on the GIP sequence and contains aminoisobutyric acid (Aib) in positions 2 and 13, a C-terminal amide, and Lys residue at position 20 that is attached to 1,20-eicosanedioic acid via a linker. The molecular weight is 4813.53 Da and the empirical formula is $C_{225}H_{348}N_{48}O_{68}$.

Structural formula:



MOUNJARO is a clear, colorless to slightly yellow, sterile solution for subcutaneous use. Each single-dose pen or single-dose vial contains a 0.5 mL solution of 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg, or 15 mg of tirzepatide and the following excipients: sodium chloride (4.1 mg), sodium phosphate dibasic heptahydrate (0.7 mg), and water for injection. Each multi-dose vial or single-patient-use KwikPen contains 2.4 mL of solution, which provides 4 doses of 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg, or 15 mg of tirzepatide per 0.6 mL. Each dose contains the following excipients: benzyl alcohol (5.4 mg), glycerin (4.8 mg), phenol (1.08 mg), sodium chloride (1.05 mg), sodium phosphate dibasic heptahydrate (0.8 mg), and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH. MOUNJARO has a pH of 6.5 to 7.5.

Each single-patient-use KwikPen contains additional volume to allow for device priming.

12 CLINICAL PHARMACOLOGY

12.1 Mechanism of Action

Tirzepatide is a GIP receptor and GLP-1 receptor agonist. It contains a C20 fatty diacid that enables albumin binding and prolongs the half-life. Tirzepatide selectively binds to and activates both the GIP and GLP-1 receptors, the targets for native GIP and GLP-1.

Tirzepatide enhances first- and second-phase insulin secretion, and reduces glucagon levels, both in a glucose-dependent manner.

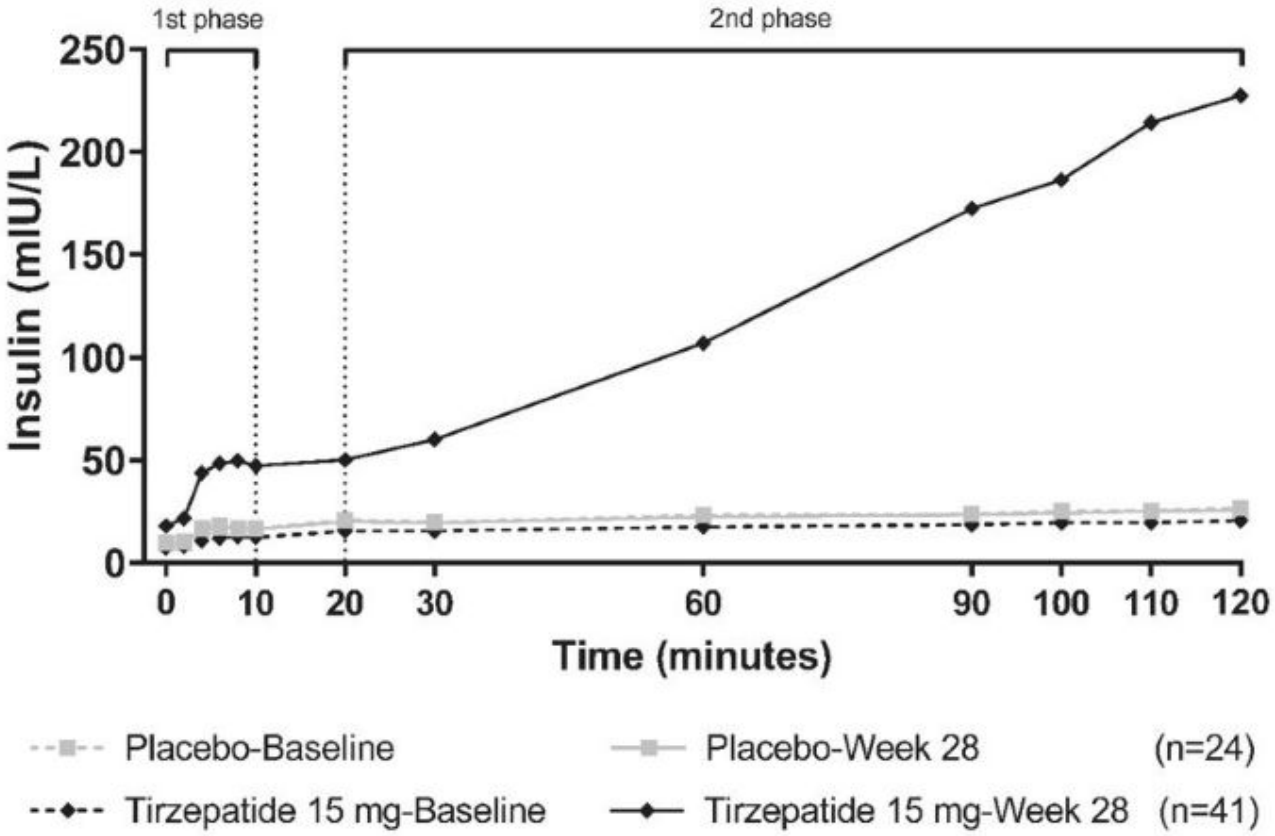
12.2 Pharmacodynamics

Tirzepatide lowers fasting and postprandial glucose concentration, decreases food intake, and reduces body weight in patients with type 2 diabetes mellitus.

First and Second-Phase Insulin Secretion

Tirzepatide enhances the first- and second-phase insulin secretion. (Figure 1)

Figure 1: Mean insulin concentration at 0-120 minutes during hyperglycemic clamp at baseline and Week 28



Insulin Sensitivity

Tirzepatide increases insulin sensitivity, as demonstrated in a hyperinsulinemic euglycemic clamp study after 28 weeks of treatment.

Glucagon Secretion

Tirzepatide reduces fasting and postprandial glucagon concentrations. Tirzepatide 15 mg reduced fasting glucagon concentration by 28% and glucagon AUC after a mixed meal by 43%, compared with no change for placebo after 28 weeks of treatment.

Gastric Emptying

Tirzepatide delays gastric emptying. The delay is largest after the first dose and this effect diminishes over time. Tirzepatide slows post-meal glucose absorption and reduces postprandial glucose.

Exposure-Response Relationships

Exposure-response analyses integrating phase 2 and phase 3 efficacy data predict that after 52 weeks, tirzepatide 2.5 mg administered once weekly may reduce HbA1c by approximately 1.8% and fasting plasma glucose by approximately 43 mg/dL.

12.3 Pharmacokinetics

The pharmacokinetics of tirzepatide is similar between healthy subjects and patients with type 2 diabetes mellitus. Steady-state plasma tirzepatide concentrations were achieved following 4 weeks of once weekly administration. Tirzepatide exposure increases in a dose-proportional manner.

Absorption

Following subcutaneous administration, the time to maximum plasma concentration of tirzepatide ranges from 8 to 72 hours. The mean absolute bioavailability of tirzepatide following subcutaneous administration is 80%. Similar exposure was achieved with subcutaneous administration of tirzepatide in the abdomen, thigh, or upper arm.

Distribution

The mean apparent steady-state volume of distribution of tirzepatide following subcutaneous administration in patients with type 2 diabetes mellitus is approximately 10.3 L. Tirzepatide is highly bound to plasma albumin (99%).

Elimination

The apparent population mean clearance of tirzepatide is 0.061 L/h with an elimination half-life of approximately 5 days, enabling once-weekly dosing.

Metabolism

Tirzepatide is metabolized by proteolytic cleavage of the peptide backbone, beta-oxidation of the C20 fatty diacid and amide hydrolysis.

Excretion

The primary excretion routes of tirzepatide metabolites are via urine and feces. Intact tirzepatide is not observed in urine or feces.

Specific Populations

The intrinsic factors of age, gender, race, ethnicity, or body weight do not have a clinically relevant effect on the PK of tirzepatide.

Pediatric Patients

A population pharmacokinetic analysis was conducted for tirzepatide 5 mg and 10 mg using data from 93 pediatric patients 10 years of age and older with type 2 diabetes mellitus. The tirzepatide exposure in pediatric patients was within the range observed in adult patients.

Patients with Renal Impairment

Renal impairment does not impact the pharmacokinetics of tirzepatide. The pharmacokinetics of tirzepatide after a single 5 mg dose was evaluated in patients with different degrees of renal impairment (mild, moderate, severe, ESRD) compared with subjects with normal renal function. This was also shown for patients with both type 2 diabetes mellitus and renal impairment based on data from clinical studies *[see Use in Specific Populations (8.6)]*.

Patients with Hepatic Impairment

Hepatic impairment does not impact the pharmacokinetics of tirzepatide. The pharmacokinetics of tirzepatide after a single 5 mg dose was evaluated in patients with different degrees of hepatic impairment (mild, moderate, severe) compared with subjects with normal hepatic function *[see Use in Specific Populations (8.7)]*.

Drug Interactions Studies

Potential for Tirzepatide to Influence the Pharmacokinetics of Other Drugs

In vitro studies have shown low potential for tirzepatide to inhibit or induce CYP enzymes, and to inhibit drug transporters.

MOUNJARO delays gastric emptying and thereby has the potential to impact the absorption of concomitantly administered oral medications *[see Drug Interactions (7.2)]*.

The impact of tirzepatide on gastric emptying was greatest after a single dose of 5 mg and diminished after subsequent doses.

Following a first dose of tirzepatide 5 mg, acetaminophen maximum concentration (C_{max}) was reduced by 50%, and the median peak plasma concentration (t_{max}) occurred 1 hour later. After coadministration at week 4, there was no meaningful impact on acetaminophen C_{max} and t_{max}. Overall acetaminophen exposure (AUC_{0-24hr}) was not influenced.

Following administration of a combined oral contraceptive (0.035 mg ethinyl estradiol and 0.25 mg norgestimate) in the presence of a single dose of tirzepatide 5 mg, mean C_{max} of ethinyl estradiol, norgestimate, and norelgestromin was reduced by 59%,66%, and 55%, while mean AUC was reduced by 20%, 21%, and 23%, respectively. A delay in t_{max} of 2.5 to 4.5 hours was observed.

12.6 Immunogenicity

The observed incidence of anti-drug antibodies is highly dependent on the sensitivity and specificity of the assay. Differences in assay methods preclude meaningful comparisons of the incidence of anti-drug antibodies in the trials described below with the incidence of anti-drug antibodies in other trials.

During the 40- to 104-week treatment periods with ADA sampling conducted up to 44 to 108 weeks in seven clinical trials in adults with type 2 diabetes mellitus *[see Clinical Studies (14)],* 51% (2,570/5,025) of MOUNJARO-treated patients developed anti-tirzepatide antibodies. In these trials, anti-tirzepatide antibody formation in 34% and 14% of MOUNJARO-treated adult patients showed cross-reactivity to native GIP or native GLP-1, respectively.

Of the 2,570 MOUNJARO-treated patients who developed anti-tirzepatide antibodies during the treatment periods in these seven trials, 2% and 2% developed neutralizing antibodies against tirzepatide activity on the GIP or GLP-1 receptors, respectively, and 0.9% and 0.4% developed neutralizing antibodies against native GIP or GLP-1, respectively.

During the 30-week double-blind placebo-controlled period of the glycemic control trial in pediatric patients 10 years of age or older with type 2 diabetes mellitus *[see Clinical Studies (14.5)]*, 30/61 (49%) of MOUNJARO-treated pediatric patients developed anti-tirzepatide antibodies. Anti-tirzepatide antibodies showed cross reactivity to native GIP or native GLP-1 in 26% and 8% of MOUNJARO-treated pediatric patients, respectively. There were no neutralizing antibodies against tirzepatide activity on the GIP or GLP-1 receptors. No pediatric patients developed neutralizing antibodies against native GIP or GLP-1.

There was no identified clinically significant effect of anti-tirzepatide antibodies on pharmacokinetics or effectiveness of MOUNJARO. More MOUNJARO-treated adult and pediatric patients who developed anti-tirzepatide antibodies experienced hypersensitivity reactions or injection site reactions than those who did not develop these antibodies *[see*

Adverse Reactions (6.1)].

13 NONCLINICAL TOXICOLOGY

13.1 Carcinogenesis, Mutagenesis, Impairment of Fertility

A 2-year carcinogenicity study was conducted with tirzepatide in male and female rats at doses of 0.15, 0.50, and 1.5 mg/kg (0.1-, 0.4-, and 1-fold the MRHD of 15 mg once weekly based on AUC) administered by subcutaneous injection twice weekly. A statistically significant increase in thyroid C-cell adenomas was observed in males (≥0.5 mg/kg) and females (≥0.15 mg/kg), and a statistically significant increase in thyroid C-cell adenomas and carcinomas combined was observed in males and females at all doses examined. In a 6-month carcinogenicity study in rasH2 transgenic mice, tirzepatide at doses of 1, 3, and 10 mg/kg administered by subcutaneous injection twice weekly was not tumorigenic.

Tirzepatide was not genotoxic in a rat bone marrow micronucleus assay.

In fertility and early embryonic development studies, male and female rats were administered twice weekly subcutaneous doses of 0.5, 1.5, or 3 mg/kg (0.3-, 1-, and 2-fold and 0.3-, 0.9-, and 2-fold, respectively, the MRHD of 15 mg once weekly based on AUC). No effects of tirzepatide were observed on sperm morphology, mating, fertility, and conception. In female rats, an increase in the number of females with prolonged diestrus and a decrease in the mean number of corpora lutea resulting in a decrease in the mean number of implantation sites and viable embryos was observed at all dose levels. These effects were considered secondary to the pharmacological effects of tirzepatide on food consumption and body weight.

14 CLINICAL STUDIES

14.1 Overview of Clinical Studies

The effectiveness of MOUNJARO as an adjunct to diet and exercise to improve glycemic control in adults with type 2 diabetes mellitus was established in five trials. In these trials, MOUNJARO was studied as monotherapy (SURPASS-1); as an add-on to metformin, sulfonylureas, and/or sodium-glucose co-transporter 2 inhibitors (SGLT2 inhibitors) (SURPASS-2, -3, and -4); and in combination with basal insulin with or without metformin (SURPASS-5). In these trials, MOUNJARO (5 mg, 10 mg, and 15 mg given subcutaneously once weekly) was compared with placebo, semaglutide 1 mg, insulin degludec, and/or insulin glargine *[see Clinical Studies (14.2, 14.3, 14.4)]*.

In adult patients with type 2 diabetes mellitus, treatment with MOUNJARO produced a statistically significant reduction from baseline in HbA1c compared to placebo. The effectiveness of MOUNJARO was not impacted by age, gender, race, ethnicity, region, or by baseline BMI, HbA1c, diabetes duration, or renal function.

MOUNJARO 5 mg and 10 mg was studied in pediatric patients 10 years of age and older with type 2 diabetes in combination with metformin and/or basal insulin *[see Clinical Studies (14.5)]*.

14.2 Monotherapy Use of MOUNJARO in Adult Patients with Type 2 Diabetes

Mellitus

SURPASS-1 (NCT03954834) was a 40-week double-blind trial that randomized 478 adult patients with type 2 diabetes mellitus with inadequate glycemic control with diet and exercise to subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, MOUNJARO 15 mg, or placebo once weekly.

Patients had a mean age of 54 years, and 52% were men. The mean duration of type 2 diabetes mellitus was 4.7 years, and the mean BMI was 32 kg/m^2. Overall, 36% were White, 35% were Asian, 25% were American Indians/Alaska Natives, and 5% were Black or African American; 43% identified as Hispanic or Latino ethnicity.

Monotherapy with MOUNJARO 5 mg, 10 mg and 15 mg once weekly for 40 weeks resulted in a statistically significant reduction in HbA1c compared with placebo (see Table 4).

Table 4: Results at Week 40 in a Trial of MOUNJARO as Monotherapy in Adult Patients with Type 2 Diabetes Mellitus with Inadequate Glycemic Control with Diet and Exercise

	Placebo	MOUNJARO 5 mg	MOUNJARO 10 mg	MOUNJARO 15 mg
Modified Intent-to-Treat (mITT) Population (N)[a]	113	121	121	120
HbA1c (%)				
Baseline (mean)	8.1	8.0	7.9	7.9
Change at Week 40[b]	-0.1	-1.8	-1.7	-1.7
Difference from placebo[b] (95% CI)	--	-1.7[c] (-2.0, -1.4)	-1.6[c] (-1.9, -1.3)	-1.6[c] (-1.9, -1.3)
Patients (%) achieving HbA1c <7%[d]	23	82[c]	85[c]	78[c]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	155	154	153	154
Change at Week 40[b]	4	-40	-40	-39
Difference from placebo[b] (95% CI)	--	-43[c] (-55, -32)	-43[c] (-55, -32)	-42[c] (-54, -30)
Body Weight (kg)				
Baseline (mean)	84.5	87.0	86.2	85.5
Change at Week 40[b]	-1.0	-6.3	-7.0	-7.8
Difference from placebo[b] (95% CI)	--	-5.3[c] (-6.8, -3.9)	-6.0[c] (-7.4, -4.6)	-6.8[c] (-8.3, -5.4)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. Patients who discontinued study treatment because they did not meet study enrollment criteria were excluded. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 25%, 2%, 3%, and 2% of patients randomized to placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. At Week 40 the HbA1c data were missing for 12%, 6%, 7%, and 14% of patients randomized to placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. Missing Week 40 data were imputed using placebo-based multiple imputation.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

[c] p<0.001 (two-sided) for superiority versus placebo, adjusted for multiplicity.
[d] Analyzed using logistic regression adjusted for baseline value and other stratification factors.

14.3 MOUNJARO Use in Combination with Metformin, Sulfonylureas, and/or SGLT2 Inhibitors in Adult Patients with Type 2 Diabetes Mellitus

Add-on to metformin

SURPASS-2 (NCT03987919) was a 40-week open-label trial (double-blind with respect to MOUNJARO dose assignment) that randomized 1,879 adult patients with type 2 diabetes mellitus with inadequate glycemic control on stable doses of metformin alone to the addition of subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, or MOUNJARO 15 mg once weekly or subcutaneous semaglutide 1 mg once weekly.

Patients had a mean age of 57 years and 47% were men. The mean duration of type 2 diabetes mellitus was 8.6 years, and the mean BMI was 34 kg/m^2. Overall, 83% were White, 4% were Black or African American, and 1% were Asian; 70% identified as Hispanic or Latino ethnicity.

Treatment with MOUNJARO 10 mg and 15 mg once weekly for 40 weeks resulted in a statistically significant reduction in HbA1c compared with semaglutide 1 mg once weekly (see Table 5 and Figure 2).

Table 5: Results at Week 40 in a Trial of MOUNJARO versus Semaglutide 1 mg in Adult Patients with Type 2 Diabetes Mellitus Added to Metformin

	Semaglutide 1 mg	MOUNJARO 5 mg	MOUNJARO 10 mg	MOUNJARO 15 mg
Modified Intent-to-Treat (mITT) Population (N)[a]	468	470	469	469
HbA1c (%)				
Baseline (mean)	8.3	8.3	8.3	8.3
Change at Week 40[b]	-1.9	-2.0	-2.2	-2.3
Difference from semaglutide[b] (95% CI)	--	-0.2[c] (-0.3, -0.0)	-0.4[d] (-0.5, -0.3)	-0.5[d] (-0.6, -0.3)
Patients (%) achieving HbA1c <7%[e]	79	82	86[f]	86[f]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	171	174	174	172
Change at Week 40[b]	-49	-55	-59	-60
Body Weight (kg)				
Baseline (mean)	93.7	92.5	94.8	93.8
Change at Week 40[b]	-5.7	-7.6	-9.3	-11.2
Difference from semaglutide[b] (95% CI)	--	-1.9[c] (-2.8, -1.0)	-3.6[d] (-4.5, -2.7)	-5.5[d] (-6.4, -4.6)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. Patients who discontinued study treatment because they did not meet study enrollment criteria were excluded. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 3%, 2%, 1%, and 1% of patients randomized to semaglutide 1 mg, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. At Week 40 the HbA1c endpoint was missing for 5%, 4%, 5%, and 5% of patients randomized to semaglutide

1 mg, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. Missing Week 40 data were imputed using multiple imputation with retrieved dropout.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

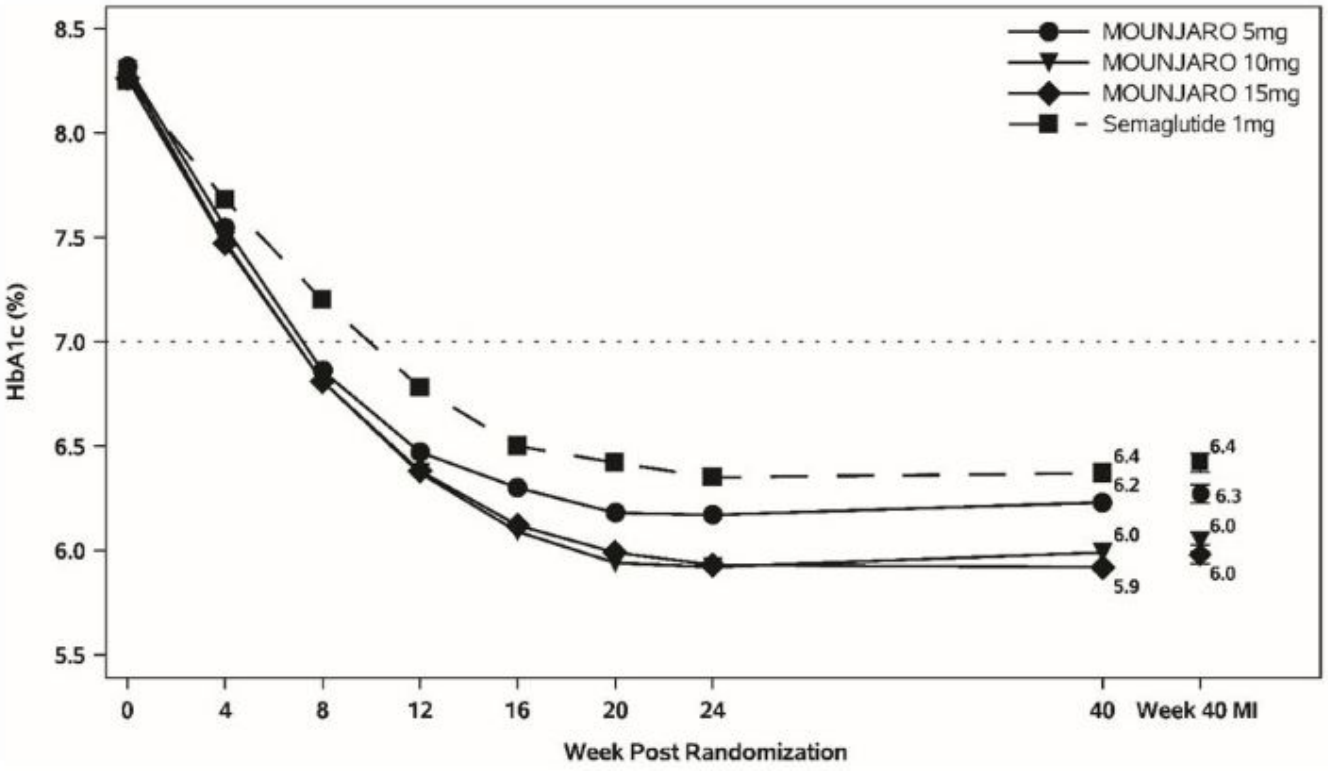
[c] p<0.05 (two-sided) for superiority versus semaglutide, adjusted for multiplicity.

[d] p<0.001 (two-sided) for superiority versus semaglutide, adjusted for multiplicity.

[e] Analyzed using logistic regression adjusted for baseline value and other stratification factors.

[f] p<0.01 (two-sided) for superiority versus semaglutide, adjusted for multiplicity.

Figure 2: Mean HbA1c (%) Over Time - Baseline to Week 40



Number of patients			
MOUNJARO 5mg	470	451	470
MOUNJARO 10mg	469	445	469
MOUNJARO 15mg	469	447	469
Semaglutide 1mg	468	443	468

Note: Displayed results are from modified Intent-to-Treat Full Analysis Set. (1) Observed mean value from Week 0 to Week 40, and (2) least-squares mean ± standard error at Week 40 multiple imputation (MI).

Add-on to metformin with or without SGLT2 inhibitor

SURPASS-3 (NCT03882970) was a 52-week open-label trial that randomized 1,444 adult patients with type 2 diabetes mellitus with inadequate glycemic control on stable doses of metformin with or without SGLT2 inhibitor to the addition of subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, MOUNJARO 15 mg once weekly, or insulin degludec 100 units/mL once daily. In this trial, 32% of patients were on SGLT2 inhibitor.

Insulin degludec was initiated at 10 units once daily and adjusted weekly throughout the trial using a treat-to-target algorithm based on self-measured fasting blood glucose values. At Week 52, 26% of patients randomized to insulin degludec achieved the fasting serum glucose target of <90 mg/dL, and the mean daily insulin degludec dose was 49 U (0.5 U per kilogram).

Patients had a mean age of 57 years, and 56% were men. The mean duration of type 2 diabetes mellitus was 8.4 years, and the mean baseline BMI was 34 kg/m^2. Overall, 91% were White, 3% were Black or African American, and 5% were Asian; 29% identified as Hispanic or Latino ethnicity.

Treatment with MOUNJARO 10 mg and 15 mg once weekly for 52 weeks resulted in a statistically significant reduction in HbA1c compared with daily insulin degludec (see Table 6).

Table 6: Results at Week 52 in a Trial of MOUNJARO versus Insulin Degludec in Adult Patients with Type 2 Diabetes Mellitus Added to Metformin with or without SGLT2 Inhibitor

	Insulin Degludec	**MOUNJARO 5 mg**	**MOUNJARO 10 mg**	**MOUNJARO 15 mg**
Modified Intent-to-Treat (mITT)[a] Population (N)	359	358	360	358
HbA1c (%)				
Baseline (mean)	8.1	8.2	8.2	8.2
Change at Week 52[b]	-1.3	-1.9	-2.0	-2.1
Difference from insulin degludec[b] (95% CI)	--	-0.6[c] (-0.7, -0.5)	-0.8[c] (-0.9, -0.6)	-0.9[c] (-1.0, -0.7)
Patients (%) achieving HbA1c <7%[d]	58	79[c]	82[c]	83[c]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	167	172	170	168
Change at Week 52[b]	-51	-47	-50	-54
Body Weight (kg)				
Baseline (mean)	94.0	94.4	93.8	94.9
Change at Week 52[b]	1.9	-7.0	-9.6	-11.3
Difference from insulin degludec[b] (95% CI)	--	-8.9[c] (-10.0, -7.8)	-11.5[c] (-12.6, -10.4)	-13.2[c] (-14.3, -12.1)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. Patients who discontinued study treatment because they did not meet study enrollment criteria were excluded. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 1%,1%, 1%, and 2% of patients randomized to insulin degludec, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. At Week 52 the HbA1c endpoint was missing for 9%, 6%, 10%, and 5% of patients randomized to insulin degludec, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. Missing Week 52 data were imputed using multiple imputation with retrieved dropout.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

[c] $p<0.001$ (two-sided) for superiority versus insulin degludec, adjusted for multiplicity.

[d] Analyzed using logistic regression adjusted for baseline value and other stratification factors.

Add-on to 1-3 oral anti-hyperglycemic agents (metformin, sulfonylurea, or SGLT-2 inhibitor)

SURPASS-4 (NCT03730662) was a 104-week open-label trial (52-week primary endpoint) that randomized 2002 adult patients with type 2 diabetes mellitus with increased CV risk to subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, MOUNJARO 15 mg once weekly, or insulin glargine 100 units/mL once daily (1:1:1:3 ratio) on a background of metformin (95%) and/or sulfonylureas (54%) and/or SGLT2 inhibitors (25%).

Patients had a mean age of 64 years, and 63% were men. The mean duration of type 2 diabetes mellitus was 11.8 years, and the mean baseline BMI was 33 kg/m^2. Overall, 82% were White, 4% were Black or African American, and 4% were Asian; 48% identified as Hispanic or Latino ethnicity. Across all treatment groups, 87% had a history of CV disease. At baseline, eGFR was ≥90 mL/min/1.73 m^2 in 43%, 60 to 90 mL/min/1.73 m^2 in 40%, 45 to 60 mL/min/1.73 m^2 in 10%, and 30 to 45 mL/min/1.73 m^2 in 6% of patients.

Insulin glargine was initiated at 10 U once daily and adjusted weekly throughout the trial using a treat-to-target algorithm based on self-measured fasting blood glucose values. At Week 52, 30% of patients randomized to insulin glargine achieved the fasting serum glucose target of <100 mg/dL, and the mean daily insulin glargine dose was 44 U (0.5 U per kilogram).

Treatment with MOUNJARO 10 mg and 15 mg once weekly for 52 weeks resulted in a statistically significant reduction in HbA1c compared with insulin glargine once daily (see Table 7).

Table 7: Results at Week 52 in a Trial of MOUNJARO versus Insulin Glargine in Adult Patients with Type 2 Diabetes Mellitus Added to Metformin and/or Sulfonylurea and/or SGLT2 Inhibitor

	Insulin Glargine	**MOUNJARO 5 mg**	**MOUNJARO 10 mg**	**MOUNJARO 15 mg**
Modified Intent-to-Treat (mITT) Population (N)[a]	998	328	326	337
HbA1c (%)				
Baseline (mean)	8.5	8.5	8.6	8.5
Change at Week 52[b]	-1.4	-2.1	-2.3	-2.4
Difference from insulin glargine[b] (95% CI)	--	-0.7[c] (-0.9, -0.6)	-0.9[c] (-1.1, -0.8)	-1.0[c] (-1.2, -0.9)
Patients (%) achieving HbA1c <7%[d]	49	75[c]	83[c]	85[c]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	168	172	176	174
Change at Week 52[b]	-49	-44	-50	-55
Body Weight (kg)				
Baseline (mean)	90.2	90.3	90.6	90.0
Change at Week 52[b]	1.7	-6.4	-8.9	-10.6
Difference from insulin glargine[b] (95% CI)	--	-8.1[c] (-8.9, -7.3)	-10.6[c] (-11.4, -9.8)	-12.2[c] (-13.0, -11.5)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. Patients who discontinued study treatment because

they did not meet study enrollment criteria were excluded. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 1%, 0%, 0%, and 1% of patients randomized to insulin glargine, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. At Week 52 the HbA1c endpoint was missing for 9%, 9%, 6%, and 4% of patients randomized to insulin glargine, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. Missing Week 52 data were imputed using multiple imputation with retrieved dropout.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

[c] p<0.001 (two-sided) for superiority versus insulin glargine, adjusted for multiplicity.

[d] Analyzed using logistic regression adjusted for baseline value and other stratification factors.

14.4 MOUNJARO Use in Combination with Basal Insulin with or without Metformin in Adult Patients with Type 2 Diabetes Mellitus

SURPASS-5 (NCT04039503) was a 40-week double-blind trial that randomized 475 adult patients with type 2 diabetes mellitus with inadequate glycemic control on insulin glargine 100 units/mL, with or without metformin, to subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, MOUNJARO 15 mg once weekly, or placebo. The dose of background insulin glargine was adjusted using a treat-to-target algorithm based on self-measured fasting blood glucose values, targeting <100 mg/dL.

Patients had a mean age of 61 years, and 56% were men. The mean duration of type 2 diabetes mellitus was 13.3 years, and the mean baseline BMI was 33 kg/m^2. Overall, 80% were White, 1% were Black or African American, and 18% were Asian; 5% identified as Hispanic or Latino ethnicity.

The mean dose of insulin glargine at baseline was 34, 32, 35, and 33 units/day for patients receiving MOUNJARO 5 mg, 10 mg, 15 mg, and placebo, respectively. At randomization, the initial insulin glargine dose in patients with HbA1c ≤8.0% was reduced by 20%. At week 40, mean dose of insulin glargine was 38, 36, 29, and 59 units/day for patients receiving MOUNJARO 5 mg, 10 mg, 15 mg, and placebo, respectively.

Treatment with MOUNJARO 5 mg once weekly, 10 mg once weekly and 15 mg once weekly for 40 weeks resulted in a statistically significant reduction in HbA1c compared with placebo (see Table 8).

Table 8: Results at Week 40 in a Trial of MOUNJARO Added to Basal Insulin with or without Metformin in Adult Patients with Type 2 Diabetes Mellitus

	Placebo	MOUNJARO 5 mg	MOUNJARO 10 mg	MOUNJARO 15 mg
Modified Intent-to-Treat (mITT) Population (N)[a]	119	116	118	118
HbA1c (%)				
Baseline (mean)	8.4	8.3	8.4	8.2
Change at Week 40[b]	-0.9	-2.1	-2.4	-2.3
Difference from placebo[b] (95% CI)	--	-1.2[c] (-1.5, -1.0)	-1.5[c] (-1.8, -1.3)	-1.5[c] (-1.7, -1.2)
Patients (%) achieving HbA1c <7%[d]	35	87[c]	90[c]	85[c]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	164	163	163	160
Change at Week 40[b]	-39	-58	-64	-63

Difference from placebo[b] (95% CI)	--	-19[c] (-27, -11)	-25[c] (-32, -17)	-23[c] (-31, -16)
Body Weight (kg)				
Baseline (mean)	94.2	95.8	94.6	96.0
Change at Week 40[b]	1.6	-5.4	-7.5	-8.8
Difference from placebo[b] (95% CI)	--	-7.1[c] (-8.7, -5.4)	-9.1[c] (-10.7, -7.5)	-10.5[c] (-12.1, -8.8)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. Patients who discontinued study treatment because they did not meet study enrollment criteria were excluded. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 4%, 1%, 0%, and 1% of patients randomized to placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. At Week 40 the HbA1c endpoint was missing for 2%, 6%, 3%, and 7% of patients randomized to placebo, MOUNJARO 5 mg, 10 mg, and 15 mg, respectively. Missing Week 40 data were imputed using placebo-based multiple imputation.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

[c] $p<0.001$ (two-sided) for superiority versus placebo, adjusted for multiplicity.

[d] Analyzed using logistic regression adjusted for baseline value and other stratification factors.

14.5 MOUNJARO Use in Combination with Metformin or Basal Insulin, or Both in Pediatric Patients 10 Years of Age and Older with Type 2 Diabetes Mellitus

SURPASS-PEDS (NCT05260021) was a 30-week double-blind, placebo-controlled trial with a 22-week open-label extension that randomized 99 pediatric patients 10 years of age and older with type 2 diabetes mellitus with inadequate glycemic control on metformin (69%), or basal insulin (8%), or both (23%) to receive subcutaneous MOUNJARO 5 mg, MOUNJARO 10 mg, or placebo once weekly as add-on therapy.

Patients had a mean age of 15 years, and 61% were female. The mean duration of type 2 diabetes mellitus was 2.4 years, mean HbA1c was 8.0%, mean weight was 97 kg, and the mean baseline BMI was 35 kg/m^2. Overall, 58% were White, 11% were Black or African American, 6% were Asian, 20% were American Indian or Alaska Native, and 5% were other races; 66% identified as Hispanic or Latino ethnicity.

Treatment with MOUNJARO 5 mg once weekly and 10 mg once weekly for 30 weeks, both pooled and individually, resulted in a statistically significant reduction in HbA1c compared with placebo (see Table 9).

Table 9: Results at Week 30 in a Trial of MOUNJARO Added to Metformin or Basal Insulin, or Both in Pediatric Patients 10 Years of Age and Older with Type 2 Diabetes Mellitus

	Placebo	MOUNJARO 5 mg	MOUNJARO 10 mg	MOUNJARO 5 mg/10 mg pooled
Modified Intent-to-Treat (mITT) Population (N)[a]	34	32	33	65
HbA1c (%)				
Baseline (mean)	8.0	8.2	7.9	8.1
Change at Week 30[b]	-0.2	-1.9	-2.2	-2.0
Difference from placebo[b] (95%		-1.7[c]	-2.0[c]	-1.8[c]

CI)	--	(-2.4, -1.0)	(-2.7, -1.3)	(-2.4, -1.2)
Patients (%) with HbA1c ≤6.5% at Week 30[d]	28	68[c]	81[c]	75[c]
Fasting Serum Glucose (mg/dL)				
Baseline (mean)	156[f]	148[f]	152[f]	150[f]
Change at Week 30[b]	-5	-35	-51	-43
Difference from placebo[b] (95% CI)	--	-30[e] (-53, -8)	-46[c] (-68, -24)	-38[c] (-57, -19)
BMI (kg/m^2)				
Baseline (mean)	34.7	33.9	37.7	35.8
Percent Change at Week 30[b]	-0.5	-6.9	-10.8	-8.8
Difference from placebo (%)[b] (95% CI)	--	-6.4[c] (-9.5, -3.2)	-10.3[c] (-13.5, -7.1)	-8.3[c] (-11.0, -5.6)

[a] The modified intent-to-treat population consists of all randomly assigned participants who were exposed to at least 1 dose of study drug. During the trial, rescue medication (additional antihyperglycemic medication) was initiated by 6%, 0%, and 0% of patients randomized to placebo, MOUNJARO 5 mg, and 10 mg, respectively. At Week 30 the HbA1c endpoint was missing for 6%, 9%, and 18% of patients randomized to placebo, MOUNJARO 5 mg, and 10 mg, respectively. Missing Week 30 data were imputed using multiple imputation with placebo wash-out or with assumption of missing at random.

[b] Least-squares mean from ANCOVA adjusted for baseline value and other stratification factors.

[c] $p<0.001$ (two-sided) for superiority versus placebo, adjusted for multiplicity.

[d] Response in (%) is calculated by combining proportion of participants achieving target in imputed datasets using Rubin's rule.

[e] $p<0.01$ (two-sided) for superiority versus placebo, adjusted for multiplicity.

[f] Any missing values at baseline were imputed as missing at random.

14.6 Cardiovascular Outcomes Trial of MOUNJARO in Adults with Type 2 Diabetes Mellitus and Established Cardiovascular Disease

SURPASS-CVOT (NCT04255433) was a Phase 3, event-driven, multicenter, international, randomized, double-blind, active comparator controlled, parallel-group study. In this trial, 13,299 patients with inadequately controlled type 2 diabetes mellitus and established CV disease were randomized to once weekly subcutaneous MOUNJARO (up to 15 mg) versus once weekly subcutaneous dulaglutide (1.5 mg), a GLP-1 receptor agonist indicated for glycemic control and to reduce the risk of major adverse cardiovascular events (MACE) in adults with type 2 diabetes mellitus who have established CV disease or multiple CV risk factors. MOUNJARO was initiated at a dosage of 2.5 mg once weekly and increased by 2.5 mg every 4 weeks to a maximum dosage of 15 mg once weekly or the maximum tolerated dose. The dose of dulaglutide was initiated and maintained at 1.5 mg once weekly. The primary endpoint was the time to first occurrence of a three-part composite MACE outcome of CV death, MI, or stroke (MACE-3).

Patients eligible to enroll in the trial had type 2 diabetes mellitus and established CV disease, were 40 years of age or older, had a HbA1c of ≥7.0% and ≤10.5%, and a BMI ≥ 25 kg/m^2. The mean age was 64 years, the mean duration of type 2 diabetes mellitus was 15 years, the mean HbA1c concentration was 8.4%, and the mean BMI was 33 kg/m^2. Overall, 71% of patients enrolled were male; 82% were White, 2% were Black or African American, and 9% were Asian; 30% identified Hispanic or Latino ethnicity.

Patients reported the following types of CV disease at baseline: coronary artery disease

(65%); prior coronary revascularization (57%); prior MI (47%); peripheral arterial disease (25%); prior lower extremity arterial revascularization (6%); prior stroke (19%); prior carotid revascularization (3%); and prior heart failure (20%). Of patients enrolled in the study, 20% had diabetic neuropathy, 2% had proliferative diabetic retinopathy, and 29% had non-proliferative diabetic retinopathy. The percentage of patients with an eGFR < 60 mL/min/1.73 m^2 was 23%.

The most common antihyperglycemic medications taken at baseline were: metformin (81%), sodium-glucose cotransporter-2 (SGLT-2) inhibitors (31%), sulfonylureas (22%), and insulins (49%). At baseline CV disease and CV risk factors were managed with: statins (86%), antiplatelet therapies (83%), beta blockers (65%), angiotensin-converting enzyme (ACE) inhibitors (40%), and angiotensin receptor blockers (40%). During the trial, investigators were to modify anti-hyperglycemic and CV medications to achieve local standard of care treatment targets with respect to blood glucose, lipids, and blood pressure.

Over a median follow-up of 210.1 weeks, MOUNJARO was non-inferior to dulaglutide for reducing the occurrence of MACE-3. The estimated hazard ratio for time to first MACE was 0.92 (95.3% CI: 0.83, 1.01) for MOUNJARO compared to dulaglutide. Superiority to dulaglutide was not established. In this trial, 98% of participants had a complete assessment for the primary endpoint and 99.7% had vital status established. Refer to Figure 3 and Table 10.

Figure 3: Kaplan-Meier Curve: Time to First Occurrence of a MACE-3 (CV death, non-fatal MI, or non-fatal stroke) in the SURPASS-CVOT Trial

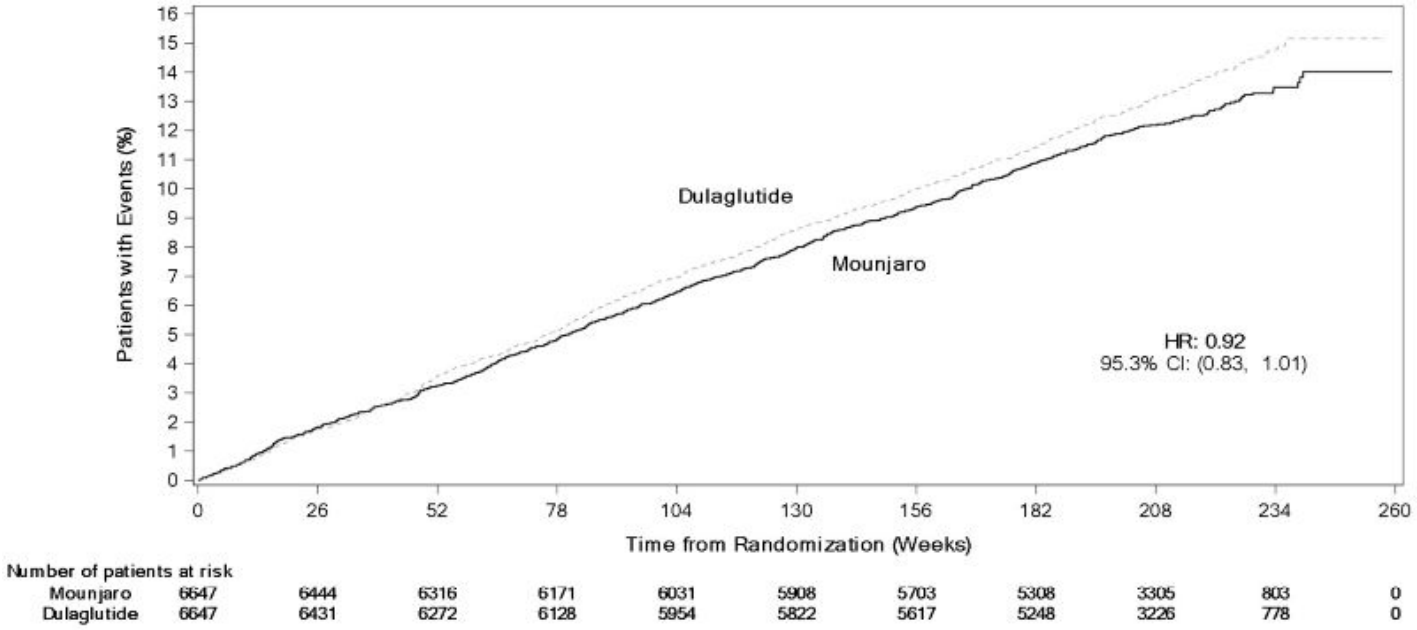


Table 10: Treatment Effect for MACE and All Cause Death, All Randomized and Treated Participants, Median Study Observation Time 210.1 weeks

	Dulaglutide N=6,647 (%)	MOUNJARO N=6,647 (%)	Hazard ratio vs Dulaglutide (95% CI)

MACE-3 (Composite of time to first occurrence of CV death, non-fatal MI, or non-fatal stroke)[a,b,c]	863 (13.0)	803 (12.1)	0.92 (0.83, 1.01)
CV Death[d]	409 (6.2)	367 (5.5)	0.89 (0.77, 1.02)
Non-Fatal MI[d]	310 (4.7)	292 (4.4)	0.93 (0.80, 1.09)
Non-Fatal Stroke[d]	221 (3.3)	215 (3.2)	0.97 (0.80, 1.16)
Fatal or Non-Fatal MI[d]	358 (5.4)	313 (4.7)	0.87 (0.74, 1.01)
Fatal or Non-Fatal Stroke[d]	249 (3.7)	229 (3.4)	0.91 (0.76, 1.09)
All-Cause Death[d]	670 (10.1)	567 (8.5)	0.84 (0.75, 0.94)

[a] 95.3% confidence interval for the MACE-3 composite endpoint of CV death, non-fatal MI, or non-fatal stroke

[b] Cox-proportional hazards models with treatment as a factor and stratified for SGLT2i use.

[c] p=0.007 for noninferiority of MOUNJARO to dulaglutide

[d] Not controlled for family-wise type I error rate

Reductions from baseline in HbA1c and body weight were observed with MOUNJARO and dulaglutide treatment at Month 36.

16 HOW SUPPLIED/STORAGE AND HANDLING

16.1 How Supplied

MOUNJARO is a clear, colorless to slightly yellow solution available in cartons containing 4 pre-filled single-dose pens, 1 single-dose vial, 1 multi-dose vial, or 1 Single-Patient-Use KwikPen as follows:

Single-Dose Vial and Prefilled Pen		
Strength	**4-pack Single-dose Pen NDC**	**1-pack Single-dose Vial NDC**
2.5 mg/0.5 mL	0002-1506-80	0002-1152-01
5 mg/0.5 mL	0002-1495-80	0002-1243-01
7.5 mg/0.5 mL	0002-1484-80	0002-2214-01
10 mg/0.5 mL	0002-1471-80	0002-2340-01
12.5 mg/0.5 mL	0002-1460-80	0002-2423-01
15 mg/0.5 mL	0002-1457-80	0002-3002-01

Multi-Dose Vial		
Doses per Vial	**Strength**	**1-pack Multi-dose Vial NDC**
4 doses of 2.5 mg/0.6 mL	10 mg/2.4 mL (4.17 mg/mL)	0002-4052-11
4 doses of 5 mg/0.6 mL	20 mg/2.4 mL (8.33 mg/mL)	0002-4103-11
4 doses of 7.5 mg/0.6 mL	30 mg/2.4 mL (12.5 mg/mL)	0002-4210-11
4 doses of 10 mg/0.6 mL	40 mg/2.4 mL (16.7 mg/mL)	0002-4304-11
4 doses of 12.5 mg/0.6 mL	50 mg/2.4 mL (20.8 mg/mL)	0002-4523-11
4 doses of 15 mg/0.6 mL	60 mg/2.4 mL (25 mg/mL)	0002-4612-11

Single-Patient-Use KwikPen (with four weekly doses)		
Doses per KwikPen	**Strength**	**1-pack Single-Patient-Use KwikPen NDC**
4 doses of 2.5 mg	10 mg/2.4 mL (4.17 mg/mL)	0002-3466-11
4 doses of 5 mg	20 mg/2.4 mL (8.33 mg/mL)	0002-3455-11
4 doses of 7.5 mg	30 mg/2.4 mL (12.5 mg/mL)	0002-3444-11
4 doses of 10 mg	40 mg/2.4 mL (16.7 mg/mL)	0002-3433-11
4 doses of 12.5 mg	50 mg/2.4 mL (20.8 mg/mL)	0002-3422-11
4 doses of 15 mg	60 mg/2.4 mL (25 mg/mL)	0002-3411-11

16.2 Storage and Handling

- Do not freeze MOUNJARO. Do not use MOUNJARO if frozen.
- Protect MOUNJARO from heat and light.
- Store MOUNJARO in the original carton to protect from light.

MOUNJARO Single-dose Pen and Single-dose Vial

- Store MOUNJARO single-dose pen and single-dose vial in a refrigerator at 2°C to 8°C (36°F to 46°F).
- If needed, each single-dose pen or single-dose vial can be stored unrefrigerated at temperatures not to exceed 30°C (86°F) for up to a total of 21 days.
- Discard the single-dose pen or single-dose vial after a total of 21 days at room temperature.

MOUNJARO Multi-dose Vial or Single-Patient-Use KwikPen

Unopened vial or single-patient-use KwikPen:

- Store unopened multi-dose vial or single-patient-use KwikPen in the refrigerator at 2°C to 8°C (36°F to 46°F). The unopened multi-dose vial or single-patient-use KwikPen can be used until the expiration date on the label if kept in the refrigerator.
- If stored at room temperature [up to 30°C (86°F)], throw away unopened multi-dose vial or single-patient-use KwikPen after 30 days.

After vial or single-patient-use KwikPen has been opened:

- Store opened (in-use) multi-dose vial or single-patient-use KwikPen in the original carton in the refrigerator at 2°C to 8°C (36°F to 46°F) or at room temperature [up to 30°C (86°F)].
- Throw away opened multi-dose vial or single-patient-use KwikPen after a total of 30 days at room temperature, 30 days after first use, or after taking 4 weekly doses, even if there is medicine left in it.

17 PATIENT COUNSELING INFORMATION

Advise the patient to read the FDA-approved patient labeling (*Medication Guide and Instructions for Use*).

Risk of Thyroid C-Cell Tumors

Inform patients that MOUNJARO causes thyroid C-cell tumors in rats and that the human relevance of this finding has not been determined. Counsel patients to report symptoms of thyroid tumors (e.g., a lump in the neck, persistent hoarseness,

dysphagia, or dyspnea) to their healthcare provider *[see Boxed Warning and Warnings and Precautions (5.1)]*.

Acute Pancreatitis

Inform patients of the potential risk for acute pancreatitis and its symptoms: severe abdominal pain that sometimes radiates to the back, and which may or may not be accompanied by nausea or vomiting. Instruct patients to discontinue MOUNJARO promptly and contact their physician if pancreatitis is suspected *[see Warnings and Precautions (5.2)]*.

Hypoglycemia with Concomitant Use of Insulin Secretagogues or Insulin

Inform patients that the risk of hypoglycemia is increased when MOUNJARO is used with an insulin secretagogue (such as a sulfonylurea) or insulin. Educate patients on the signs and symptoms of hypoglycemia *[see Warnings and Precautions (5.3)]*.

Hypersensitivity Reactions

Inform patients that serious hypersensitivity reactions have been reported with use of MOUNJARO. Advise patients on the symptoms of hypersensitivity reactions and instruct them to stop taking MOUNJARO and seek medical advice promptly if such symptoms occur *[see Warnings and Precautions (5.4)]*.

Acute Kidney Injury Due to Volume Depletion

Inform patients of the potential risk of acute kidney injury due to dehydration associated with gastrointestinal adverse reactions. Advise patients to take precautions to avoid fluid depletion. Inform patients of the signs and symptoms of acute kidney injury and instruct them to promptly report any of these signs or symptoms or persistent (or extended) nausea, vomiting, and diarrhea to their healthcare provider *[see Warnings and Precautions (5.5)]*.

Severe Gastrointestinal Adverse Reactions

Inform patients of the potential risk of severe gastrointestinal adverse reactions. Instruct patients to contact their healthcare provider if they have severe or persistent gastrointestinal symptoms *[see Warnings and Precautions (5.6)].*

Diabetic Retinopathy Complications in Patients with a History of Diabetic Retinopathy

Inform patients to contact their healthcare provider if changes in vision are experienced during treatment with MOUNJARO *[see Warnings and Precautions (5.7)]*.

Acute Gallbladder Disease

Inform patients of the risk of acute gallbladder disease. Instruct patients to contact their healthcare provider for appropriate clinical follow-up if gallbladder disease is suspected *[see Warnings and Precautions (5.8)]*.

Pulmonary Aspiration During General Anesthesia or Deep Sedation

Inform patients that MOUNJARO may cause their stomach to empty more slowly which may lead to complications with anesthesia or deep sedation during planned surgeries or procedures. Instruct patients to inform healthcare providers prior to any planned surgeries or procedures if they are taking MOUNJARO *[see Warnings and Precautions (5.9)].*

Never Share a MOUNJARO KwikPen Between Patients

Advise patients that they must never share a MOUNJARO KwikPen with another person, even if the pen needle is changed, because doing so carries a risk for transmission of blood-borne pathogens *[see Warnings and Precautions (5.10)]*.

Pregnancy

Advise a pregnant woman of the potential risk to a fetus. Advise women to inform their healthcare provider if they are pregnant or intend to become pregnant *[see Use in Specific Populations (8.1)].*

Contraception

Use of MOUNJARO may reduce the efficacy of oral hormonal contraceptives. Advise patients using oral hormonal contraceptives to switch to a non-oral contraceptive method or add a barrier method of contraception for 4 weeks after initiation and for 4 weeks after each dose escalation with MOUNJARO *[see Drug Interactions (7.2), Use in Specific Populations (8.3), and Clinical Pharmacology (12.3)].*

Administration

Instruct patients how to prepare and administer the correct dose of MOUNJARO and assess their ability to inject subcutaneously to ensure the proper administration of MOUNJARO. Inform patients and their caregiver(s) which

MOUNJARO presentation (e.g., single-dose pen or vial, multi-dose vial, single-patient use KwikPen) they will receive and ensure they receive training appropriate for that specific presentation. If the prescribed MOUNJARO presentation changes, ensure patients and caregivers receive appropriate training and instruct them to consult the Instructions for Use for the newly prescribed presentation. Advise caregivers that MOUNJARO KwikPen is not recommended for self-administration by pediatric patients or those with visual impairment unless their healthcare provider determines the patient can properly administer. Instruct patients using MOUNJARO vials to always use a new syringe and needle for each injection and to use a syringe appropriate for dose administration (e.g., a 1 mL syringe capable of measuring a 0.5 mL or 0.6 mL dose) *[see Dosage and Administration (2.2)]*.

Missed Doses

Inform patients if a dose is missed, it should be administered as soon as possible within 4 days after the missed dose. If more than 4 days have passed, the missed dose should be skipped and the next dose should be administered on the regularly scheduled day. In each case, inform patients to resume their regular once weekly dosing schedule *[see Dosage and Administration (2.1)].*

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MOU-0012-USPI-20260827

Mounjaro Medication Guide

Medication Guide
MOUNJARO® [mown-JAHR-OH]
(tirzepatide)
injection, for subcutaneous use

Do not share your MOUNJARO KwikPen or needles with other people, even if the needle has been changed. You may give other people a serious infection, or get a serious infection from them.

What is the most important information I should know about MOUNJARO?
MOUNJARO may cause serious side effects, including:

- **Possible thyroid tumors, including cancer.** Tell your healthcare provider if you get a lump or swelling in your neck, hoarseness, trouble swallowing, or shortness of breath. These may be symptoms of thyroid cancer. In studies with rats, MOUNJARO and medicines that work like MOUNJARO caused thyroid tumors, including thyroid cancer. It is not known if MOUNJARO will cause thyroid tumors, or a type of thyroid cancer called medullary thyroid carcinoma (MTC) in people.
- Do not use MOUNJARO if you or any of your family have ever had a type of thyroid cancer called medullary thyroid carcinoma (MTC), or if you have an endocrine system condition called Multiple Endocrine Neoplasia syndrome type 2 (MEN 2).

What is MOUNJARO?

- MOUNJARO is an injectable prescription medicine that is used:
  - along with diet and exercise to improve blood sugar (glucose) in adults and children 10 years of age and older with type 2 diabetes mellitus.
  - to reduce the risk of major cardiovascular events such as heart attack, stroke, or death in adults with type 2 diabetes mellitus who are at high risk for these events.
- It is not known if MOUNJARO is safe and effective for use in children under 10 years of age.

Do not use MOUNJARO if:

- you or any of your family have ever had a type of thyroid cancer called MTC or if you have an endocrine system condition called MEN 2.
- you have had a serious allergic reaction to tirzepatide or any of the ingredients in MOUNJARO. See the end of this Medication Guide for a complete list of ingredients in MOUNJARO. See "**What are the possible side effects of MOUNJARO?**" for symptoms of a serious allergic reaction.

Before using MOUNJARO, tell your healthcare provider about all of your medical conditions, **including if you:**

- have or have had problems with your pancreas.
- have severe problems with your stomach, such as slowed emptying of your stomach (gastroparesis) or problems with digesting food.
- have a history of diabetic retinopathy.
- are scheduled to have surgery or other procedures that use anesthesia or deep sleepiness (deep sedation).
- are pregnant or plan to become pregnant. It is not known if MOUNJARO will harm your unborn baby. Tell your healthcare provider if you become pregnant while using MOUNJARO.
  - **Birth control pills by mouth may not work as well while using MOUNJARO**. If you take birth control pills by mouth, your healthcare provider

may recommend another type of birth control for 4 weeks after you start MOUNJARO and for 4 weeks after each increase in your dose of MOUNJARO. Talk to your healthcare provider about birth control methods that may be right for you while using MOUNJARO.

- are breastfeeding or plan to breastfeed. MOUNJARO may pass into your breast milk. Talk to your healthcare provider about the best way to feed your baby while using MOUNJARO.

Tell your healthcare provider about all the medicines you take, including prescription and over-the-counter medicines, vitamins, and herbal supplements. MOUNJARO may affect the way some medicines work, and some medicines may affect the way MOUNJARO works.

Before using MOUNJARO, talk to your healthcare provider about low blood sugar and how to manage it. Tell your healthcare provider if you are taking other medicines to treat diabetes including insulin or sulfonylureas.

Know the medicines you take. Keep a list of them to show your healthcare provider and pharmacist when you get a new medicine.

How should I use MOUNJARO?

- Read the **Instructions for Use** that comes with MOUNJARO.
- Use MOUNJARO exactly as your healthcare provider tells you to.
- **Your healthcare provider should show you how to prepare and inject your dose of MOUNJARO before injecting for the first time.**
- A caregiver may give you MOUNJARO injections or you may self-inject if a healthcare provider determines that it is appropriate.
- Children **should not** inject themselves with MOUNJARO KwikPen. An adult caregiver should give a child his or her injections with MOUNJARO KwikPen.
- Self-injection is not recommended for people who are visually impaired.
- If using MOUNJARO vials, always use a new syringe and needle for each injection.
- Inject MOUNJARO under the skin (subcutaneously) of your stomach (abdomen), thigh, or another person should inject in the back of the upper arm. **Do not** inject MOUNJARO into a muscle (intramuscularly) or vein (intravenously).
- **Use MOUNJARO 1 time each week, at any time of the day.**
- You may change the day of the week you use MOUNJARO as long as the time between the 2 doses is at least **3** days (72 hours).
- If you miss a dose of MOUNJARO, take the missed dose as soon as possible within 4 days (96 hours) after the missed dose. If more than 4 days have passed, skip the missed dose and take your next dose on the regularly scheduled day. **Do not** take **2** doses of MOUNJARO within **3** days of each other.
- MOUNJARO may be taken with or without food.
- **Do not** mix insulin and MOUNJARO together in the same injection.
- You may give an injection of MOUNJARO and insulin in the same body area (such as your stomach area), but not right next to each other.
- Change (rotate) your injection site with each weekly injection. **Do not** use the same site for each injection.
- If you take too much MOUNJARO, call your healthcare provider or Poison Help line at 1-800-222-1222 or go to the nearest hospital emergency room right away.

What are the possible side effects of MOUNJARO?

MOUNJARO may cause serious side effects, including:

- See **"What is the most important information I should know about MOUNJARO?"**
- **inflammation of your pancreas (pancreatitis).** Stop using MOUNJARO and call your healthcare provider right away if you have severe pain in your stomach area (abdomen) that will not go away, with or without nausea or vomiting. Sometimes you may feel the pain from your abdomen to your back.
- **low blood sugar (hypoglycemia).** Your risk for getting low blood sugar may be higher if you use MOUNJARO with another medicine that can cause low blood sugar, such as a sulfonylurea or insulin.
  Signs and symptoms of low blood sugar may include:

  - dizziness or light-headedness
  - sweating
  - confusion or drowsiness
  - headache
  - blurred vision
  - slurred speech
  - shakiness
  - fast heartbeat
  - anxiety, irritability, or mood changes
  - hunger
  - weakness
  - feeling jittery

- **serious allergic reactions.** Stop using MOUNJARO and get medical help right away if you have any symptoms of a serious allergic reaction including:

  - swelling of your face, lips, tongue, or throat
  - problems breathing or swallowing
  - severe rash or itching
  - fainting or feeling dizzy
  - very rapid heartbeat

- **dehydration leading to kidney problems.** Diarrhea, nausea, and vomiting may cause a loss of fluids (dehydration) which may cause kidney problems. It is important for you to drink fluids to help reduce your chance of dehydration. Tell your healthcare provider right away if you have nausea, vomiting, or diarrhea that does not go away.
- **severe stomach problems.** Stomach problems, sometimes severe, have been reported in people who use MOUNJARO. Tell your healthcare provider if you have stomach problems that are severe or will not go away.
- **changes in vision.** Tell your healthcare provider if you have changes in vision during treatment with MOUNJARO.
- **gallbladder problems.** Gallbladder problems have happened in some people who use MOUNJARO. Tell your healthcare provider right away if you get symptoms of gallbladder problems which may include:

  - pain in your upper stomach (abdomen)
  - fever
  - yellowing of skin or eyes (jaundice)
  - clay-colored stools

- **food or liquid getting into the lungs during surgery or other procedures that use anesthesia or deep sleepiness (deep sedation).** MOUNJARO may increase the chance of food getting into your lungs during surgery or other procedures. Tell all your healthcare providers that you are taking MOUNJARO before you are scheduled to have surgery or other procedures.

The most common side effects of MOUNJARO include:

- nausea
- diarrhea
- decreased appetite
- vomiting
- constipation
- indigestion
- stomach (abdominal) pain

Talk to your healthcare provider if you have any side effect that bothers you or that does not go away. These are not all the possible side effects of MOUNJARO. Call your doctor for medical advice about side effects. You may report side effects to FDA at 1-800-FDA-1088.

How should I store MOUNJARO?

- Store MOUNJARO in the original carton to protect it from light.
- Do not freeze MOUNJARO. Do not use MOUNJARO if frozen.
- **Store each single-dose pen or single-dose vial** in the refrigerator between 36°F to 46°F (2°C to 8°C). If needed, each single-dose pen or single-dose vial can be stored at room temperature up to 86°F (30°C) for up to a total of 21 days. Throw away single-dose pen or single-dose vial after a total of 21 days at room temperature.
- **Store multi-dose vial or single-patient-use KwikPen** in the refrigerator between 36°F to 46°F (2°C to 8°C) or at room temperature [up to 86°F (30°C)]. Throw away the multi-dose vial or single-patient-use KwikPen after a total of 30 days at room temperature, 30 days after first use, or after taking 4 weekly doses, even if it still has medicine in it.

Keep MOUNJARO and all medicines out of the reach of children.

General information about the safe and effective use of MOUNJARO.
Medicines are sometimes prescribed for purposes other than those listed in a Medication Guide. Do not use MOUNJARO for a condition for which it was not prescribed. Do not give MOUNJARO to other people, even if they have the same symptoms you have. It may harm them. You can ask your pharmacist or healthcare provider for information about MOUNJARO that is written for health professionals.

What are the ingredients in MOUNJARO?
Active ingredient: tirzepatide
Inactive ingredients: sodium chloride, sodium phosphate dibasic heptahydrate, and water for injection. Benzyl alcohol, glycerin, and phenol are also inactive ingredients in the multi-dose vial and single-patient-use KwikPen. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH.
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For more information, go to www.MOUNJARO.com or call 1-800-545-5979.

This Medication Guide has been approved by the U.S. Food and Drug Administration — Revised: August 2026

MOU-0010-MG-20260827

Mounjaro Single-Dose Pen Instructions for Use

INSTRUCTIONS FOR USE
MOUNJARO® (mown-JAHR-OH)
(tirzepatide)
injection, for subcutaneous use



2.5 mg/0.5 mL single-dose pen
5 mg/0.5 mL single-dose pen
7.5 mg/0.5 mL single-dose pen
10 mg/0.5 mL single-dose pen
12.5 mg/0.5 mL single-dose pen
15 mg/0.5 mL single-dose pen
use 1 time each week

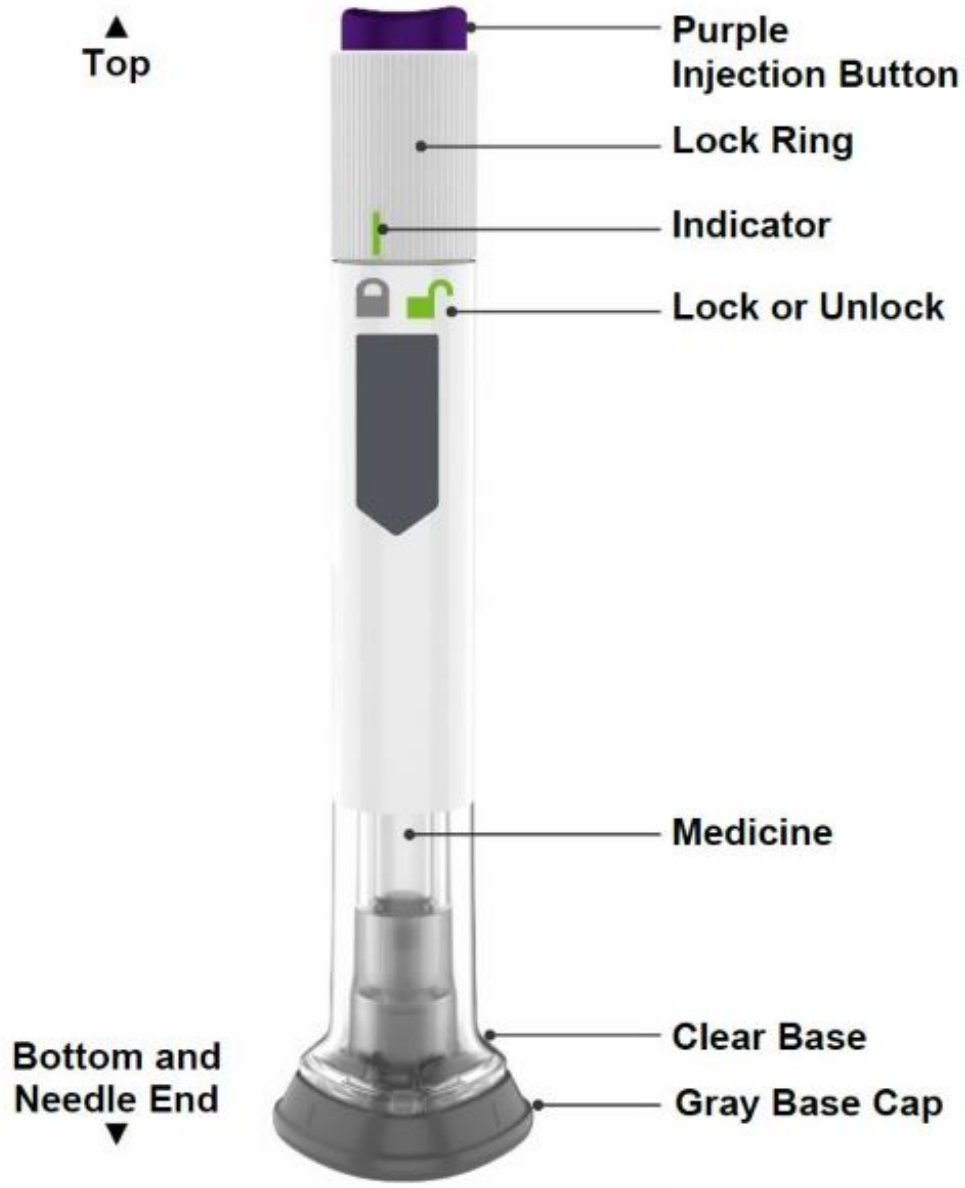
Important information you need to know before injecting MOUNJARO

Read this Instructions for Use and the Medication Guide before using your MOUNJARO Pen and each time you get a refill. There may be new information. This information does not take the place of talking to your healthcare provider about your medical condition or treatment.

Talk to your healthcare provider about how to inject MOUNJARO the right way.

- MOUNJARO is a single-dose prefilled pen.
- MOUNJARO is used 1 time each week.
- Inject under the skin (subcutaneously) only.
- You or another person can inject into your stomach (abdomen) or thigh.
- Another person can inject into the back of your upper arm.

Guide to parts



Preparing to inject MOUNJARO

Remove the Pen from the refrigerator.
Leave the gray base cap on until you are ready to inject.

Check the Pen label to make sure you have the right medicine and dose, and that it has not expired.



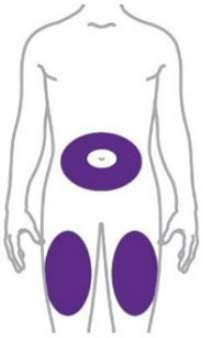
Inspect the Pen to make sure that it is not damaged.

Make sure the medicine:

- is not frozen
- is not cloudy
- is colorless to slightly yellow
- does not have particles

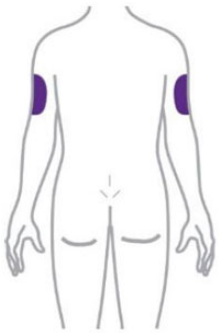
Wash your hands.

Step 1 **Choose your injection site**



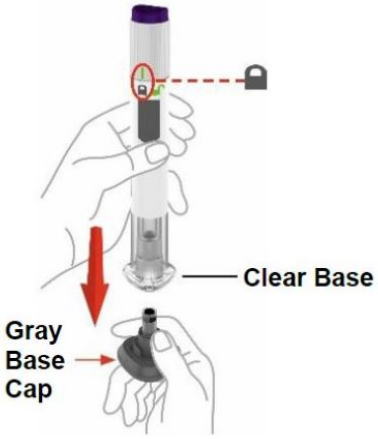
Your healthcare provider can help you choose the injection site that is best for you.

You or another person can inject the medicine in your stomach (abdomen) or thigh.



Another person should give you the injection in the back of your upper arm.
Change (rotate) your injection site each week.
You may use the same area of your body but be sure to choose a different injection site in that area.

Step 2 **Pull off the gray base cap**

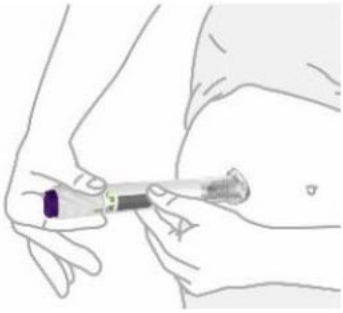


Make sure the Pen is **locked**.
Do not unlock the Pen until you place the clear base on your skin and are ready to inject.

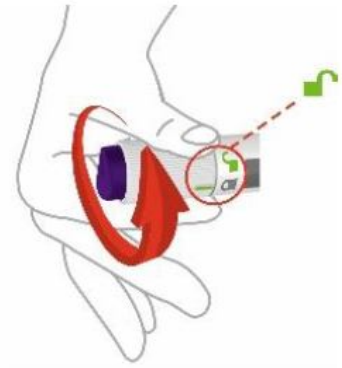
Pull the gray base cap straight off and throw it away in your household trash.
Do not put the gray base cap back on – this could damage the needle.
Do not touch the needle.

Step 3 **Place clear base on skin, then unlock**

Place the clear base flat against your skin at the injection site.

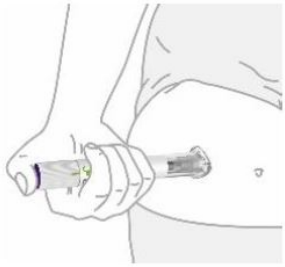


Unlock by turning the lock ring.



Step 4 **Press and hold up to 10 seconds**

Press and hold the purple injection button for up to 10 seconds.
Listen for:
• First click = injection started
• Second click = injection completed



You will know your injection is complete when the gray plunger is



visible.

After your injection, place the used Pen in a sharps container.
See **Disposing of your used Pen.**

Disposing of your used Pen

- Put your used Pen in an FDA-cleared sharps disposal container right away after use. **Do not** throw away (dispose of) Pens in your household trash.

- If you do not have an FDA-cleared sharps disposal container, you may use a household container that is:

  - made of a heavy-duty plastic,
  - can be closed with a tight-fitting, puncture-resistant lid, without sharps being able to come out,
  - upright and stable during use,
  - leak-resistant, and
  - properly labeled to warn of hazardous waste inside the container.

- When your sharps disposal container is almost full, you will need to follow your community guidelines for the right way to dispose of your sharps disposal container. There may be state or local laws about how you should throw away used needles and syringes. For more information about safe sharps disposal, and for specific information about sharps disposal in the state that you live in, go to the FDA's website at: http://www.fda.gov/safesharpsdisposal.

- **Do not** recycle your used sharps disposal container.

Storage and handling

- Store your Pen in the refrigerator between 36°F to 46°F (2°C to 8°C).

- You may store your Pen at room temperature up to 86°F (30°C) for up to a total of

21 days.

- Discard the Pen after a total of 21 days at room temperature.
- **Do not** freeze your Pen. If the Pen has been frozen, throw the Pen away and use a new Pen.
- Store your Pen in the original carton to protect your Pen from light.
- The Pen has glass parts. Handle it carefully. If you drop the Pen on a hard surface, **do not** use it. Use a new Pen for your injection.
- Keep your MOUNJARO Pen and all medicines out of the reach of children.

Commonly asked questions

What if I see air bubbles in my Pen?

Air bubbles are normal.

What if my Pen is not at room temperature?

It is not necessary to warm the Pen to room temperature.

What if I unlock the Pen and press the purple injection button before pulling off the gray base cap?

Do not remove the gray base cap. Throw away the Pen and get a new Pen.

What if there is a drop of liquid on the tip of the needle when I remove the gray base cap?

A drop of liquid on the tip of the needle is normal. **Do not** touch the needle.

Do I need to hold the injection button down until the injection is complete?

This is not necessary, but it may help you keep the Pen steady against your skin.

I heard more than 2 clicks during my injection—2 loud clicks and 1 soft one. Did I get my complete injection?

Some people may hear a soft click right before the second loud click. That is the normal operation of the Pen. **Do not** remove the Pen from your skin until you hear the second loud click.

I am not sure if my Pen worked the right way.



Check to see if you have received your dose. Your dose was delivered the right way if the gray plunger is visible. Also, see **Step 4** of the instructions.
If you do not see the gray plunger, contact Lilly at 1-800-Lilly-Rx (1-800-545-5979) for further instructions. Until then, store your Pen safely to avoid an accidental needle stick.

What if there is a drop of liquid or blood on my skin after my injection?

This is normal. Press a cotton ball or gauze over the injection site. **Do not** rub the injection site.

Other information

• If you have vision problems, **do not** use your Pen without help from a person trained to use the MOUNJARO Pen.

Where to learn more

• If you have questions or problems with your MOUNJARO Pen, contact Lilly at 1-800-Lilly-Rx (1-800-545-5979) or call your healthcare provider.

• For more information about the MOUNJARO Pen, visit our website at www.mounjaro.com.

Scan this code to launch www.mounjaro.com

Marketed by:

Lilly USA, LLC

Indianapolis, IN 46285, USA

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This Instructions for Use has been approved by the U.S. Food and Drug Administration.
Revised: April 2026

MOU-0004-IFU-20260422

Mounjaro Single-Dose Vial Instructions for Use

INSTRUCTIONS FOR USE
MOUNJARO® [mown-JAHR-OH]
(tirzepatide)
injection, for subcutaneous use

2.5 mg/0.5 mL single-dose vial
5 mg/0.5 mL single-dose vial
7.5 mg/0.5 mL single-dose vial
10 mg/0.5 mL single-dose vial

12.5 mg/0.5 mL single-dose vial
15 mg/0.5 mL single-dose vial

Important information you need to know before injecting MOUNJARO

Read this Instructions for Use before you start taking MOUNJARO and each time you get a new vial. There may be new information. This information does not take the place of talking to your healthcare provider about your medical condition or your treatment.

Do not share your needles or syringes with other people. You may give other people a serious infection or get a serious infection from them.

Talk to your healthcare provider about how to inject MOUNJARO the right way.

- MOUNJARO is a single-dose vial.
- MOUNJARO is used 1 time each week.
- Inject under the skin (subcutaneously) only.
- You or another person may inject into your stomach (abdomen) or thigh.
- Another person can inject into the back of your upper arm.

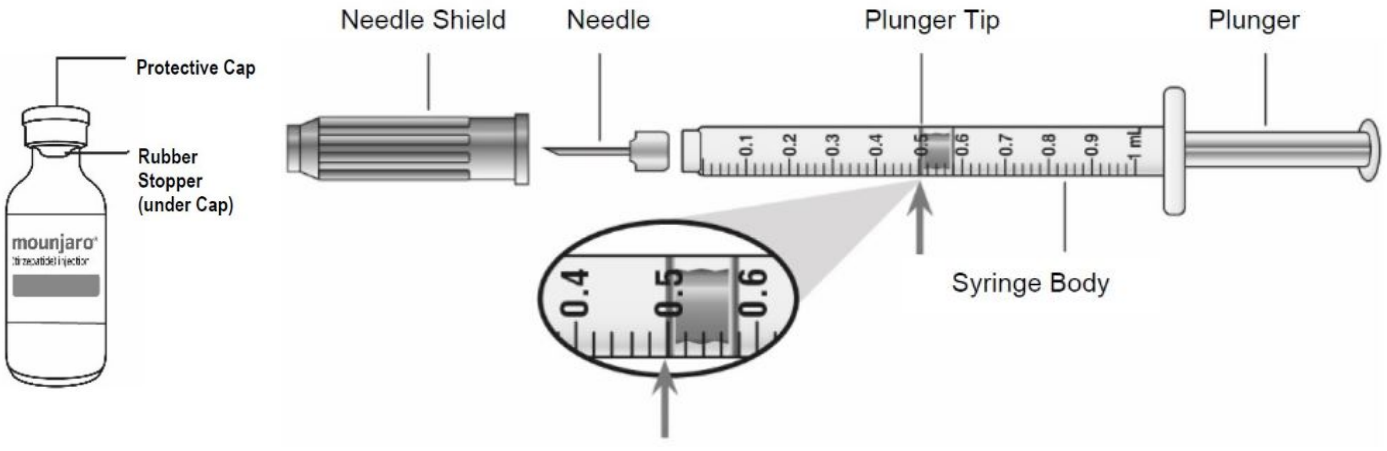
Gather supplies needed to give your injection

- 1 single-dose MOUNJARO vial
- 1 syringe and 1 needle, supplied separately (for example, use a 1 mL syringe and needle as recommended by your healthcare provider)
- 1 alcohol swab
- gauze
- 1 sharps container for throwing away used needles and syringes. **See** "Disposing of used needles and syringes" at the end of these instructions.

Guide to parts

Needle and Syringe (not included)

Vial



Note: The needle and syringe are not included. The needle and syringe recommended by your healthcare provider may look different than the needle and syringe in this Instructions for Use.

Preparing to inject MOUNJARO

Remove the vial from the refrigerator.

Check the vial label to make sure you have the right medicine and dose, and that it has not expired.

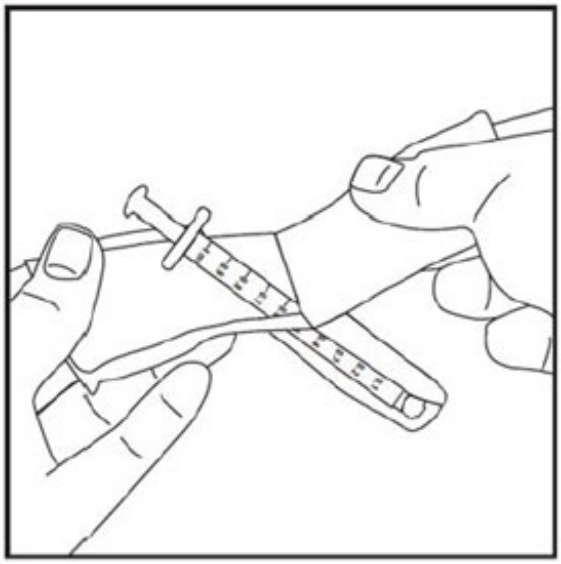
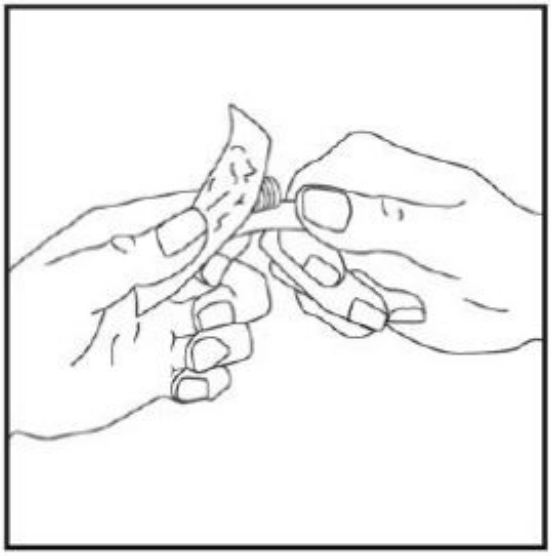
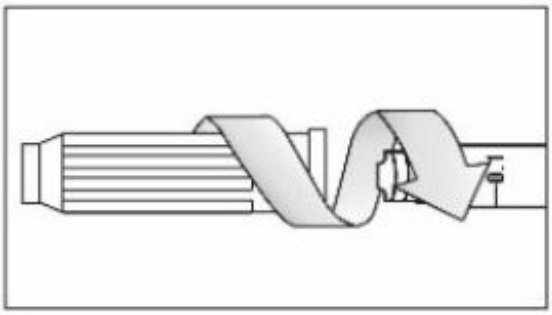
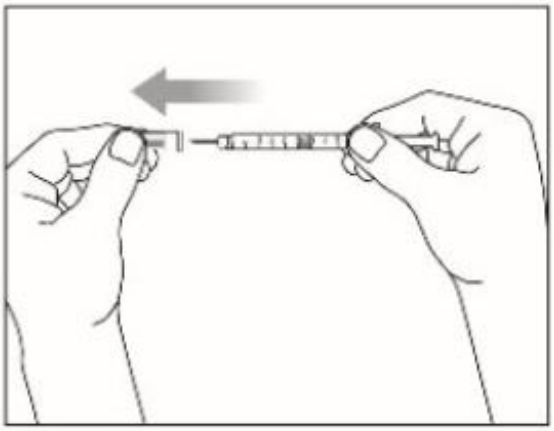
Make sure the medicine:

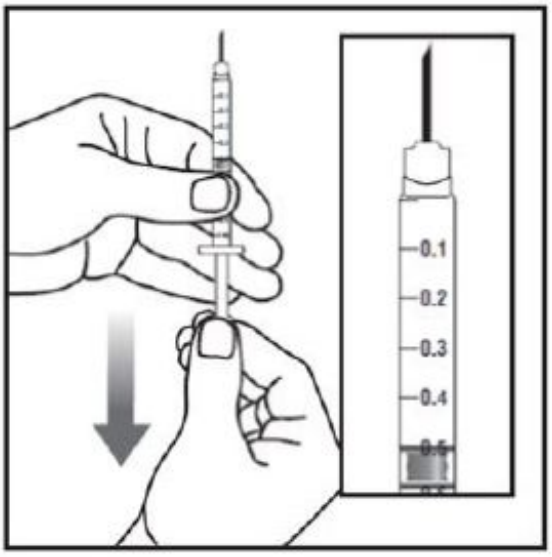
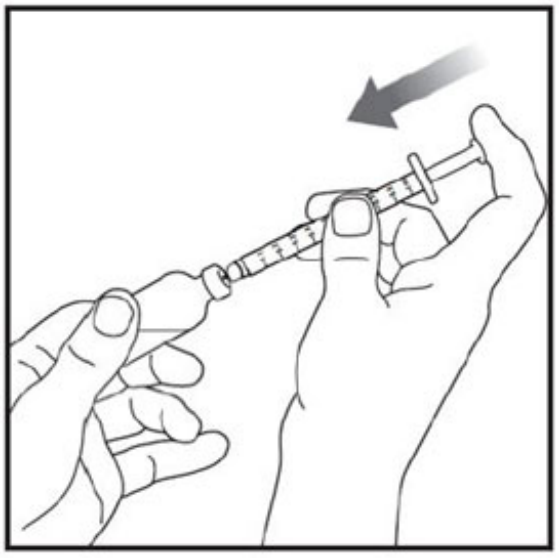
- is not frozen
- is not cloudy
- is colorless to slightly yellow
- does not have particles

Always use a new syringe and needle for each injection to prevent infections and blocked needles. Do not reuse or share your syringes or needles with other people. You may give other people a serious infection or get a serious infection from them.

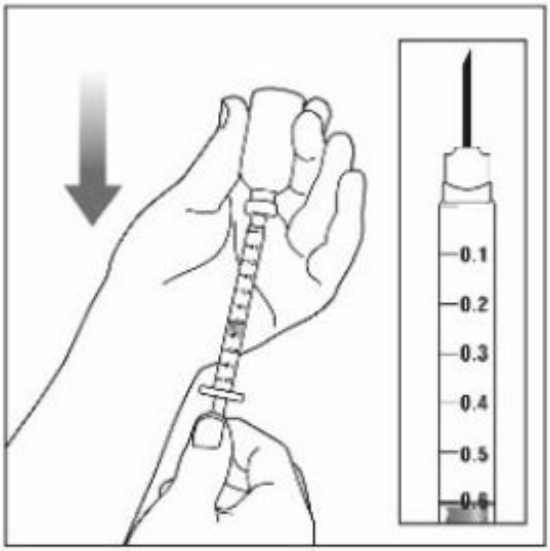
Wash your hands with soap and water.

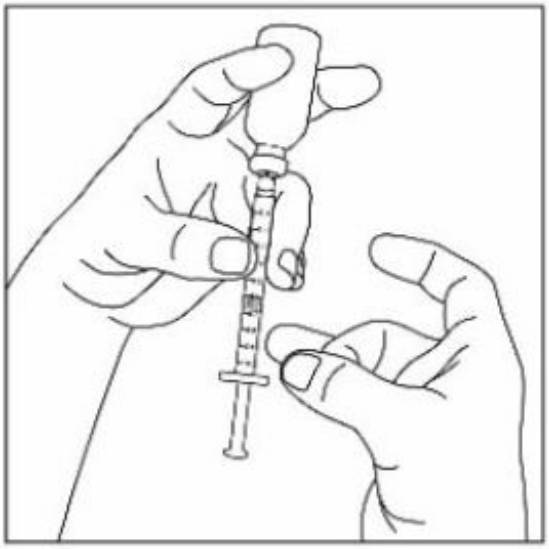
Step 1: Pull off the plastic protective cap. **Do not** remove the rubber stopper.	
Step 2: Wipe the rubber stopper with an alcohol swab.	

Step 3: Remove the outer wrapping from the syringe.	
Step 4: Remove the outer wrapping from the needle. The syringe that your healthcare provider recommended may have a pre-attached needle. If the needle is attached, skip to step 6.	
Step 5: Place the needle on top of the syringe and turn until it is tight and firmly attached.	
Step 6: Remove the needle shield by pulling straight off.	

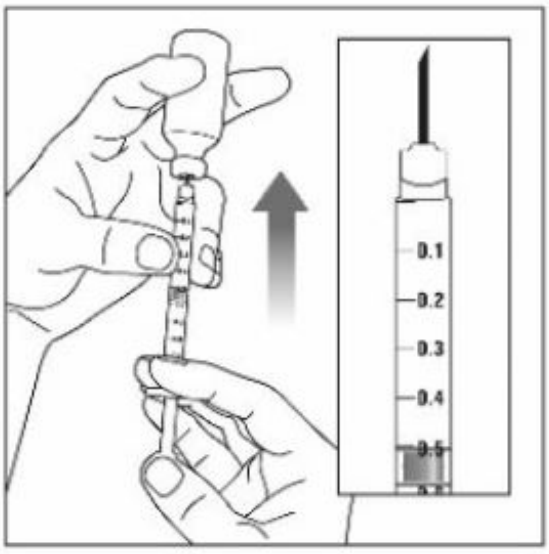
Step 7: Hold the syringe in one hand with the needle pointing up. With the other hand pull down on the plunger until the plunger tip reaches the line on the syringe indicating that 0.5 mL of air has been drawn into the syringe.	
Step 8: Push the needle through the rubber stopper of the vial.	
Step 9: Push the plunger all the way in. This puts air into the vial and makes it easier to pull the solution from the vial.	

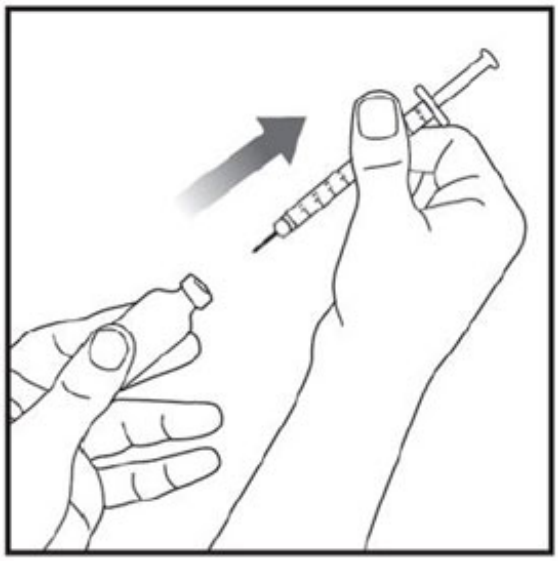
Step 10:
Turn the vial and syringe upside down. Make sure that the tip of the needle is in the liquid and slowly pull the plunger down until the plunger tip is past the 0.5 mL line.
If there are air bubbles, tap the syringe gently a few times to let any air bubbles rise to the top.





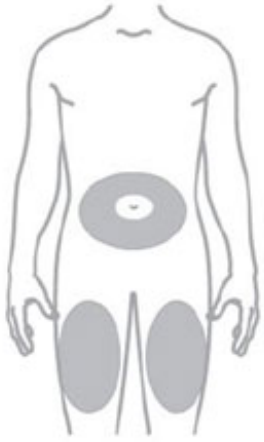
Step 11:
Slowly push the plunger up until the plunger tip reaches the 0.5 mL line.

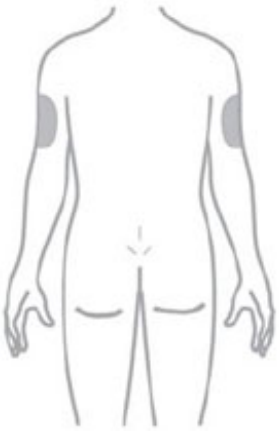
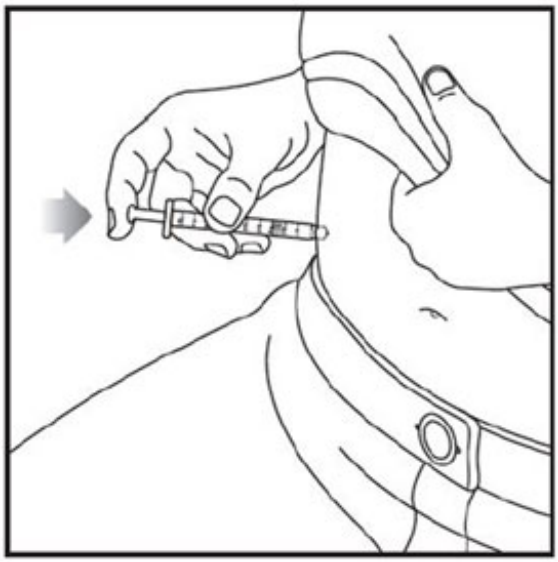


Step 12: Pull the syringe out of the rubber stopper of the vial.	

Injecting MOUNJARO

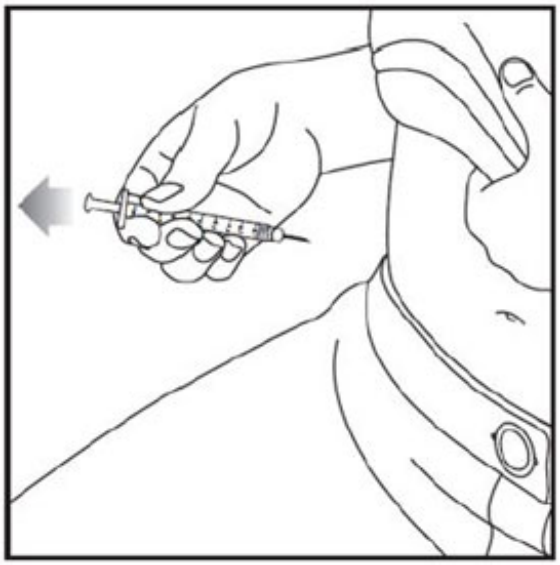
- Inject exactly as your healthcare provider has shown you. Your healthcare provider should tell you if you should pinch the skin before injecting.
- **Change (rotate) your injection site within the area you choose for each dose** to reduce your risk of getting lipodystrophy (pits in skin or thickened skin) and localized cutaneous amyloidosis (skin with lumps) at the injection sites.
- **Do not** inject where the skin has pits, is thickened, or has lumps.
- **Do not** inject where the skin is tender, bruised, scaly or hard, or into scars or damaged skin.
- **Do not** mix MOUNJARO with any other medicine.
- **Do not** inject MOUNJARO in the same injection site used for other medicines.

Step 13: Choose your injection site. You can inject MOUNJARO under the skin (subcutaneously) of your stomach area (abdomen) or thighs. Someone else can inject in your stomach area, thighs, or the back of the upper arms.	

	
Step 14: Insert the needle into your skin.	
Step 15: Push down on the plunger to inject your dose. The needle should stay in your skin for at least 5 seconds to make sure you have injected all of your dose.	

Step 16:
Pull the needle out of your skin.

- If you see blood after you take the needle out of your skin, press the injection site with a piece of gauze. **Do not** rub the area.
- **Do not** recap the needle. Recapping the needle can lead to a needle stick injury.



Disposing of used needles and syringes

- Put your used needle and syringe in an FDA-cleared sharps disposal container right away after use. **Do not** throw away (dispose of) loose needles and syringes in your household trash.
- If you do not have an FDA-cleared sharps disposal container, you may use a household container that is:
    - made of a heavy-duty plastic,
    - can be closed with a tight-fitting, puncture-resistant lid, without sharps being able to come out,
    - upright and stable during use,
    - leak-resistant, and
    - properly labeled to warn of hazardous waste inside the container.
- When your sharps disposal container is almost full, you will need to follow your community guidelines for the right way to dispose of your sharps disposal container. There may be state or local laws about how you should throw away used needles and syringes. For more information about safe sharps disposal, and for specific information about sharps disposal in the state that you live in, go to the FDA's website at: http://www.fda.gov/safesharpsdisposal.
- **Do not** dispose of your used sharps disposal container in your household trash unless your community guidelines permit this. **Do not** recycle your used sharps disposal container.

Storing MOUNJARO

- Store all unopened vials in the refrigerator at 36°F to 46°F (2°C to 8°C).
- You may store the unopened vial at room temperature up to 86°F (30°C) for up to a total of 21 days.
- Discard the vial after a total of 21 days at room temperature.
- **Do not** freeze. **Do not** use if MOUNJARO has been frozen.
- Store the vial in the original carton to protect from light.
- Throw away all opened vials after use, even if there is medicine left in the vial.

Keep MOUNJARO vials, syringes, needles, and all medicines out of the reach of children.

If you have any questions or problems with your MOUNJARO, contact Lilly at 1-800-Lilly-Rx (1-800-545-5979) or call your healthcare provider for help.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

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MON-VL-0003-IFU-20260422

This Instructions for Use has been approved by the U.S. Food and Drug Administration.

Revised: April 2026

Mounjaro Multiple-Dose Vial Instructions for Use

INSTRUCTIONS FOR USE

MOUNJARO® [mown-JAHR-OH]

(tirzepatide)

injection, for subcutaneous use

Multi-dose vial

Each vial contains 4 doses, one dose taken weekly.

Important information you need to know before injecting MOUNJARO

Read this Instructions for Use before you start taking MOUNJARO and each time you get a new vial. There may be new information. This information does not take the place of talking to your healthcare provider about your medical condition or your treatment.

Do not share your needles or syringes with other people.

Talk to your healthcare provider about how to inject MOUNJARO the right way.

- MOUNJARO is a multi-dose vial. **The vial contains 4 doses, one dose taken weekly.**
- Inject **0.6 mL** in a single weekly injection, under the skin (subcutaneously).
- You or another person may inject into your stomach (abdomen) or thigh.
- Another person can inject into the back of your upper arm.

Gather supplies needed to give your injection

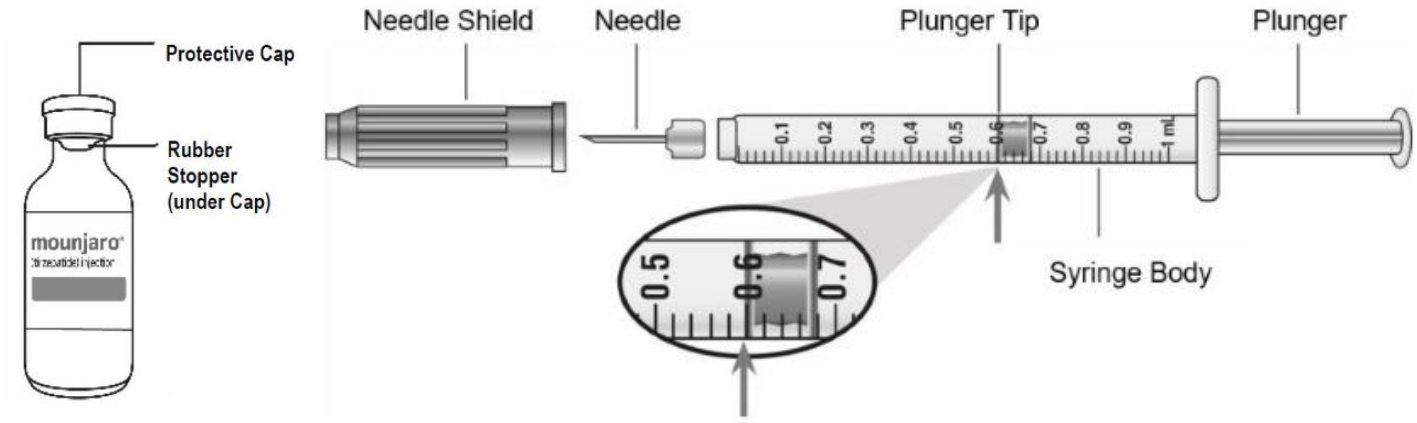
- 1 multi-dose MOUNJARO vial
- 1 syringe and 1 needle, supplied separately (for example, use a 1 mL syringe and needle as recommended by your healthcare provider)
- 1 alcohol swab
- gauze
- 1 sharps container for throwing away used needles and syringes. **See** “Disposing of

used needles and syringes" at the end of these instructions.

Guide to parts

Vial

Needle and Syringe (not included)



Note: The needle and syringe are not included. The needle and syringe recommended by your healthcare provider may look different than the needle and syringe in this Instructions for Use.

Preparing to inject MOUNJARO

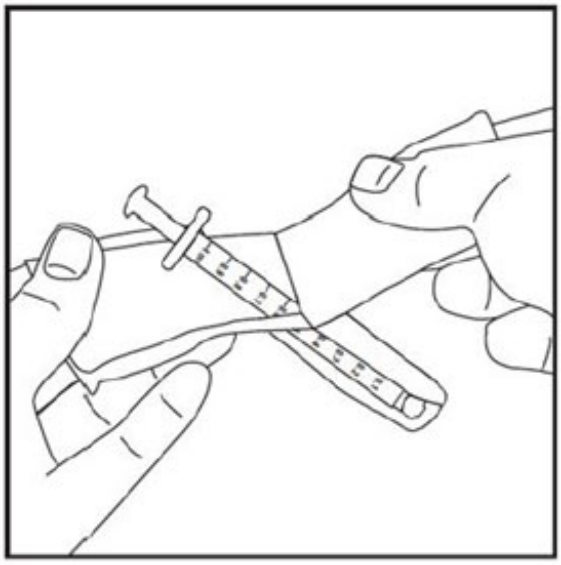
Check the vial label to make sure you have the right medicine and dose, and that it has not expired.

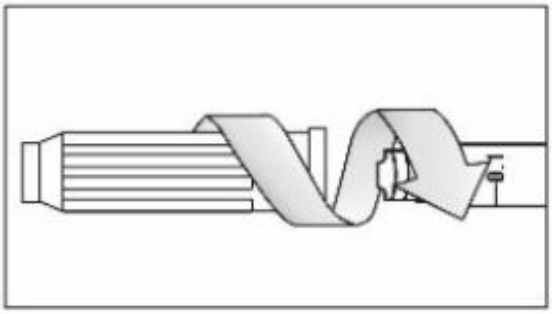
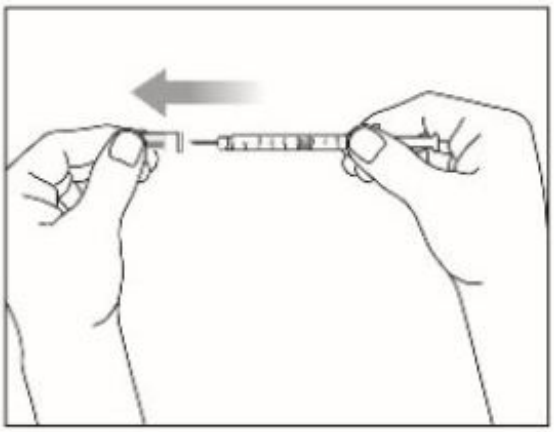
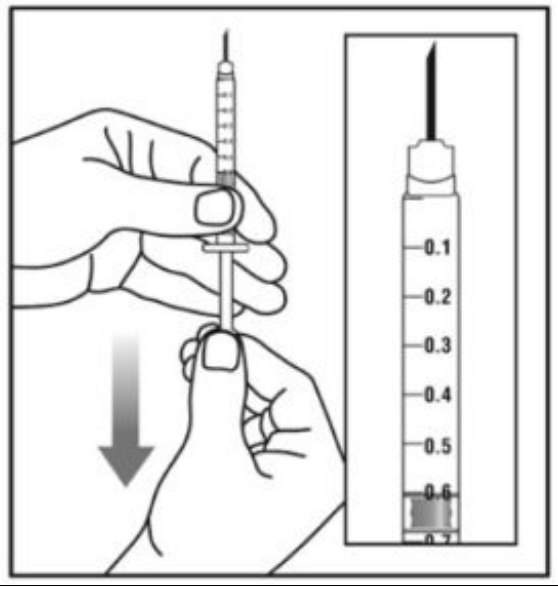
Make sure the medicine:

- is not frozen
- is colorless to slightly yellow
- is not cloudy
- does not have particles

Always use a new syringe and needle for each injection to prevent infections and blocked needles. Do not reuse or share your syringes or needles with other people. You may give other people a serious infection or get a serious infection from them.

Wash your hands with soap and water.

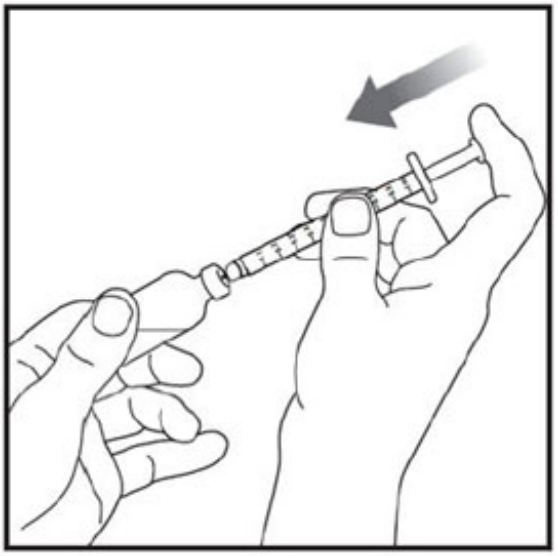
Step 1: If you are using a new vial, pull off the plastic protective cap. **Do not** remove the rubber stopper.	
Step 2: Wipe the rubber stopper with an alcohol swab.	
Step 3: Remove the outer wrapping from the syringe.	

Step 4: Remove the outer wrapping from the needle. The syringe that your healthcare provider recommended may have a pre-attached needle. If the needle is attached, skip to step 6.	
Step 5: Place the needle on top of the syringe and turn until it is tight and firmly attached.	
Step 6: Remove the needle shield by pulling straight off.	
Step 7: Hold the syringe in one hand with the needle pointing up. With the other hand pull down on the plunger until the plunger tip reaches the line on the syringe indicating that **0.6 mL** of air has been drawn into the syringe.	

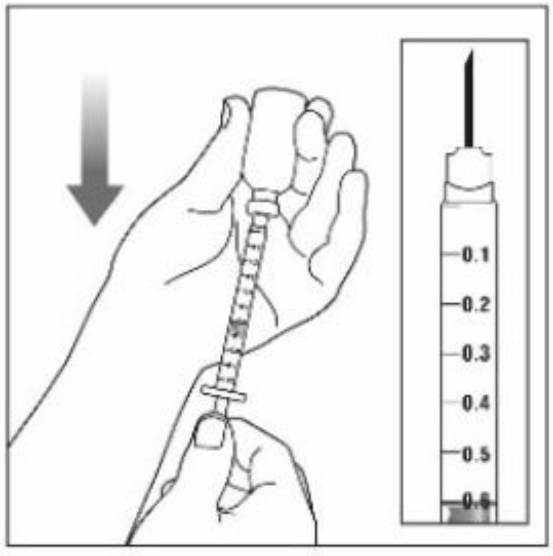
Step 8:
Push the needle through the rubber stopper of the vial.

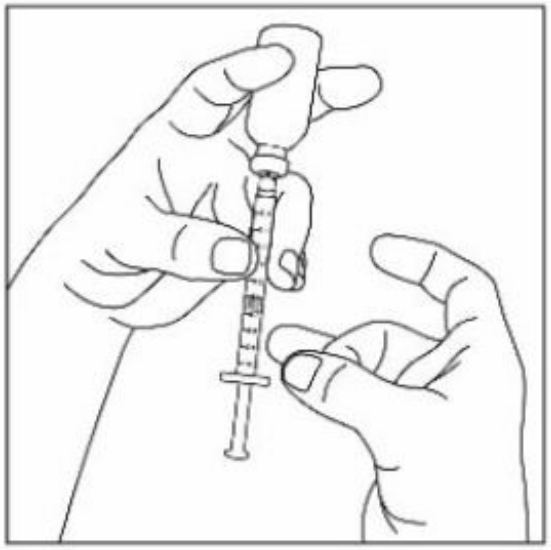
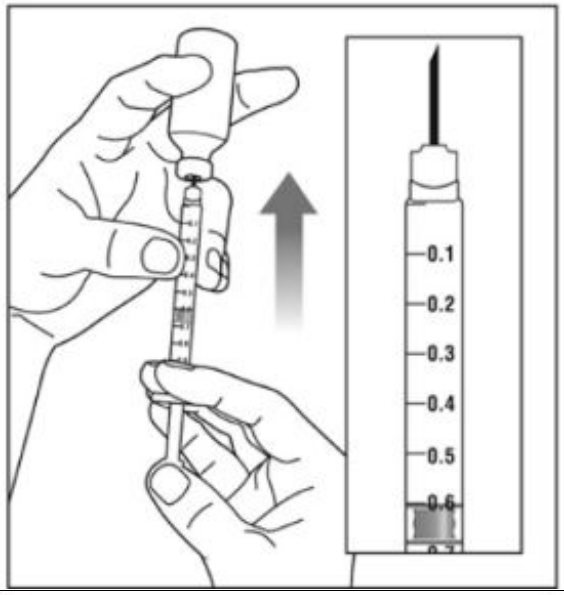
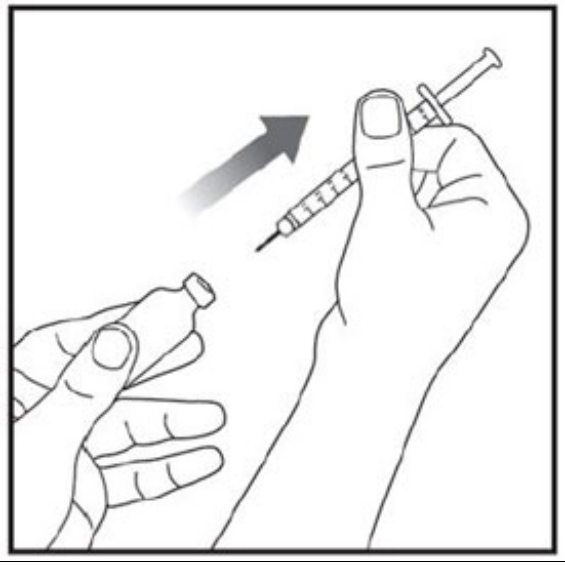


Step 9:
Push the plunger all the way in. This puts air into the vial and makes it easier to pull the solution from the vial.



Step 10:
Turn the vial and syringe upside down. Make sure that the tip of the needle is in the liquid and slowly pull the plunger down until the plunger tip is past the **0.6 mL** line.
If there are air bubbles, tap the syringe gently a few times to let any air bubbles rise to the top.



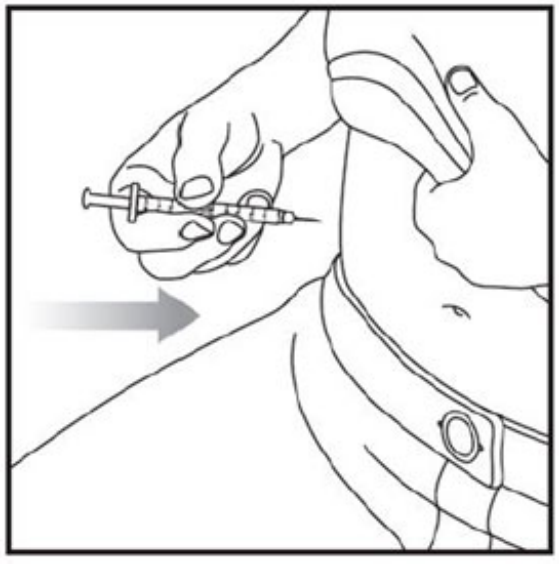
	
Step 11: Slowly push the plunger up until the plunger tip reaches the **0.6 mL** line.	
Step 12: Pull the syringe out of the rubber stopper of the vial.	

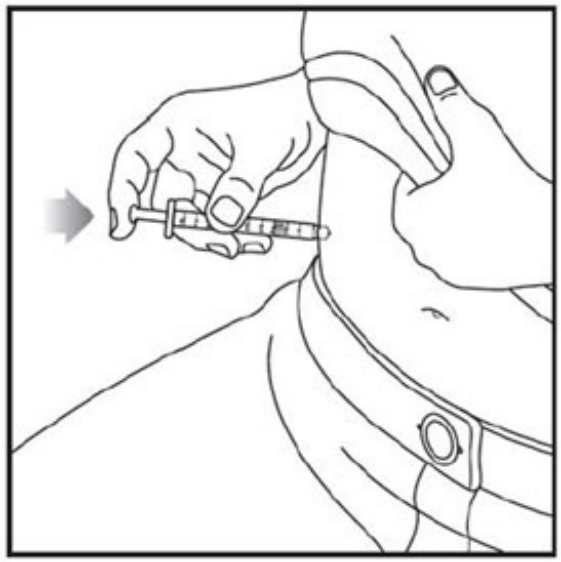
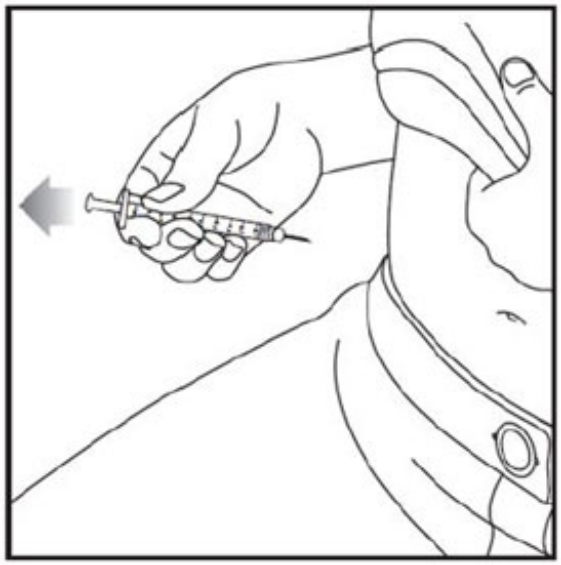
Injecting MOUNJARO

- Inject exactly as your healthcare provider has shown you. Your healthcare provider should tell you if you should pinch the skin before injecting.
- **Change (rotate) your injection site within the area you choose for each**

dose to reduce your risk of getting lipodystrophy (pits in skin or thickened skin) and localized cutaneous amyloidosis (skin with lumps) at the injection sites.

- **Do not** inject where the skin has pits, is thickened, or has lumps.
- **Do not** inject where the skin is tender, bruised, scaly or hard, or into scars or damaged skin.
- **Do not** mix MOUNJARO with any other medicine.
- **Do not** inject MOUNJARO in the same injection site used for other medicines.

Step 13: Choose your injection site. You can inject MOUNJARO under the skin (subcutaneously) of your stomach area (abdomen) or thighs. Someone else can inject in your stomach area, thighs, or the back of the upper arms.	
Step 14: Insert the needle into your skin.	

Step 15: Push down on the plunger to inject your dose. The needle should stay in your skin for at least 5 seconds to make sure you have injected all of your dose.	
Step 16: Pull the needle out of your skin. • If you see blood after you take the needle out of your skin, press the injection site with a piece of gauze. **Do not** rub the area. • **Do not** recap the needle. Recapping the needle can lead to a needle stick injury.	

Disposing of used needles and syringes

- Put your used needle and syringe in an FDA-cleared sharps disposal container right away after use. **Do not** throw away (dispose of) loose needles and syringes in your household trash.
- If you do not have an FDA-cleared sharps disposal container, you may use a household container that is:

    - made of a heavy-duty plastic,
    - can be closed with a tight-fitting, puncture-resistant lid, without sharps being able to come out,
    - upright and stable during use,
    - leak-resistant, and
    - properly labeled to warn of hazardous waste inside the container.

- When your sharps disposal container is almost full, you will need to follow your community guidelines for the right way to dispose of your sharps disposal container. There may be state or local laws about how you should throw away used needles and syringes. For more information about safe sharps disposal, and for specific information about sharps disposal in the state that you live in, go to the FDA's website at:

http://www.fda.gov/safesharpsdisposal.
- **Do not** dispose of your used sharps disposal container in your household trash unless your community guidelines permit this. **Do not** recycle your used sharps disposal container.

Storing MOUNJARO
- **Do not** freeze. **Do not** use if MOUNJARO has been frozen.
- Store MOUNJARO in the original carton. Keep away from light.

Unopened vial:
- Store unopened vial in the refrigerator at 36°F to 46°F (2°C to 8°C). It can be used until the expiration date on the label if kept in the refrigerator.
- If stored at room temperature [up to 86°F (30°C)], throw away unopened vial after 30 days.

After vial has been opened:
- Store opened (in-use) vial in the original carton in the refrigerator at 36°F to 46°F (2°C to 8°C) or at room temperature [up to 86°F (30°C)]. Throw away opened vial after a total of 30 days at room temperature, 30 days after first use, or after taking 4 weekly doses, even if there is medicine left in it.

Keep MOUNJARO vials, syringes, needles, and all medicines out of the reach of children.

If you have any questions or problems with your MOUNJARO, contact Lilly at 1-800-Lilly-Rx (1-800-545-5979) or call your healthcare provider for help.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

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MON-MDV-0001-IFU-20260107

This Instructions for Use has been approved by the U.S. Food and Drug Administration.

Revised: January 2026

Mounjaro KwikPen Multi-Dose Pen Instructions for Use

INSTRUCTIONS FOR USE

MOUNJARO® [mown-JAHR-OH] KwikPen ® [KWIHK pehn]

(tirzepatide)

injection, for subcutaneous use

multi-dose single-patient-use prefilled pen

Each pen contains 4 fixed doses, one dose taken weekly.

This Instructions for Use contains information on how to inject MOUNJARO.



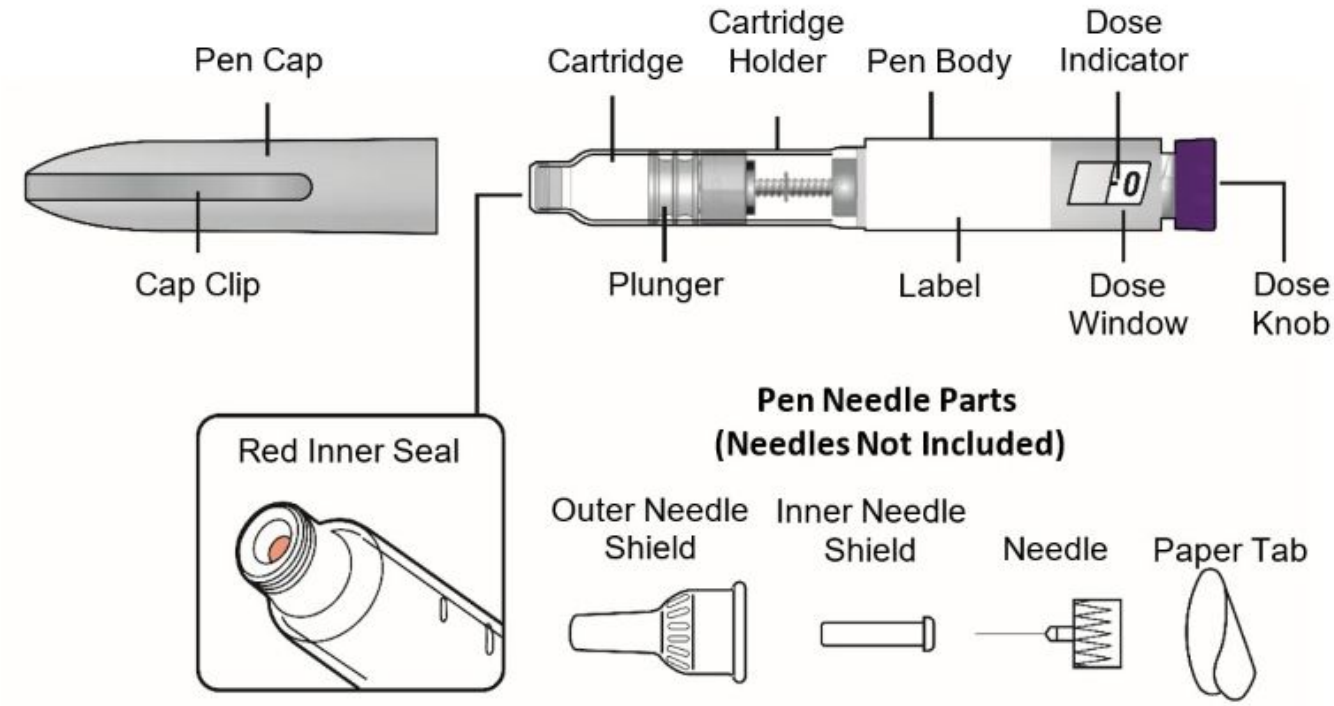
Important information you need to know before injecting MOUNJARO

Read this **INSTRUCTIONS FOR USE** before you start injecting MOUNJARO KwikPen (Pen) and each time you get another new Pen. There may be new information. This information does not take the place of talking to your healthcare provider about your medical condition or your treatment.

- MOUNJARO KwikPen is a disposable multi-dose single-patient-use prefilled pen. **The Pen contains 4 fixed doses, one dose taken weekly.** Inject a single weekly injection, under the skin (subcutaneously).
- **For your weekly dose, you need to turn the dose knob all the way until it stops AND the 1 icon is in dose window.**
- After 4 doses, throw away (discard) the Pen, including the unused medicine. The Pen will prevent you from dialing a full dose after you have given yourself 4 weekly doses. **Do not** inject the leftover medicine. **Do not** transfer MOUNJARO from your Pen into a syringe.
- **Do not** share your MOUNJARO Pen with other people, even if the pen needle has been changed. You may give other people a serious infection or get a serious infection from them.
- People who are blind or have vision problems should not use the Pen without help from a person trained to use the Pen.

Guide to parts

Parts of the MOUNJARO Pen



Supplies needed to give your injection

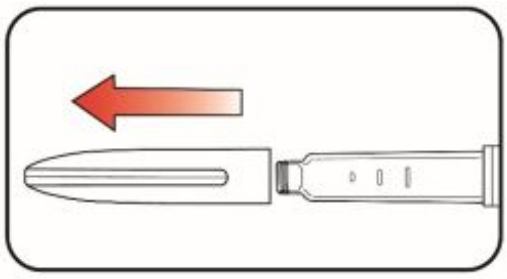
- MOUNJARO KwikPen
- KwikPen-compatible Pen Needle (not included)
- Alcohol swab
- Gauze or cotton ball
- FDA-cleared sharps disposal container or household container

Preparing to inject MOUNJARO



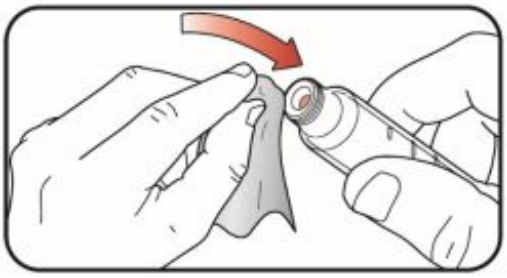
Step 1:

- Wash your hands with soap and water.



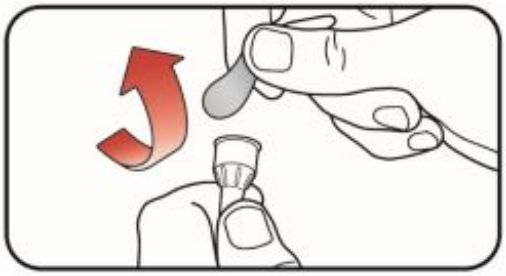
Step 2:

- Pull the Pen cap straight off.
- Inspect the Pen and label. **Do not** use if:
  - the medicine name or dose strength does not match your prescription.
  - the Pen is expired (EXP) or looks damaged.
  - the medicine has been frozen, has particles, is cloudy, or is discolored. MOUNJARO should be colorless to slightly yellow.



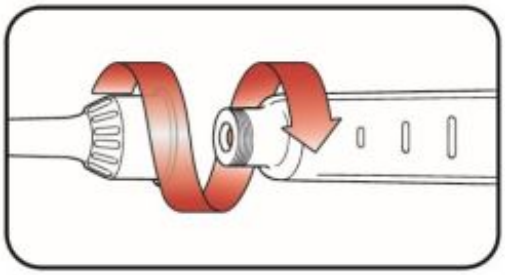
Step 3:

- Wipe the red inner seal with an alcohol swab.



Step 4:

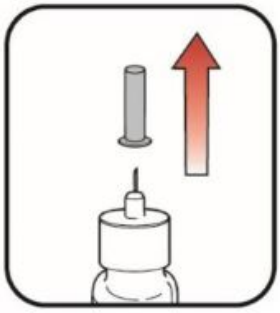
- **Select a new pen needle.** Always use a new pen needle for each injection to help prevent infections and blocked needles.
- Pull off the paper tab from the outer needle shield.



Step 5:

- Push the capped pen needle straight onto the Pen and twist the pen needle on until it is tight.





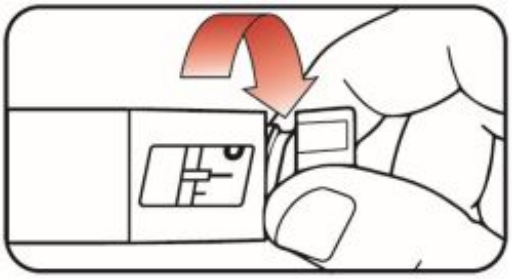
a. Outer needle shield **b. Inner needle shield**

Step 6:

a. Pull off the outer needle shield and keep it. This will be reused.
b. Pull off the inner needle shield. Put it in your household trash.

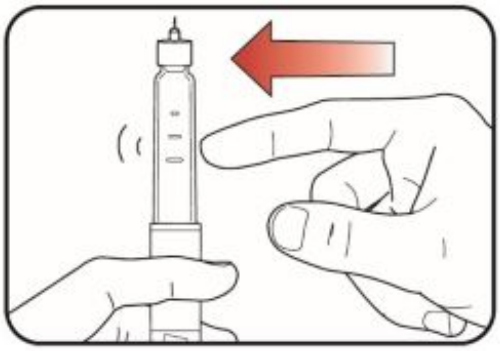
Priming your Pen

Prime before each weekly injection. Priming removes air from the cartridge and makes sure that your Pen is working correctly.



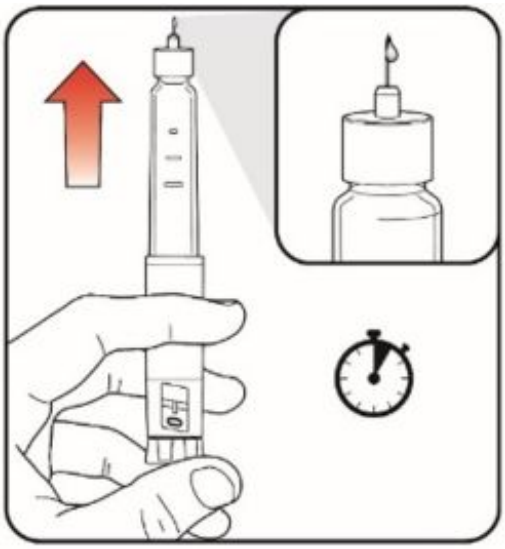
Step 7:

- Slowly turn the dose knob until you hear **2 clicks** and the  extended line is shown in the dose window.
  This is the prime position. It can be corrected by turning the dose knob in either direction until the prime position lines up to the dose indicator.



Step 8:

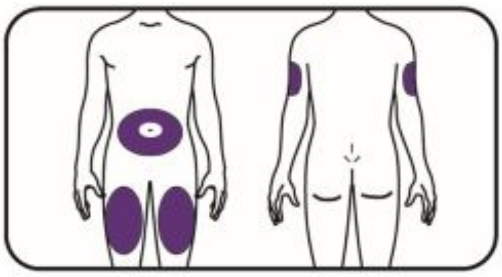
- Hold your Pen with the needle pointing up.
- Tap the cartridge holder gently to collect air bubbles at the top.



Step 9:

- Release some medicine into the air by **pushing the dose knob in** until it stops, then **slowly count to 5 while holding the dose knob**. The 0 icon must be shown in the dose window. **Do not** inject into your body.
  Your Pen has been primed if a small amount (a drop) of medicine comes out of the tip of the pen needle.
  - If you do not see medicine, repeat **steps 7-9**, no more than 2 additional times.
  - If you still do not see medicine, then change the pen needle and repeat **steps 7-9,** no more than 1 additional time.
  - If you still do not see medicine, contact Lilly at
    1-800-LillyRx (1-800-545-5979).

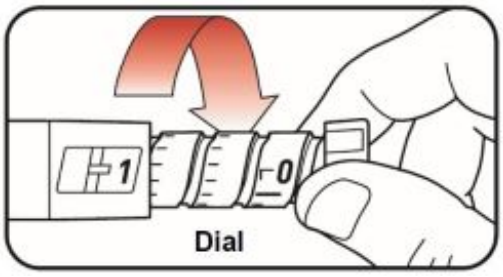
Injecting MOUNJARO



a. Front **b. Back**

Step 10:

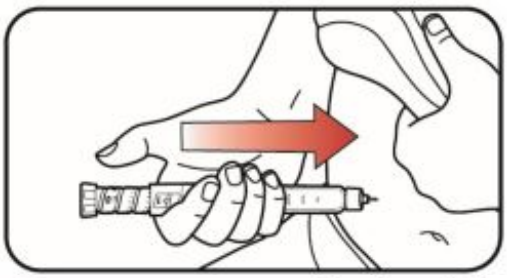
- Choose an injection site.
  a. You or another person can inject the medicine in your thigh or stomach (abdomen) at least 2 inches from the belly button.
  b. Another person should give you the injection in the back of your upper arm.
- **Change (rotate)** your injection site each week. You may use the same area of your body but be sure to choose a different injection site in that area.



Step 11:

- **Turn the dose knob all the way until it stops** AND the 1 icon is in the dose window. This is your weekly dose.

  Do not count clicks as you select the dose.

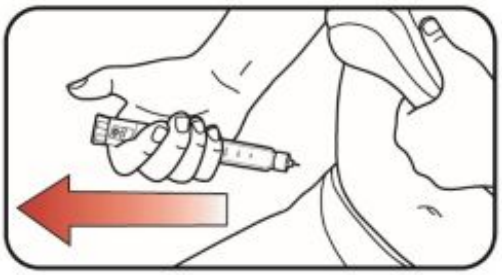


Step 12:

a. Insert the needle into your skin.



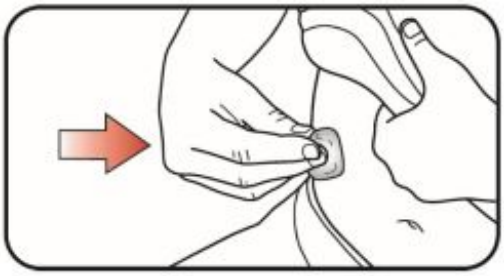
b. Inject the medicine by **pushing the dose knob in** until it stops then **slowly count to 5 while holding the dose knob.** You may notice the plunger moving with each injection. The 0 icon must be shown in the dose window before removing the needle.



Step 13:

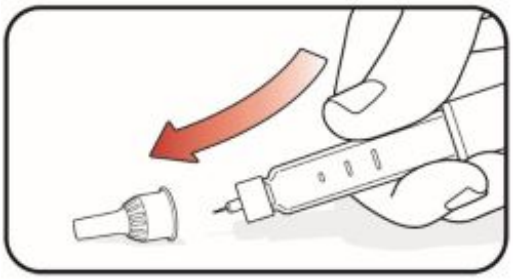
- Pull the needle out of your skin. A drop of medicine on the needle tip is normal. It will not affect your dose.
- Confirm the 0 icon is in the dose window.
  If you see the 0 icon in the dose window, you have received the full dose.
  If you do not see the 0 icon in the dose window, insert the needle back into your skin and finish your injection. **Do not** redial the dose.
  If you still do not think you received the full dose, **do not** start over or repeat the injection. See "Troubleshooting" section for more information.

After your MOUNJARO injection

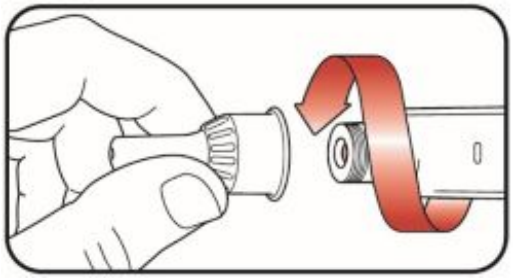


Step 14:

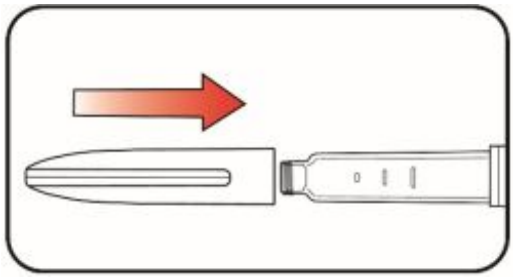
- If you see blood after you pull the needle out of your skin, lightly press the injection site with gauze or a cotton ball.
  Do not rub the injection site.



Step 15:
- Carefully replace the outer needle shield.



Step 16:
- Unscrew the capped needle and put the needle in a sharps container (see "Disposing of MOUNJARO KwikPen and pen needles" section).
  Do not store the Pen with the needle attached to prevent leaking, blocking the needle, and air from entering the Pen.



Step 17:
- Replace the Pen cap.
  Do not store the Pen without the Pen cap attached.

Storing your MOUNJARO KwikPen

Unused Pens:
- Store **unused Pens** in the **refrigerator** between 36°F to 46°F (2°C to 8°C).
- Unused Pens may be used until the expiration date (EXP) printed on the label if the Pen has been kept in the refrigerator.
- **Do not** freeze your Pen. Throw away (discard) the Pen if it has been frozen.

Used Pens:
- You may store your **used Pen** at **room temperature** up to 86°F (30°C) after your injection.
- Keep away from heat and light.
- Keep your Pen and needles out of the sight and reach of children.
- Throw away the Pen 30 days after first use even though the Pen has medicine left in it.
- Throw away the Pen after receiving 4 weekly doses. Attempting to inject any leftover medicine could result in an incomplete dose even though the Pen still has medicine left in it.

Disposing of MOUNJARO KwikPen and pen needles

- Put your used pen needles in an FDA-cleared sharps disposal container right away after use.
- **Do not** throw away (discard) loose pen needles in your household trash.

- Discard the used Pen in your household trash or FDA-cleared sharps disposal container after you have removed the needle.
- If you do not have an FDA-cleared sharps disposal container, you may use a household container that is:

  - made of a heavy-duty plastic,
  - can be closed with a tight-fitting, puncture-resistant lid, without sharps being able to come out,
  - upright and stable during use,
  - leak-resistant, and
  - properly labeled to warn of hazardous waste inside the container.

- When your sharps disposal container is almost full, you will need to follow your community guidelines for the right way to dispose of your sharps disposal container. There may be state or local laws about how you should throw away (discard) used needles. For more information about safe sharps disposal, and for specific information about sharps disposal in the state you live in, go to the FDA's website at: http://www.fda.gov/safesharpsdisposal.
- Do not dispose of your used sharps disposal container in your household trash unless your community guidelines permit this. Do not recycle your used sharps disposal container.

Troubleshooting

- If you cannot remove the Pen cap, gently twist the Pen cap back and forth, and then pull the Pen cap straight off.
- If the dose knob is hard to push:

  - pushing the dose knob more slowly will make it easier to inject.
  - your needle may be blocked. Put on a new pen needle and prime the Pen.
  - you may have dust, food, or liquid inside the Pen. Throw the Pen away and get a new Pen.

- If the Pen prevents you from turning the dose knob until the  is in the dose window:

  - Throw away (discard) the Pen, including the unused medicine. There may not be enough medicine left in the pen to give a full dose. **Do not** attempt to inject the leftover medicine.

Additional Information:

If you have any questions or problems with MOUNJARO KwikPen:

- Call your healthcare provider
- Call 1-800-Lilly-Rx (1-800-545-5979)
- Visit www.mounjaro.com

Scan this code to launch www.mounjaro.com

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Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

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MOUNJARO KwikPen meets the current dose accuracy and functional requirements of ISO 11608-1.

This Instructions for Use has been approved by the U.S. Food and Drug Administration.

Revised: January 2026

MOUKP-0002-IFU-20260121

Medication Calendar

Use MOUNJARO 1 time a week. Write the day of the week you choose to inject. Inject on this day each week (Example: Monday).	I inject my weekly dose on the dates below. (Month/Day)	(Month/Day)	(Month/Day)	(Month/Day)

PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 2.5 mg/0.5 mL Dose

NDC 0002-1506-80

Mounjaro®

(tirzepatide) injection

2.5 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 2.5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 5 mg/0.5 mL Dose

NDC 0002-1495-80

Mounjaro®

(tirzepatide) injection

5 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly

NDC 0002-1495-80

mounjaro®
(tirzepatide) injection

4 x 5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Use one pen every week.

4 Single-dose prefilled pens

5 mg / 0.5 mL



Lilly

---

mounjaro®
(tirzepatide) injection

5 mg / 0.5 mL

4 Single-dose prefilled pens

---

5 mg / 0.5 mL

mounjaro®
(tirzepatide) injection

4 Single-dose prefilled pens

---

mounjaro®
(tirzepatide) injection

5 mg / 0.5 mL

4 Single-dose prefilled pens

PHARMACY
LABEL HERE

Store refrigerated at 2°C to 8°C (36°F to 46°F) in original carton to protect from light. DO NOT FREEZE.
MOUNJARO can be stored at room temperature up to 30°C (86°F) for up to 21 days in the carton. Discard if not used within 21 days after removing from the refrigerator.
Keep out of reach of children.
Recommended Dosage: See prescribing information.
Carton Contents:
4 Single-dose prefilled pens
Package Insert
Medication Guide
Instructions for Use

Each 0.5 mL of MOUNJARO contains 5 mg tirzepatide: sodium chloride 4.1 mg, sodium phosphate dibasic heptahydrate 0.7 mg, and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH.
MOUNJARO has a pH of 6.5 to 7.5.

No preservative.

No U.S. standard of potency.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

www.mounjaro.com

3 0002-1495-80 0

SH0670AAM01

GTIN:00002149580
Lot/Exp/Serial No

4 Single-dose prefilled pens

5 mg / 0.5 mL

mounjaro®
(tirzepatide) injection

PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 7.5 mg/0.5 mL Dose

NDC 0002-1484-80

Mounjaro®

(tirzepatide) injection

7.5 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 7.5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly

mounjaro®
(tirzepatide) injection

4 x 7.5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Use one pen every week.

4 Single-dose prefilled pens

Lilly



7.5 mg / 0.5 mL

NDC 0002-1484-80

mounjaro®
(tirzepatide) injection

7.5 mg / 0.5 mL

GTIN: 00002148480 4

Lot/ Exp./ Serial No

4 Single-dose prefilled pens

Store refrigerated at 2°C to 8°C (36°F to 46°F) in original carton to protect from light. DO NOT FREEZE.

MOUNJARO can be stored at room temperature up to 30 °C (86°F) for up to 21 days in the carton. Discard if not used within 21 days after removing from the refrigerator.

Keep out of reach of children.

Recommended Dosage: See prescribing information.

Carton Contents:
4 Single-dose prefilled pens
Package Insert
Medication Guide
Instructions for Use

Each 0.5 mL of MOUNJARO contains 7.5 mg tirzepatide; sodium chloride (4.1 mg), sodium phosphate dibasic heptahydrate (0.7 mg), and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH. MOUNJARO has a pH of 6.5 to 7.5.

No preservative.

No U.S. standard of potency.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

www.mounjaro.com

PHARMACY LABEL HERE

3 0002-1484-80 4

SH068DAAM01

4 Single-dose prefilled pens

7.5 mg / 0.5 mL

mounjaro®
(tirzepatide) injection

mounjaro®
(tirzepatide) injection

7.5 mg / 0.5 mL

4 Single-dose prefilled pens

7.5 mg / 0.5 mL

mounjaro®
(tirzepatide) injection

4 Single-dose prefilled pens

PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 10 mg/0.5 mL Dose

NDC 0002-1471-80

Mounjaro®

(tirzepatide) injection

10 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 10 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly

NDC 0002-1471-80

mounjaro®
(tirzepatide) injection

4 x 10 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Use one pen every week.

4 Single-dose prefilled pens

10 mg / 0.5 mL

Lilly

mounjaro®
(tirzepatide) injection

10 mg / 0.5 mL

4 Single-dose prefilled pens

mounjaro®
(tirzepatide) injection

10 mg / 0.5 mL

4 Single-dose prefilled pens

mounjaro®
(tirzepatide) injection

10 mg / 0.5 mL

4 Single-dose prefilled pens

PHARMACY LABEL HERE

3 0002-1471-80 4

SH06 90AAM01

Store refrigerated at 2°C to 8°C (36°F to 46°F) in original carton to protect from light. DO NOT FREEZE.
MOUNJARO can be stored at room temperature up to 30°C (86°F) for up to 21 days in the carton. Discard if not used within 21 days after removing from the refrigerator.
Keep out of reach of children.
Recommended Dosage: See prescribing information.
Carton Contents:
4 Single-dose prefilled pens
Package Insert
Medication Guide
Instructions for Use
Each 0.5 mL of MOUNJARO contains 10 mg tirzepatide; sodium chloride (4.1 mg), sodium phosphate dibasic heptahydrate (0.7 mg), and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH.
MOUNJARO has a pH of 6.5 to 7.5.
No preservative.
No U.S. standard of potency.
Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

www.mounjaro.com

LOT: 0B3002A
EXP: 18Q4
Lot/ Exp/ Serial No

4 Single-dose prefilled pens

PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 12.5 mg/0.5 mL Dose

NDC 0002-1460-80

Mounjaro®

(tirzepatide) injection

12.5 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 12.5 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly

mounjaro®
(tirzepatide) injection

12.5 mg / 0.5 mL

4 Single-dose prefilled pens



Use one pen every week.

Dispense enclosed Medication Guide to each patient.

Rx Only

For Subcutaneous Use

4 x 12.5 mg/0.5 mL prefilled pens

NDC 0002-1460-80

Lilly

mounjaro®
(tirzepatide) injection

12.5 mg / 0.5 mL

GTIN: 00300021460808

Lot / Exp. / Serial No

4 Single-dose prefilled pens

Store refrigerated at 2 °C to 8 °C (36 °F to 46 °F) in original carton to protect from light. DO NOT FREEZE.

MOUNJARO can be stored at room temperature up to 30 °C (86 °F) for up to 21 days in the carton. Discard if not used within 21 days after removing from the refrigerator.

Keep out of reach of children.

Recommended Dosage: See prescribing information.

Carton Contents:
4 Single-dose prefilled pens
Package Insert
Medication Guide
Instructions for Use

Each 0.5 mL of MOUNJARO contains 12.5 mg tirzepatide; sodium chloride (4.1 mg), sodium phosphate dibasic heptahydrate (0.7 mg), and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH. MOUNJARO has a pH of 6.5 to 7.5.

No preservative.

No U.S. standard of potency.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

www.mounjaro.com

3 0002-1460-80 8

SH07 00AAM01

PHARMACY LABEL HERE

mounjaro®
(tirzepatide) injection

12.5 mg / 0.5 mL

4 Single-dose prefilled pens



mounjaro®
(tirzepatide) injection

12.5 mg / 0.5 mL



4 Single-dose prefilled pens

PACKAGE LABEL - Mounjaro Prefilled Pen (Autoinjector) - 15 mg/0.5 mL Dose

NDC 0002-1457-80

Mounjaro®

(tirzepatide) injection

15 mg/0.5 mL

Use one pen every week.

4 Single-dose prefilled pens

4 x 15 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Lilly

NDC 0002-1457-80

mounjaro®
(tirzepatide) injection

4 x 15 mg/0.5 mL prefilled pens

Rx Only

For Subcutaneous Use

Dispense enclosed Medication Guide to each patient.

Use one pen every week.

4 Single-dose prefilled pens

15 mg / 0.5 mL



Lilly

mounjaro®
(tirzepatide) injection

15 mg / 0.5 mL

4 Single-dose prefilled pens

Lot/ Exp/ Serial No

GTIN: 00300021457308

4 Single-dose prefilled pens

PHARMACY LABEL HERE

Store refrigerated at 2°C to 8°C (36°F to 46°F) in original carton to protect from light. DO NOT FREEZE.
MOUNJARO can be stored at room temperature up to 30°C (86°F) for up to 21 days in the carton. Discard if not used within 21 days after removing from the refrigerator
Keep out of reach of children
Recommended Dosage: See prescribing information.
Carton Contents:
4 Single-dose prefilled pens
Package insert
Medication Guide
Instructions for Use

Each 0.5 mL of MOUNJARO contains 15 mg tirzepatide; sodium chloride 1.1 mg, sodium phosphate dibasic heptahydrate (0.7 mg), and water for injection. Hydrochloric acid solution and/or sodium hydroxide solution may have been added to adjust the pH. MOUNJARO has a pH of 6.5 to 7.5.

No preservative.

No U.S. standard of potency.

Marketed by:
Lilly USA, LLC
Indianapolis, IN 46285, USA

www.mounjaro.com

3 0002-1457-80 8

SH07 10AAM01

mounjaro®
(tirzepatide) injection

15 mg / 0.5 mL

4 Single-dose prefilled pens

PACKAGE LABEL - Mounjaro Single-Dose Vial - 2.5 mg/0.5 mL Dose

NDC 0002-1152-01

Mounjaro®

(tirzepatide) injection

2.5 mg/0.5 mL

Single-dose Vial - discard unused portion

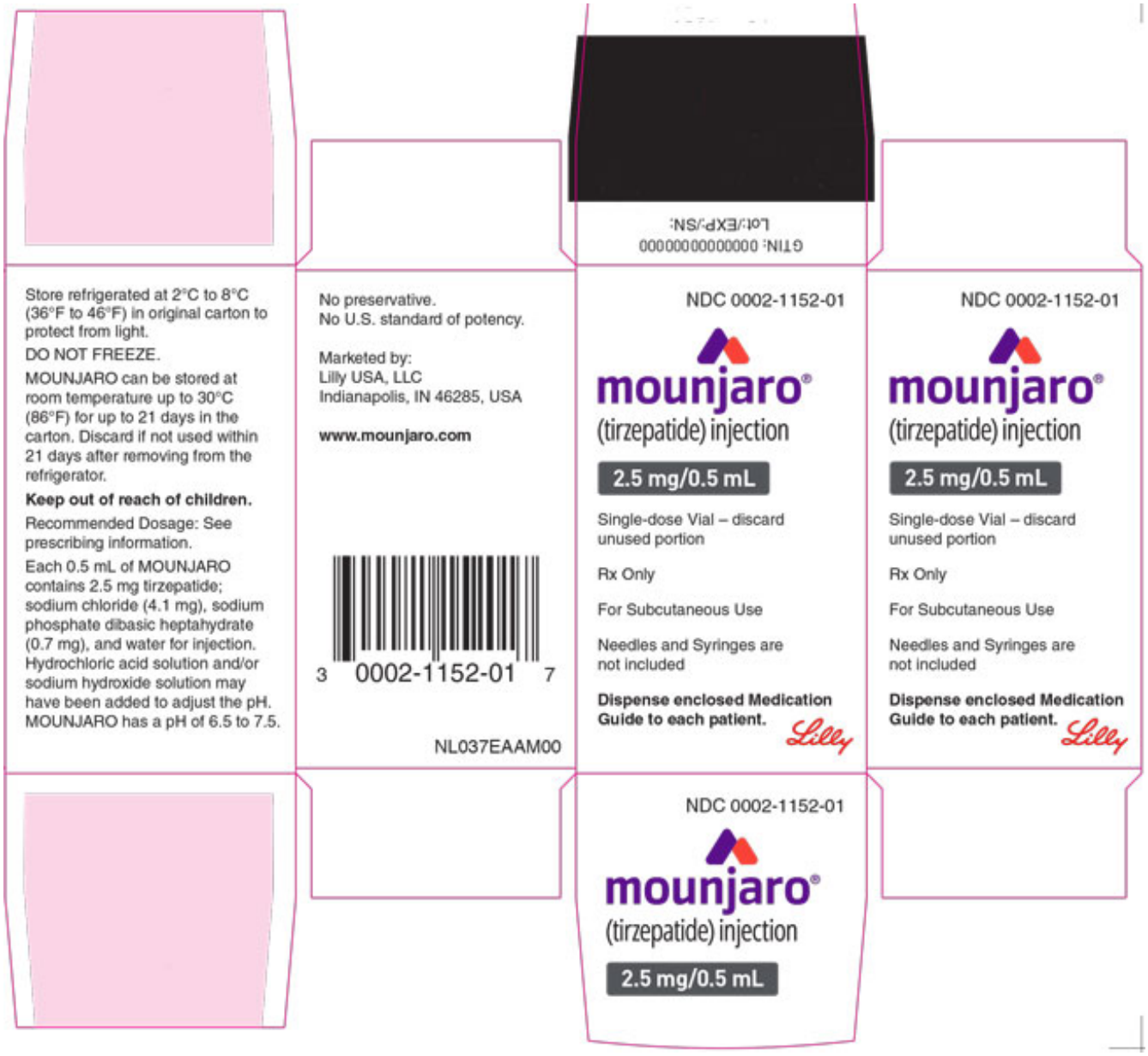
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Single-Dose Vial - 5 mg/0.5 mL Dose

NDC 0002-1243-01

Mounjaro®

(tirzepatide) injection

5 mg/0.5 mL

Single-dose Vial - discard unused portion

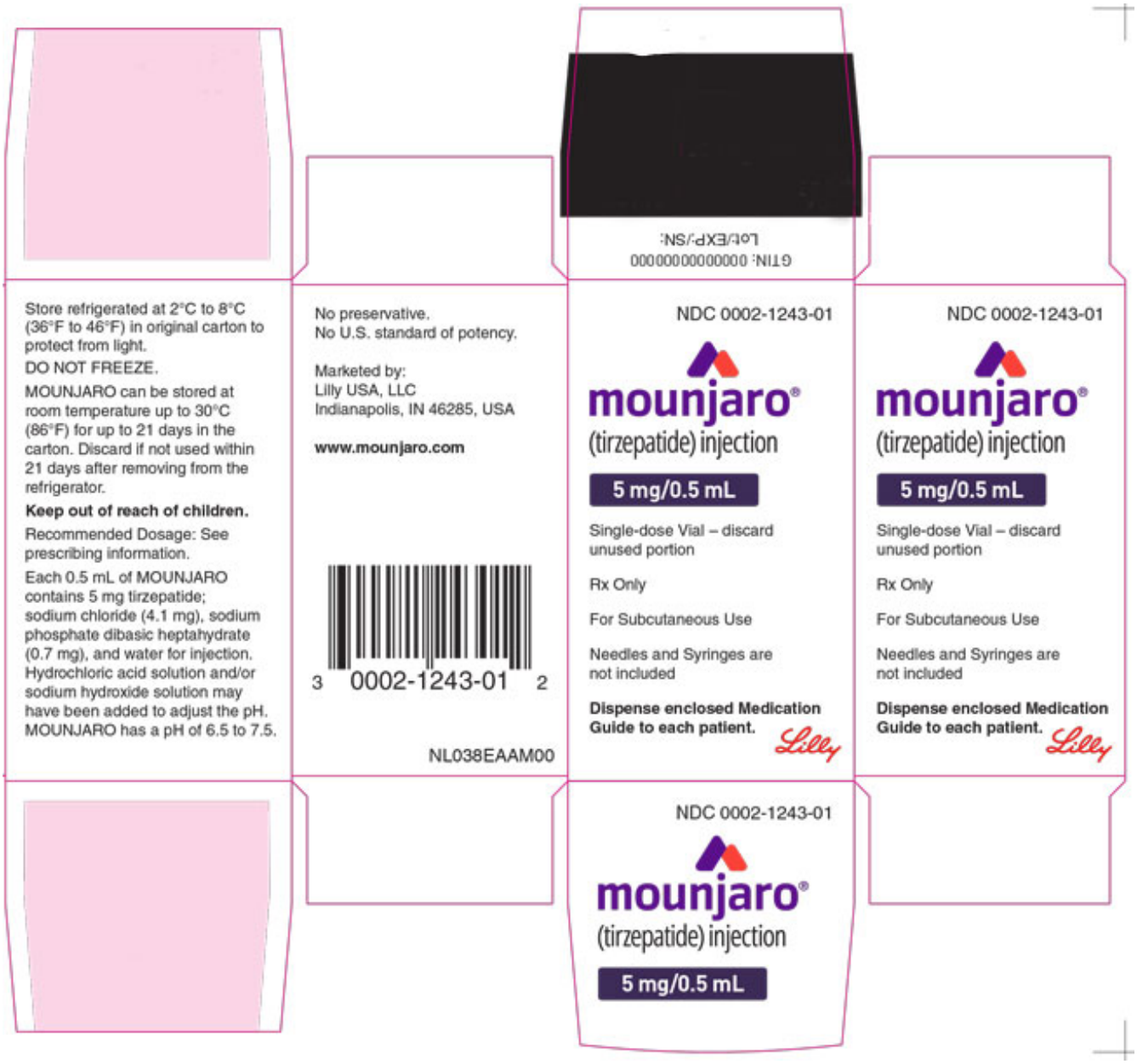
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Single-Dose Vial - 7.5 mg/0.5 mL Dose

NDC 0002-2214-01

Mounjaro®

(tirzepatide) injection

7.5 mg/0.5 mL

Single-dose Vial - discard unused portion

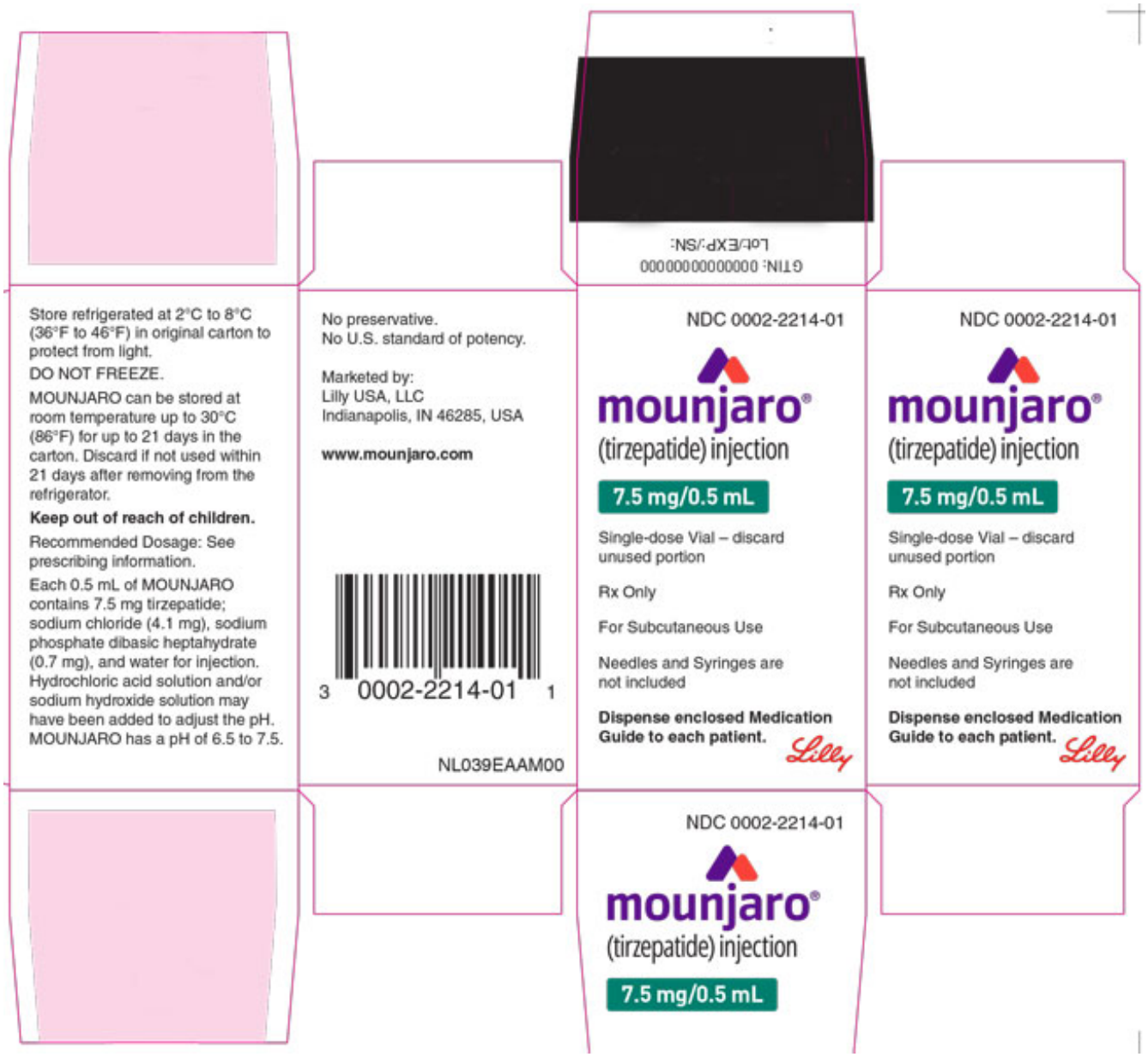
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Single-Dose Vial - 10 mg/0.5 mL Dose

NDC 0002-2340-01

Mounjaro®

(tirzepatide) injection

10 mg/0.5 mL

Single-dose Vial - discard unused portion

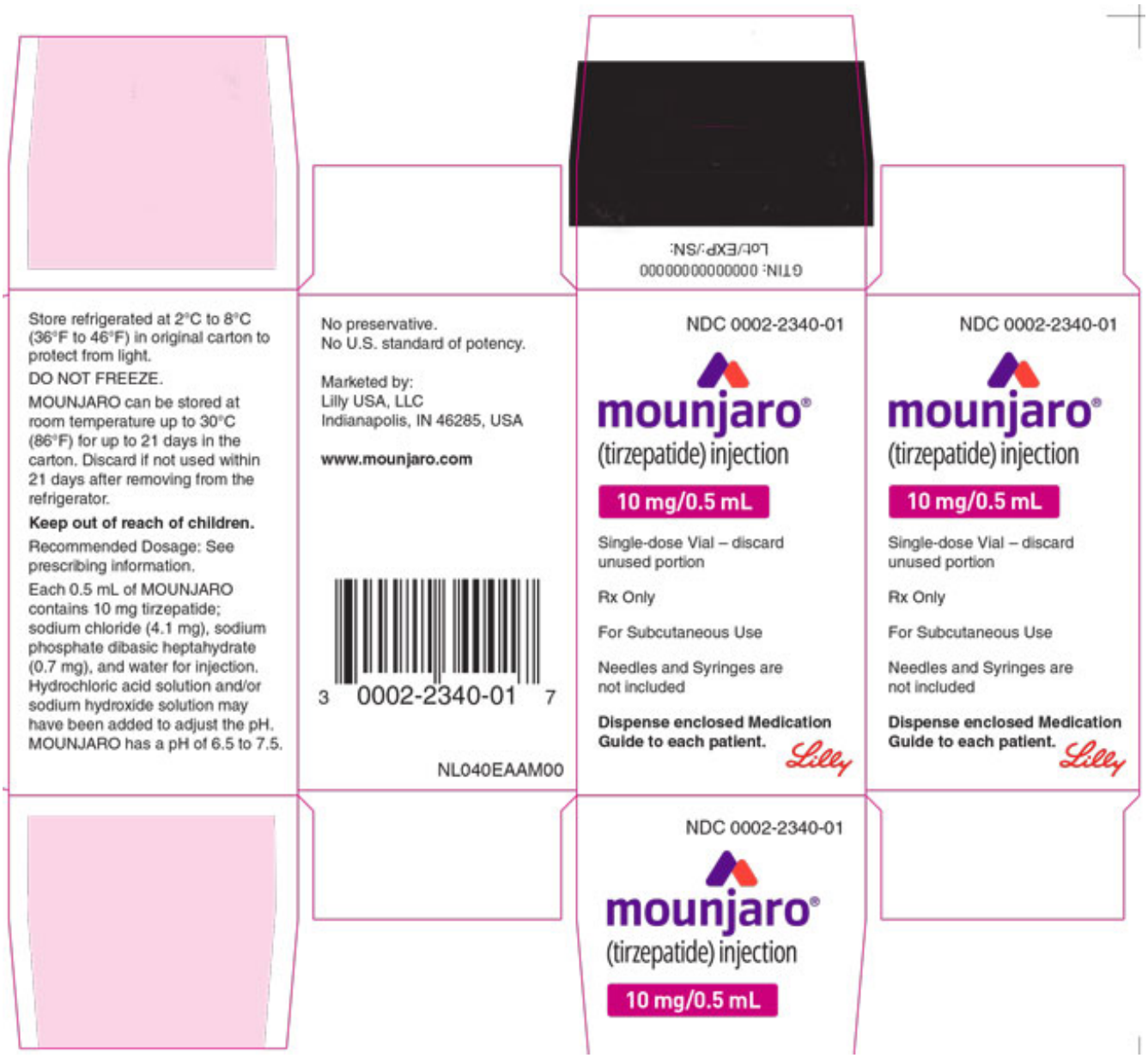
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Single-Dose Vial - 12.5 mg/0.5 mL Dose

NDC 0002-2423-01

Mounjaro®

(tirzepatide) injection

12.5 mg/0.5 mL

Single-dose Vial - discard unused portion

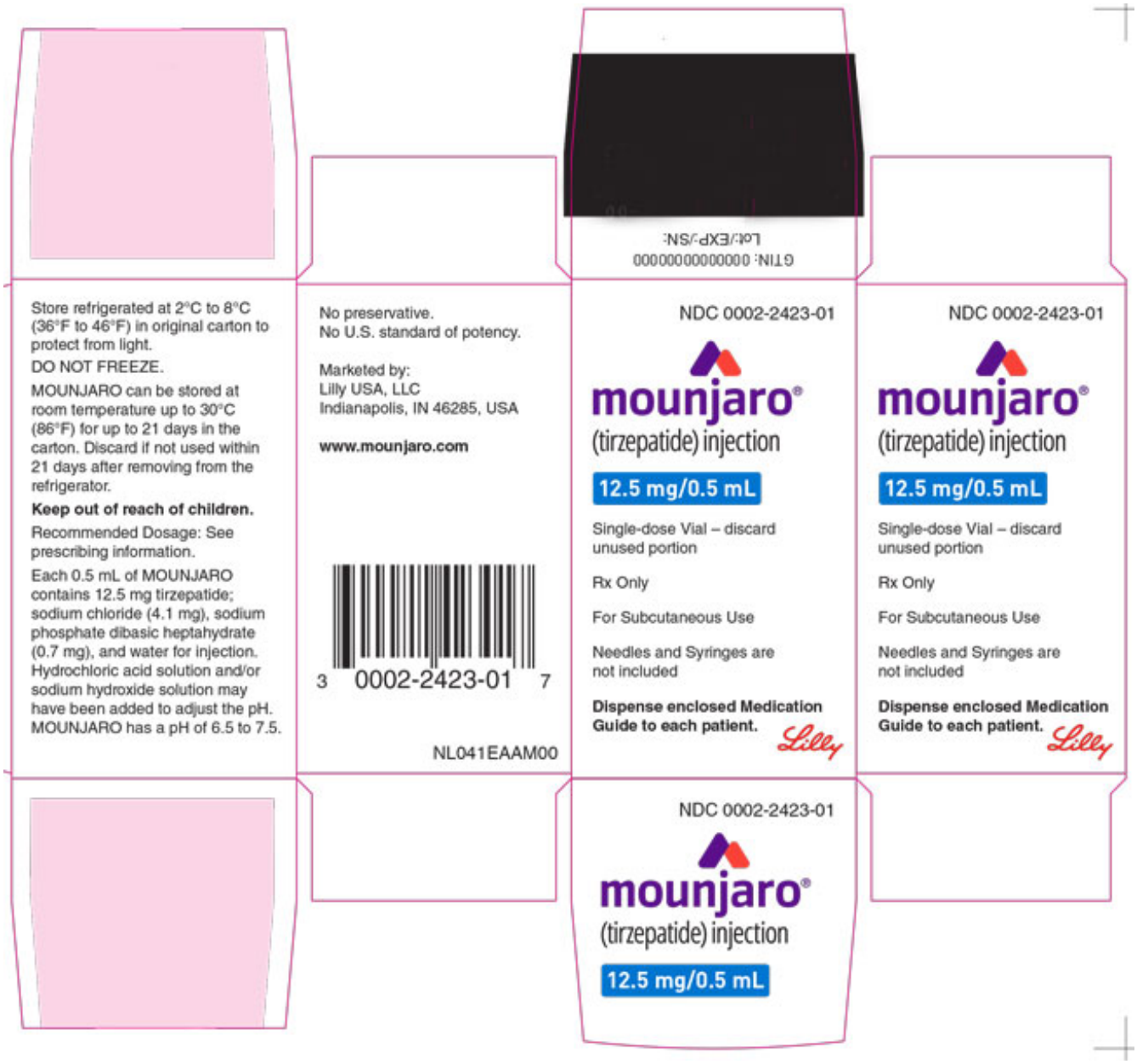
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Single-Dose Vial - 15 mg/0.5 mL Dose

NDC 0002-3002-01

Mounjaro®

(tirzepatide) injection

15 mg/0.5 mL

Single-dose Vial - discard unused portion

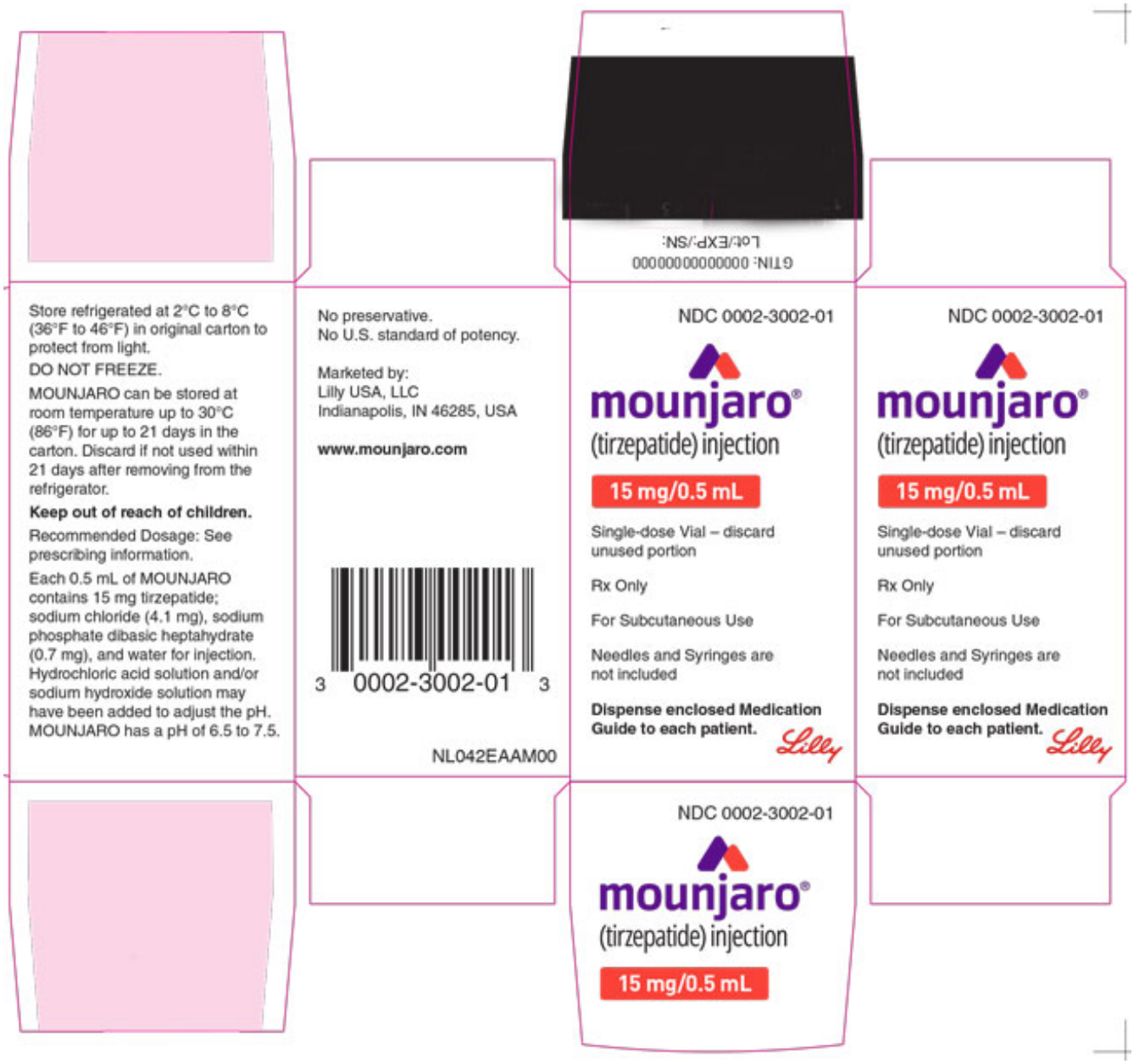
Rx Only

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 2.5 mg/0.6 mL Dose (10 mg/2.4 mL [4.17 mg/mL])

Rx Only

NDC 0002-4052-11

Mounjaro

(tirzepatide) injection

10 mg/2.4 mL
(4.17 mg/mL)

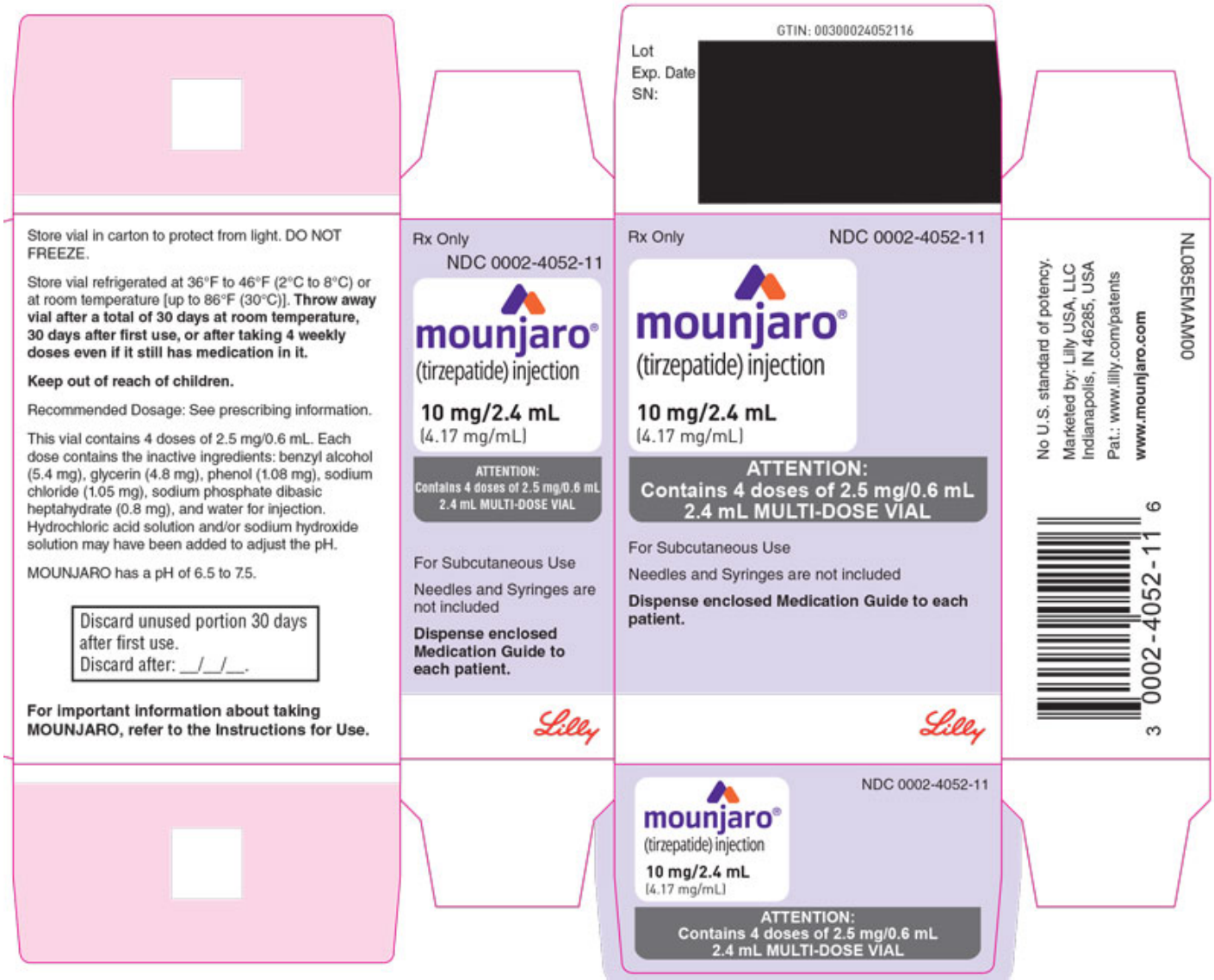
ATTENTION:
Contains 4 doses of 2.5 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 5 mg/0.6 mL Dose (20 mg/2.4 mL [8.33 mg/mL])

Rx Only

NDC 0002-4103-11

Mounjaro

(tirzepatide) injection

20 mg/2.4 mL
(8.33 mg/mL)

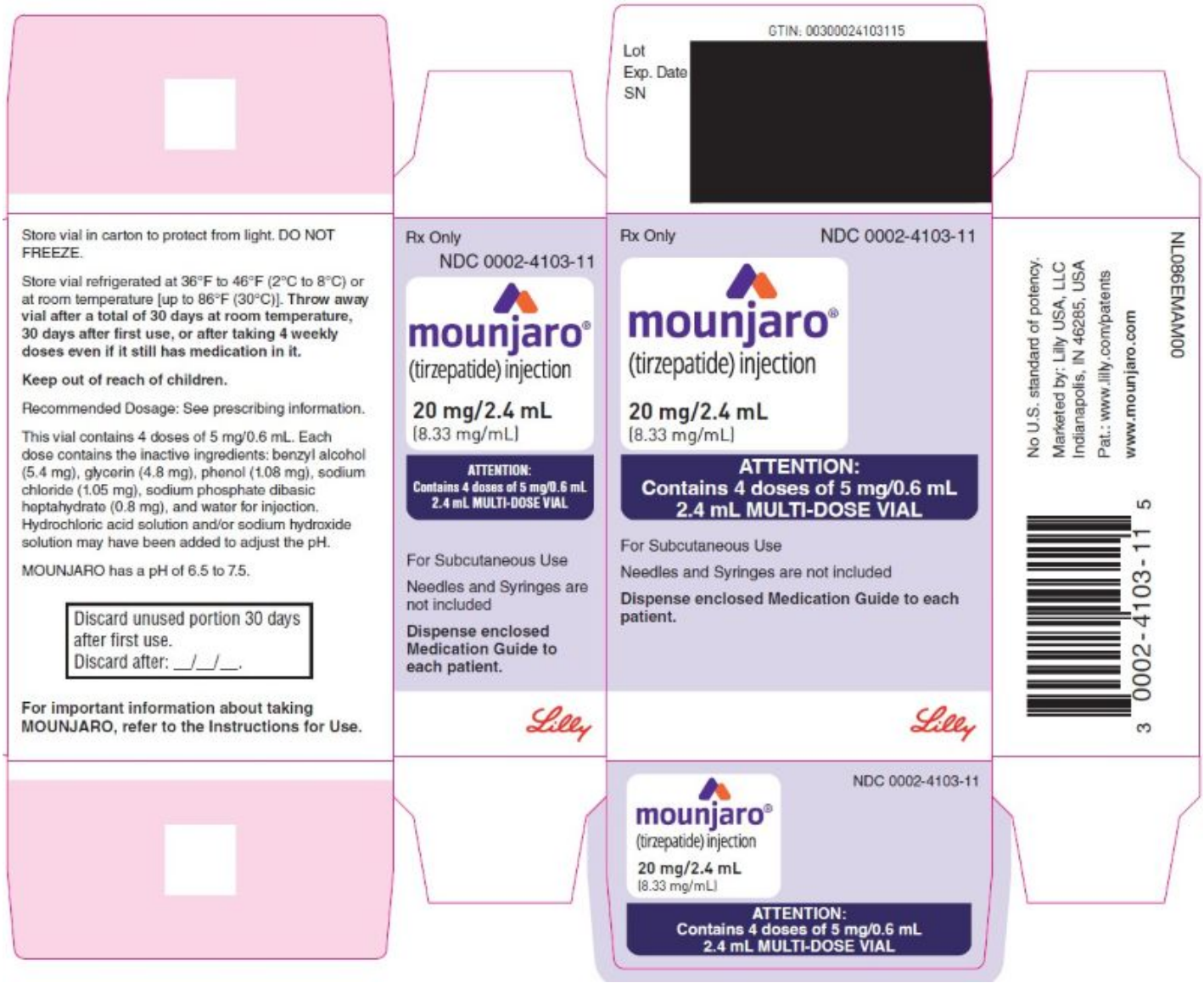
ATTENTION:
Contains 4 doses of 5 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 7.5 mg/0.6 mL Dose (30 mg/2.4 mL [12.5 mg/mL])

Rx Only

NDC 0002-4210-11

Mounjaro

(tirzepatide) injection

30 mg/2.4 mL
(12.5 mg/mL)

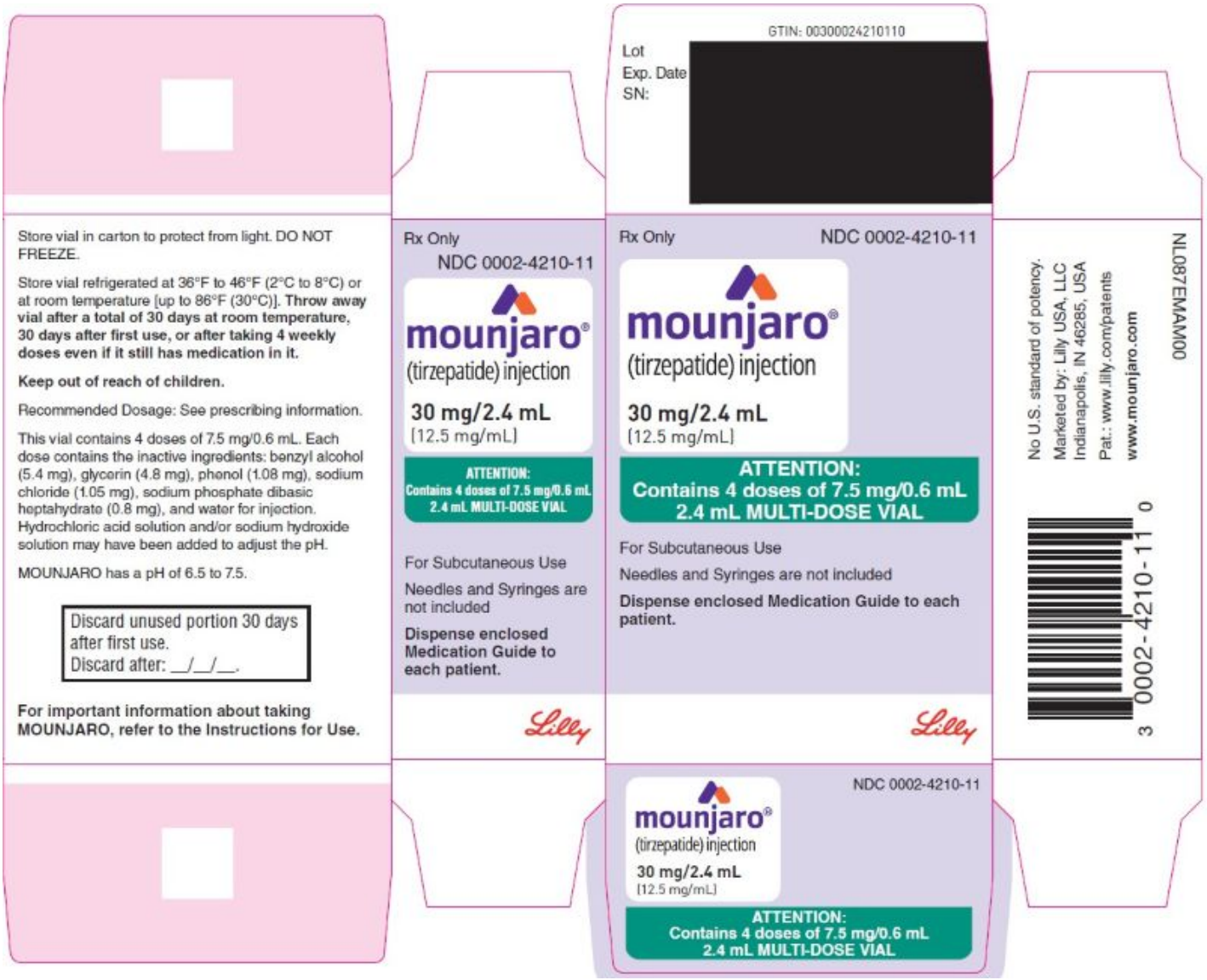
ATTENTION:
Contains 4 doses of 7.5 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 10 mg/0.6 mL Dose (40 mg/2.4 mL [16.7 mg/mL])

Rx Only

NDC 0002-4304-11

Mounjaro

(tirzepatide) injection

40 mg/2.4 mL
(16.7 mg/mL)

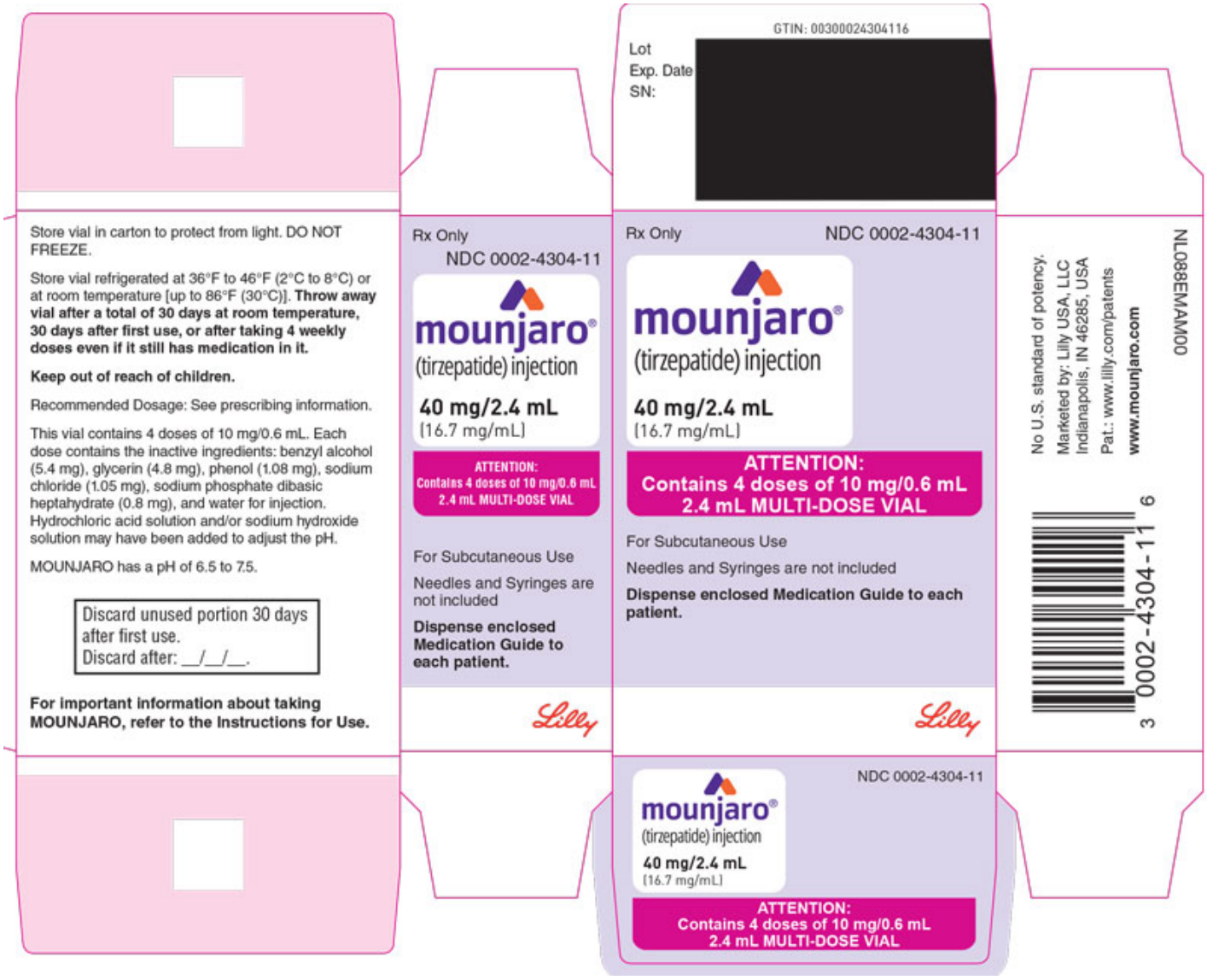
ATTENTION:
Contains 4 doses of 10 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 12.5 mg/0.6 mL Dose (50 mg/2.4 mL [20.8 mg/mL])

Rx Only

NDC 0002-4523-11

Mounjaro

(tirzepatide) injection

50 mg/2.4 mL
(20.8 mg/mL)

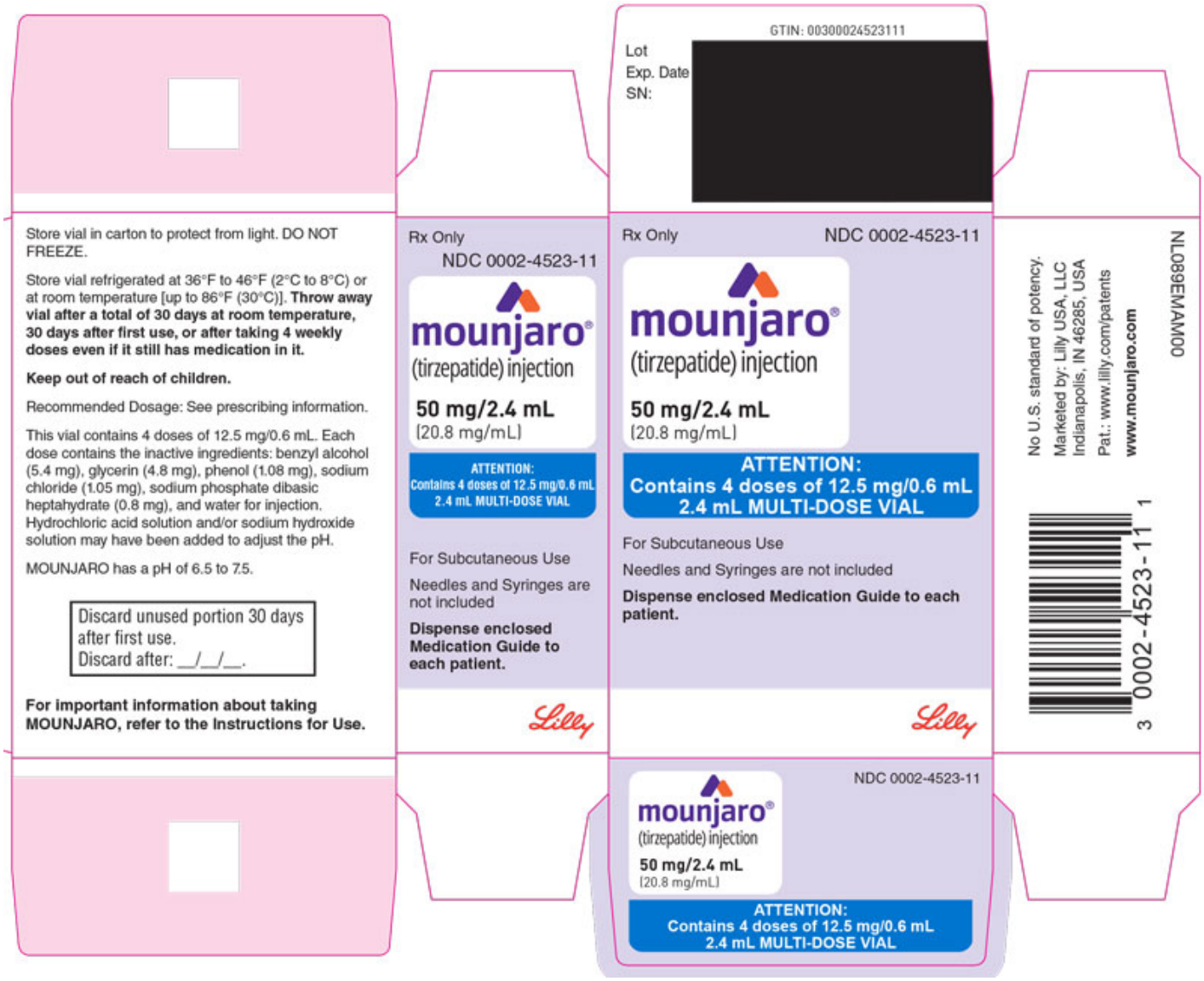
ATTENTION:
Contains 4 doses of 12.5 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL - Mounjaro Multi-Dose Vial - 15 mg/0.6 mL Dose (60 mg/2.4 mL[25 mg/mL])

Rx Only

NDC 0002-4612-11

Mounjaro

(tirzepatide) injection

60 mg/2.4 mL
(25 mg/mL)

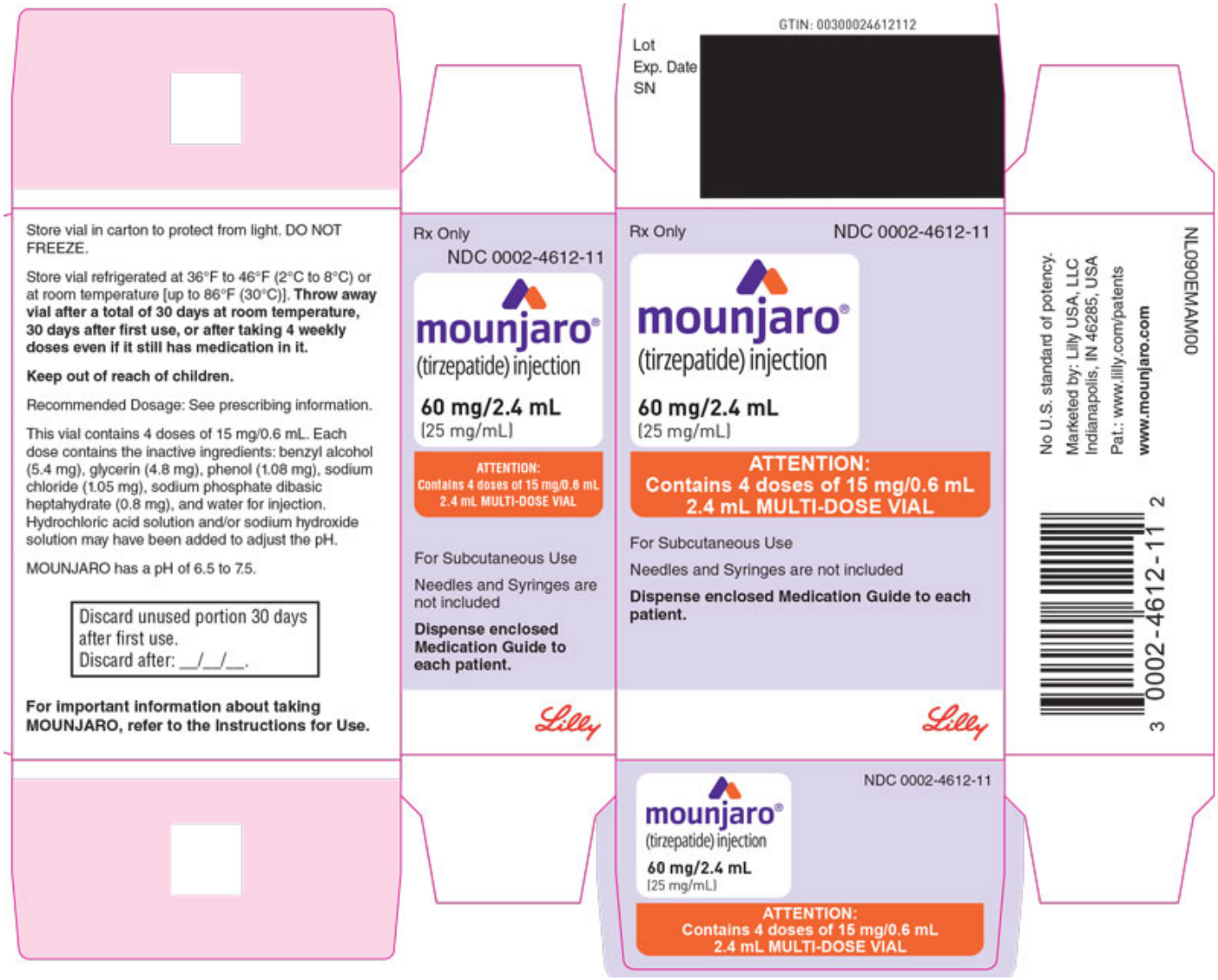
ATTENTION:
Contains 4 doses of 15 mg/0.6 mL
2.4 mL MULTI-DOSE VIAL

For Subcutaneous Use

Needles and Syringes are not included

Dispense enclosed Medication Guide to each patient.

Lilly



PACKAGE LABEL – MOUNJARO KwikPen – 2.5 mg per dose (10 mg/2.4 mL [4.17 mg/mL])

NDC 0002-3466-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

2.5 mg per dose

10 mg/2.4 mL (4.17 mg/mL)

1 Pen with 4 doses of 2.5 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



PACKAGE LABEL – MOUNJARO KwikPen – 5 mg per dose (20 mg/2.4 mL [8.33 mg/mL])

NDC 0002-3455-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

5 mg per dose

20 mg/2.4 mL (8.33 mg/mL)

1 Pen with 4 doses of 5 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

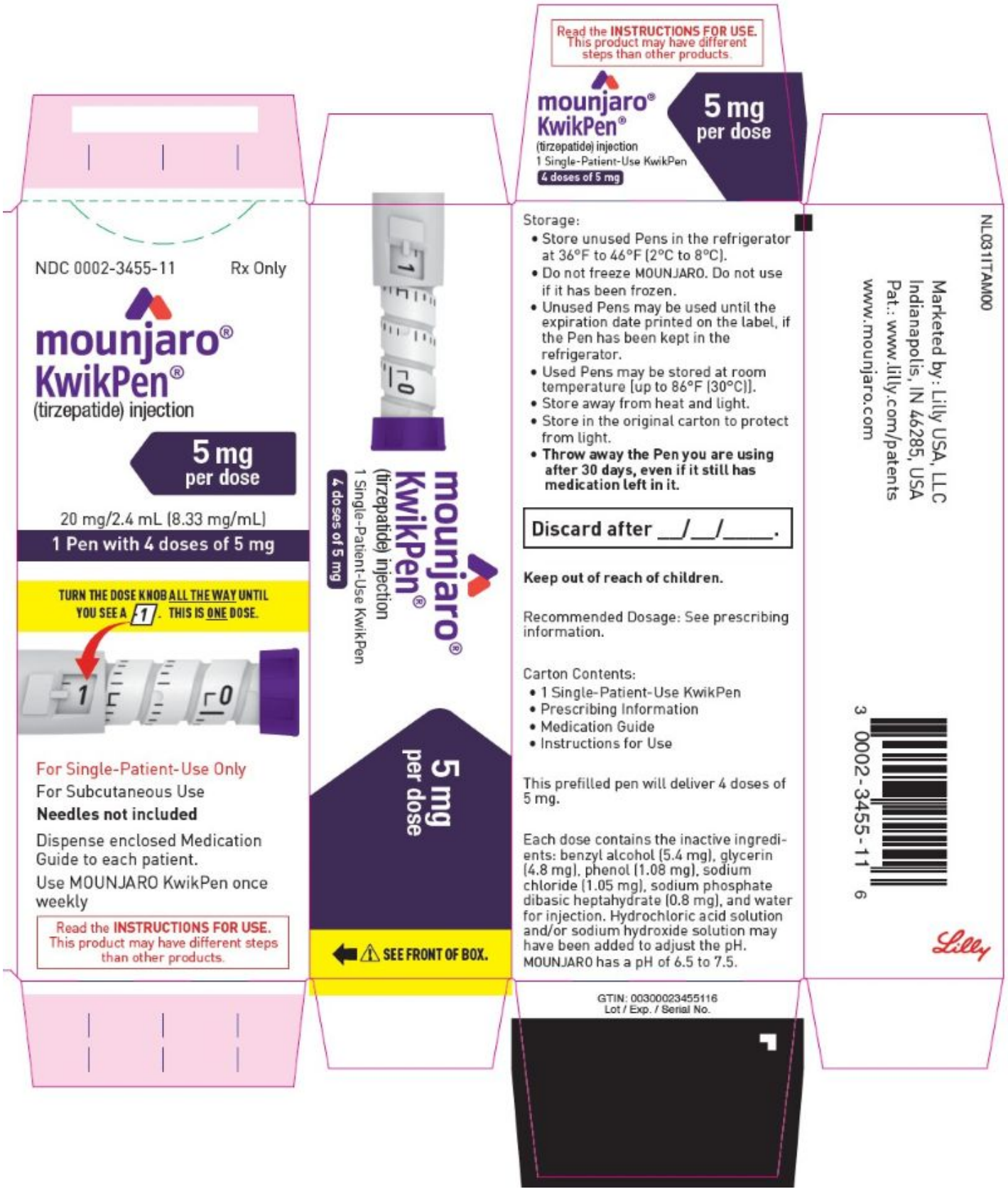
For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



PACKAGE LABEL – MOUNJARO KwikPen – 7.5 mg per dose (30 mg/2.4 mL [12.5 mg/mL])

NDC 0002-3444-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

7.5 mg per dose

30 mg/2.4 mL (12.5 mg/mL)

1 Pen with 4 doses of 7.5 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

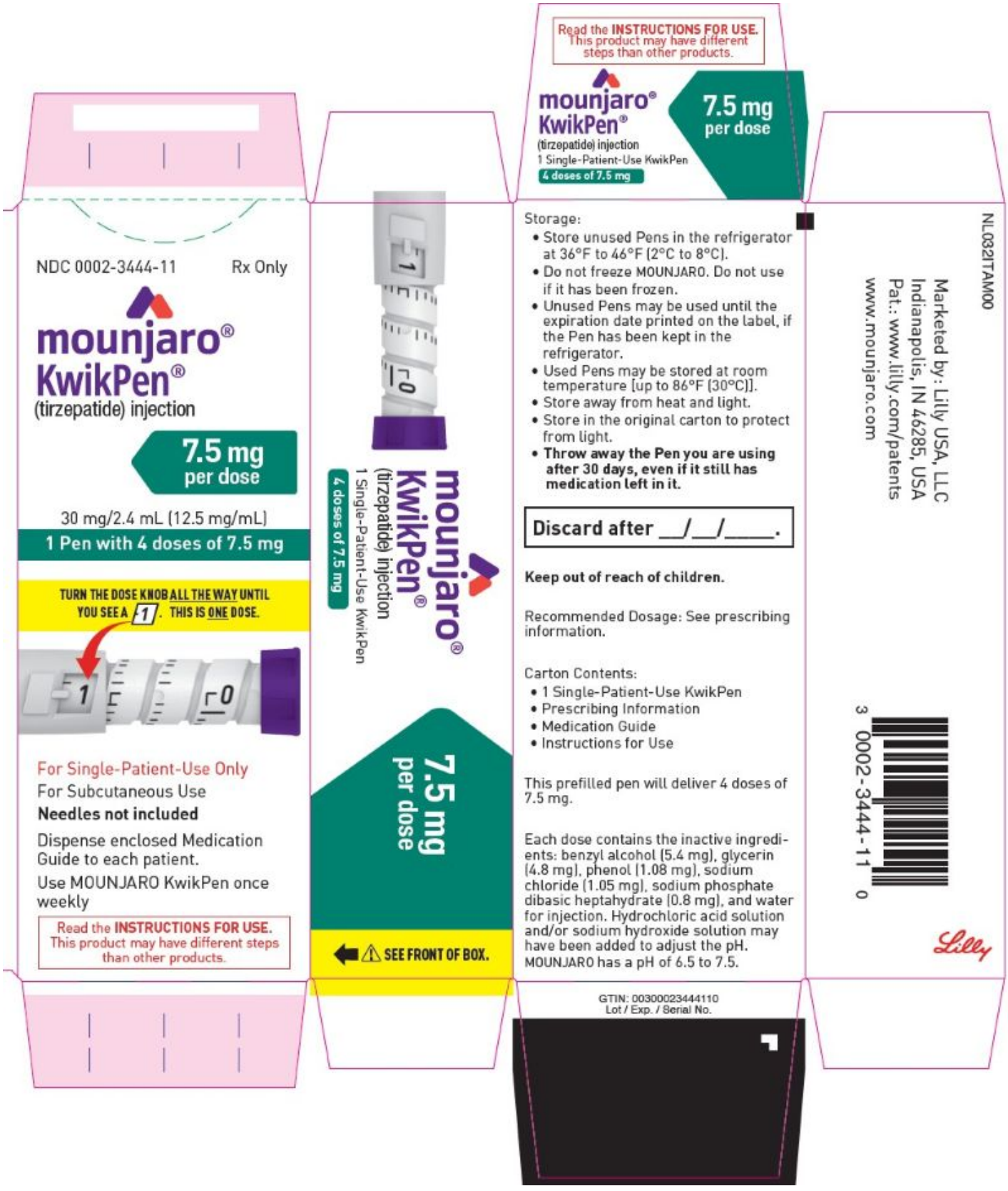
For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



PACKAGE LABEL – MOUNJARO KwikPen – 10 mg per dose (40 mg/2.4 mL [16.7 mg/mL])

NDC 0002-3433-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

10 mg per dose

40 mg/2.4 mL (16.7 mg/mL)

1 Pen with 4 doses of 10 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



PACKAGE LABEL – MOUNJARO KwikPen – 12.5 mg per dose (50 mg/2.4 mL [20.8 mg/mL])

NDC 0002-3422-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

12.5 mg per dose

50 mg/2.4 mL (20.8 mg/mL)

1 Pen with 4 doses of 12.5 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



PACKAGE LABEL – MOUNJARO KwikPen – 15 mg per dose (60 mg/2.4 mL [25 mg/mL])

NDC 0002-3411-11

Rx Only

Mounjaro®

KwikPen®

(tirzepatide) injection

15 mg per dose

60 mg/2.4 mL (25 mg/mL)

1 Pen with 4 doses of 15 mg

TURN THE DOSE KNOB ALL THE WAY UNTIL YOU SEE A  . THIS IS ONE DOSE.

For Single-Patient-Use Only

For Subcutaneous Use

Needles not included

Dispense enclosed Medication Guide to each patient.

Use MOUNJARO KwikPen once weekly

Read the INSTRUCTIONS FOR USE. This product may have different steps than other products.



MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1506
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	2.5 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-1506-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1506-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		
2	NDC:0002-1506-61	4 in 1 CARTON	06/08/2022	
2		0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1495
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	5 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-1495-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1495-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1484
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	7.5 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-1484-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1484-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1471
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	10 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	Strength
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-1471-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1471-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1460
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	12.5 mg in 0.5 mL

Inactive Ingredients

Ingredient Name	Strength
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-1460-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1460-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1457
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	15 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-1457-80	4 in 1 CARTON	05/13/2022	
1	NDC:0002-1457-01	0.5 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	05/13/2022	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1152
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	2.5 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-1152-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-1243
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	5 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-1243-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-2214
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	7.5 mg in 0.5 mL

Inactive Ingredients

Ingredient Name	Strength
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-2214-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-2340
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	10 mg in 0.5 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging				
#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-2340-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-2423

Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	12.5 mg in 0.5 mL

Inactive Ingredients

Ingredient Name	Strength
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-2423-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3002
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	15 mg in 0.5 mL

Inactive Ingredients

Ingredient Name	Strength
Sodium Chloride (UNII: 451W47IQ8X)	4.1 mg in 0.5 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	0.7 mg in 0.5 mL
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	
Water (UNII: 059QF0KO0R)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-3002-01	1 in 1 CARTON	07/28/2023	
1		0.5 mL in 1 VIAL, SINGLE-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	07/28/2023	

MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4052
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	4.17 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-4052-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4052-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/07/2026	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4103
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	8.33 mg in 1 mL

Inactive Ingredients	
Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-4103-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4103-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/07/2026	

MOUNJARO

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4210
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	12.5 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-4210-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4210-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/07/2026	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4304
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	16.7 mg in 1 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-4304-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4304-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/07/2026	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4523
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	20.8 mg in 1 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-4523-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4523-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/07/2026	

MOUNJARO

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-4612
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	25 mg in 1 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-4612-11	1 in 1 CARTON	01/07/2026	
1	NDC:0002-4612-01	2.4 mL in 1 VIAL, MULTI-DOSE; Type 0: Not a Combination Product		

Marketing Information			
Marketing Category	**Application Number or Monograph Citation**	**Marketing Start Date**	**Marketing End Date**
NDA	NDA215866	01/07/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3466
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	4.17 mg in 1 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL

Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-3466-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3466-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3455
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	8.33 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item	Package Description	Marketing	Marketing

#	Code	Package Description	Start Date	End Date
1	NDC:0002-3455-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3455-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3444
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	12.5 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-3444-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3444-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information

Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3433
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	16.7 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-3433-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3433-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3422
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety		
Ingredient Name	**Basis of Strength**	**Strength**
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	20.8 mg in 1 mL

Inactive Ingredients	
Ingredient Name	**Strength**
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging				
#	**Item Code**	**Package Description**	**Marketing Start Date**	**Marketing End Date**
1	NDC:0002-3422-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3422-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

MOUNJARO KWIKPEN

tirzepatide injection, solution

Product Information			
Product Type	HUMAN PRESCRIPTION DRUG	**Item Code (Source)**	NDC:0002-3411
Route of Administration	SUBCUTANEOUS		

Active Ingredient/Active Moiety

Ingredient Name	Basis of Strength	Strength
tirzepatide (UNII: OYN3CCI6QE) (tirzepatide - UNII:OYN3CCI6QE)	tirzepatide	25 mg in 1 mL

Inactive Ingredients

Ingredient Name	Strength
Benzyl Alcohol (UNII: LKG8494WBH)	9 mg in 1 mL
Glycerin (UNII: PDC6A3C0OX)	8 mg in 1 mL
Phenol (UNII: 339NCG44TV)	1.80 mg in 1 mL
Sodium Chloride (UNII: 451W47IQ8X)	1.75 mg in 1 mL
Sodium Phosphate, Dibasic, Heptahydrate (UNII: 70WT22SF4B)	1.34 mg in 1 mL
Water (UNII: 059QF0KO0R)	
Hydrochloric Acid (UNII: QTT17582CB)	
Sodium Hydroxide (UNII: 55X04QC32I)	

Packaging

#	Item Code	Package Description	Marketing Start Date	Marketing End Date
1	NDC:0002-3411-11	1 in 1 CARTON	01/20/2026	
1	NDC:0002-3411-01	2.4 mL in 1 SYRINGE; Type 2: Prefilled Drug Delivery Device/System (syringe, patch, etc.)		

Marketing Information

Marketing Category	Application Number or Monograph Citation	Marketing Start Date	Marketing End Date
NDA	NDA215866	01/20/2026	

Labeler - Eli Lilly and Company (006421325)

Revised: 8/2026 Eli Lilly and Company